

StudyDoc

100 Most Important Video-Based Clinical Atlas for NEET PG 2026

A high-yield visual revision atlas of dynamic clinical signs, examination findings, procedures, and imaging clips for NEET PG aspirants.

Website: <https://studymdoc.co.in>

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Video links + QR codes + exam-oriented explanations

How to use this atlas

Student workflow

- Open the topic.
- Click here to watch video or scan the QR code.
- Watch only the suggested timestamp segment.
- Return to the atlas and revise the explanation.
- Focus especially on the differential diagnosis and common traps.
- Use the quick revision box for last-minute recall.

Disclaimer

This free educational atlas links to publicly available educational videos and provides original explanatory notes for NEET PG preparation. External videos belong to their respective creators/channels. This resource is not affiliated with NBEMS/NMC and does not contain recalled exam questions.

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Topic 01 - Parkinsonian gait

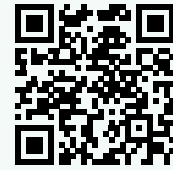
Subject: Medicine / Neurology

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:48

Source credit: Resting Tremors and Shuffling Gait in Parkinson's Disease - Clinical Snippets-By Dr. Sourya Acharya-DMIHER



What it is

Parkinsonian gait: short video directly showing shuffling gait, stooped posture, reduced arm swing, festination. High-yield anchors: basal ganglia dysfunction; dopamine deficiency; Parkinson disease; drug-induced parkinsonism; levodopa.

How to identify it in the video

short video directly showing shuffling gait, stooped posture, reduced arm swing, festination. Watch for basal ganglia dysfunction, dopamine deficiency, Parkinson disease, drug-induced parkinsonism. Video cues for Parkinsonian gait: posture and trunk position, step height and stride length, base of support, arm swing symmetry, rhythm, turning, and freezing or imbalance.

Why it happens

The mechanism of Parkinsonian gait starts with basal ganglia dysfunction and is expressed clinically through dopamine deficiency. Related exam anchors are Parkinson disease, drug-induced parkinsonism, levodopa. In Parkinsonian gait, basal ganglia dysfunction drives the walking pattern and dopamine deficiency explains the visible step, posture, or balance abnormality.

Associated conditions

- basal ganglia dysfunction
- dopamine deficiency
- Parkinson disease
- drug-induced parkinsonism
- levodopa
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
cerebellar gait	Broad-based, staggering gait with truncal sway, dysmetria, nystagmus, and worse turning.	Look for nystagmus and truncal sway.
steppage gait	High hip and knee flexion to clear a dropped foot; dorsiflexion weakness is the key clue.	Localize to common peroneal nerve or L5 root.
waddling gait	Side-to-side trunk sway from bilateral proximal pelvic-girdle weakness.	Calf hypertrophy, CK, and myopathy history.
hemiplegic gait	Spastic circumduction of one leg with ipsilateral upper-limb flexor posturing.	Extensor plantar response, and stroke imaging clues.
sensory ataxic gait	Romberg becomes positive.	Vibration and joint-position sense are reduced.
antalgic gait	Short stance phase caused by pain; the patient avoids weight bearing on the painful side.	Examine hip, knee, ankle, spine, and weight-bearing X-ray when needed.

Clinical examination points

- Parkinsonian gait: Observe posture, base, stride length, arm swing, turning, balance, and need for support.
- Parkinsonian gait: Test power, tone, reflexes, plantar response, sensation, coordination, and Romberg.

NEET PG exam angle

Parkinsonian gait is usually tested through basal ganglia dysfunction; dopamine deficiency; Parkinson disease. Separate it from cerebellar gait, steppage gait, waddling gait.

Management relevance

Treat Parkinson disease with dopaminergic therapy such as levodopa when indicated; review dopamine-blocking drugs.

Common traps

- Do not confuse with cerebellar gait: Broad-based, staggering gait with truncal sway, dysmetria, nystagmus, and worse turning.
- Do not confuse with steppage gait: High hip and knee flexion to clear a dropped foot; dorsiflexion weakness is the key clue.
- Do not confuse with waddling gait: Side-to-side trunk sway from bilateral proximal pelvic-girdle weakness.
- Parkinsonian gait needs both basal ganglia dysfunction and dopamine deficiency; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Parkinsonian gait.
- Subject: Medicine / Neurology.
- Key clue: basal ganglia dysfunction.
- Key clue: dopamine deficiency.
- Key clue: Parkinson disease.
- Closest mimics: cerebellar gait, steppage gait, waddling gait, hemiplegic gait.
- Video segment: 00:00-00:48.
- One-line recall: Parkinsonian gait points to basal ganglia dysfunction.

Source credit: Resting Tremors and Shuffling Gait in Parkinson's Disease - Clinical Snippets-By Dr. Sourya Acharya-DMIHER.

Topic 02 - Cerebellar ataxic gait

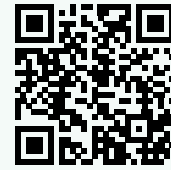
Subject: Medicine / Neurology

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:24

Source credit: Case Challenge: Wide-based, ataxic gait. (Video 4) - jwatch.org



What it is

Cerebellar ataxic gait: broad-based staggering gait or cerebellar ataxia demonstration. High-yield anchors: cerebellar lesion; broad-based gait; truncal ataxia; intention tremor; nystagmus.

How to identify it in the video

broad-based staggering gait or cerebellar ataxia demonstration. Watch for cerebellar lesion, broad-based gait, truncal ataxia, intention tremor. Video cues for Cerebellar ataxic gait: posture and trunk position, step height and stride length, base of support, arm swing symmetry, rhythm, turning, and freezing or imbalance.

Why it happens

The mechanism of Cerebellar ataxic gait starts with cerebellar lesion and is expressed clinically through broad-based gait. Related exam anchors are truncal ataxia, intention tremor, nystagmus. In Cerebellar ataxic gait, cerebellar lesion drives the walking pattern and broad-based gait explains the visible step, posture, or balance abnormality.

Associated conditions

- cerebellar lesion
- broad-based gait
- truncal ataxia
- intention tremor
- nystagmus
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
sensory ataxia	Stomping gait that worsens with eye closure; Romberg is classically positive.	Cerebellar signs may be absent.
vestibular ataxia	Veering with vertigo, nausea, and nystagmus rather than limb dysmetria.	Vertigo, nausea, and hearing symptoms guide the diagnosis.
alcohol intoxication	Check odor, slurred speech.	Gaze nystagmus, glucose, trauma, and toxicology context.
Parkinsonian gait	Stooped posture, short shuffling steps, reduced arm swing, festination, and difficulty turning.	Bradykinesia, reduced arm swing, and response to levodopa.

Clinical examination points

- Cerebellar ataxic gait: Observe posture, base, stride length, arm swing, turning, balance, and need for support.
- Cerebellar ataxic gait: Test power, tone, reflexes, plantar response, sensation, coordination, and Romberg.

NEET PG exam angle

Cerebellar ataxic gait is usually tested through cerebellar lesion; broad-based gait; truncal ataxia. Separate it from sensory ataxia, vestibular ataxia, alcohol intoxication.

Management relevance

Treat the cause, exclude stroke/toxin/metabolic causes, and use fall-prevention rehabilitation.

Common traps

- Do not confuse with sensory ataxia: Stomping gait that worsens with eye closure; Romberg is classically positive.
- Do not confuse with vestibular ataxia: Veering with vertigo, nausea, and nystagmus rather than limb dysmetria.
- Do not confuse with alcohol intoxication: Check odor, slurred speech.
- Cerebellar ataxic gait needs both cerebellar lesion and broad-based gait; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Cerebellar ataxic gait.
- Subject: Medicine / Neurology.
- Key clue: cerebellar lesion.
- Key clue: broad-based gait.
- Key clue: truncal ataxia.
- Closest mimics: sensory ataxia, vestibular ataxia, alcohol intoxication, Parkinsonian gait.
- Video segment: 00:00-00:24.
- One-line recall: Cerebellar ataxic gait points to cerebellar lesion.

Source credit: Case Challenge: Wide-based, ataxic gait. (Video 4) - jwatch.org.

Topic 03 - Hemiplegic / circumduction gait

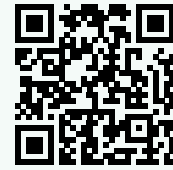
Subject: Medicine / Neurology

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Circumduction Gait(Hemiplegic gait) - Discover CLINICAL MEDICINE -Dr.T.Bhargav chandan



What it is

Hemiplegic / circumduction gait: circumduction gait after stroke or UMN lesion. High-yield anchors: UMN lesion; spasticity; stroke; internal capsule; corticospinal tract.

How to identify it in the video

circumduction gait after stroke or UMN lesion. Watch for UMN lesion, spasticity, stroke, internal capsule. Video cues for Hemiplegic / circumduction gait: posture and trunk position, step height and stride length, base of support, arm swing symmetry, rhythm, turning, and freezing or imbalance.

Why it happens

The mechanism of Hemiplegic / circumduction gait starts with UMN lesion and is expressed clinically through spasticity. Related exam anchors are stroke, internal capsule, corticospinal tract. In Hemiplegic / circumduction gait, UMN lesion drives the walking pattern and spasticity explains the visible step, posture, or balance abnormality.

Associated conditions

- UMN lesion
- spasticity
- stroke
- internal capsule
- corticospinal tract
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
Parkinsonian gait	Stooped posture, short shuffling steps, reduced arm swing, festination, and difficulty turning.	Bradykinesia, reduced arm swing, and response to levodopa.
steppage gait	High hip and knee flexion to clear a dropped foot; dorsiflexion weakness is the key clue.	Localize to common peroneal nerve or L5 root.
cerebellar gait	Broad-based, staggering gait with truncal sway, dysmetria, nystagmus, and worse turning.	Look for nystagmus and truncal sway.
antalgic gait	Short stance phase caused by pain; the patient avoids weight bearing on the painful side.	Examine hip, knee, ankle, spine, and weight-bearing X-ray when needed.

Clinical examination points

- Hemiplegic / circumduction gait: Observe posture, base, stride length, arm swing, turning, balance, and need for support.
- Hemiplegic / circumduction gait: Test power, tone, reflexes, plantar response, sensation, coordination, and Romberg.

NEET PG exam angle

Hemiplegic / circumduction gait is usually tested through UMN lesion; spasticity; stroke. Separate it from Parkinsonian gait, steppage gait, cerebellar gait.

Management relevance

Clinical action after recognizing Hemiplegic / circumduction gait: confirm UMN lesion, look for complications, and choose treatment according to the underlying diagnosis.

Common traps

- Do not confuse with Parkinsonian gait: Stooped posture, short shuffling steps, reduced arm swing, festination, and difficulty turning.
- Do not confuse with steppage gait: High hip and knee flexion to clear a dropped foot; dorsiflexion weakness is the key clue.
- Do not confuse with cerebellar gait: Broad-based, staggering gait with truncal sway, dysmetria, nystagmus, and worse turning.
- Hemiplegic / circumduction gait needs both UMN lesion and spasticity; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Hemiplegic / circumduction gait.
- Subject: Medicine / Neurology.
- Key clue: UMN lesion.
- Key clue: spasticity.
- Key clue: stroke.
- Closest mimics: Parkinsonian gait, steppage gait, cerebellar gait, antalgic gait.
- Video segment: 00:00-01:00.
- One-line recall: Hemiplegic / circumduction gait points to UMN lesion.

Source credit: Circumduction Gait(Hemiplegic gait) - Discover CLINICAL MEDICINE -Dr.T.Bhargav chandan.

Topic 04 - Steppage gait / foot drop

Subject: Medicine / Neurology / Orthopedics

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:35

Source credit: Right Foot Drop in Ambulating Patient - NEJM Group



What it is

Steppage gait / foot drop: high-stepping gait due to foot drop. High-yield anchors: common peroneal nerve palsy; L5 radiculopathy; dorsiflexion weakness.

How to identify it in the video

high-stepping gait due to foot drop. Watch for common peroneal nerve palsy, L5 radiculopathy, dorsiflexion weakness. Video cues for Steppage gait / foot drop: posture and trunk position, step height and stride length, base of support, arm swing symmetry, rhythm, turning, and freezing or imbalance.

Why it happens

The mechanism of Steppage gait / foot drop starts with common peroneal nerve palsy and is expressed clinically through L5 radiculopathy. Related exam anchors are dorsiflexion weakness. In Steppage gait / foot drop, common peroneal nerve palsy drives the walking pattern and L5 radiculopathy explains the visible step, posture, or balance abnormality.

Associated conditions

- common peroneal nerve palsy
- L5 radiculopathy
- dorsiflexion weakness
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant
- traumatic injury
- sports injury

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
hemiplegic gait	Spastic circumduction of one leg with ipsilateral upper-limb flexor posturing.	Extensor plantar response, and stroke imaging clues.
sensory ataxia	Stomping gait that worsens with eye closure; Romberg is classically positive.	Cerebellar signs may be absent.
cerebellar gait	Broad-based, staggering gait with truncal sway, dysmetria, nystagmus, and worse turning.	Look for nystagmus and truncal sway.

Clinical examination points

- Steppage gait / foot drop: Observe posture, base, stride length, arm swing, turning, balance, and need for support.
- Steppage gait / foot drop: Test power, tone, reflexes, plantar response, sensation, coordination, and Romberg.

NEET PG exam angle

Steppage gait / foot drop is usually tested through common peroneal nerve palsy; L5 radiculopathy; dorsiflexion weakness. Separate it from hemiplegic gait, sensory ataxia, cerebellar gait.

Management relevance

Look for peroneal compression or L5 radiculopathy; ankle-foot orthosis and physiotherapy reduce falls.

Common traps

- Do not confuse with hemiplegic gait: Spastic circumduction of one leg with ipsilateral upper-limb flexor posturing.
- Do not confuse with sensory ataxia: Stomping gait that worsens with eye closure; Romberg is classically positive.
- Do not confuse with cerebellar gait: Broad-based, staggering gait with truncal sway, dysmetria, nystagmus, and worse turning.
- Steppage gait / foot drop needs both common peroneal nerve palsy and L5 radiculopathy; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Steppage gait / foot drop.
- Subject: Medicine / Neurology / Orthopedics.
- Key clue: common peroneal nerve palsy.
- Key clue: L5 radiculopathy.
- Key clue: dorsiflexion weakness.
- Closest mimics: hemiplegic gait, sensory ataxia, cerebellar gait.
- Video segment: 00:00-00:35.
- One-line recall: Steppage gait / foot drop points to common peroneal nerve palsy.

Source credit: Right Foot Drop in Ambulating Patient - NEJM Group.

Topic 05 - Waddling gait

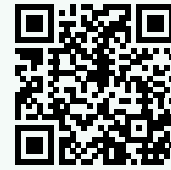
Subject: Medicine / Pediatrics / Orthopedics

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Waddling gait in Myopathy; Department of Medicine; DMIMSU - Clinical Snippets-By Dr. Sourya Acharya-DMIHER



What it is

Waddling gait: waddling gait from proximal muscle weakness. High-yield anchors: proximal myopathy; Duchenne muscular dystrophy; pelvic girdle weakness.

How to identify it in the video

waddling gait from proximal muscle weakness. Watch for proximal myopathy, Duchenne muscular dystrophy, pelvic girdle weakness. Video cues for Waddling gait: posture and trunk position, step height and stride length, base of support, arm swing symmetry, rhythm, turning, and freezing or imbalance.

Why it happens

The mechanism of Waddling gait starts with proximal myopathy and is expressed clinically through Duchenne muscular dystrophy. Related exam anchors are pelvic girdle weakness. In Waddling gait, proximal myopathy drives the walking pattern and Duchenne muscular dystrophy explains the visible step, posture, or balance abnormality.

Associated conditions

- proximal myopathy
- Duchenne muscular dystrophy
- pelvic girdle weakness
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant
- developmental age-related normal variant
- congenital or inherited disorder

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
Trendelenburg gait	Single-leg stance shows pelvic drop on the unsupported side.	Test hip abductors.
hip disease	Painful hip rotation and groin pain point to hip pathology.	Confirm with pelvis/hip X-ray.
cerebellar gait	Broad-based, staggering gait with truncal sway, dysmetria, nystagmus, and worse turning.	Look for nystagmus and truncal sway.

Clinical examination points

- Waddling gait: Observe posture, base, stride length, arm swing, turning, balance, and need for support.
- Waddling gait: Test power, tone, reflexes, plantar response, sensation, coordination, and Romberg.

NEET PG exam angle

Waddling gait is usually tested through proximal myopathy; Duchenne muscular dystrophy; pelvic girdle weakness. Separate it from Trendelenburg gait, hip disease, cerebellar gait.

Management relevance

Clinical action after recognizing Waddling gait: confirm proximal myopathy, look for complications, and choose treatment according to the underlying diagnosis.

Common traps

- Do not confuse with Trendelenburg gait: Single-leg stance shows pelvic drop on the unsupported side.
- Do not confuse with hip disease: Painful hip rotation and groin pain point to hip pathology.
- Do not confuse with cerebellar gait: Broad-based, staggering gait with truncal sway, dysmetria, nystagmus, and worse turning.
- Waddling gait needs both proximal myopathy and Duchenne muscular dystrophy; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Waddling gait.
- Subject: Medicine / Pediatrics / Orthopedics.
- Key clue: proximal myopathy.
- Key clue: Duchenne muscular dystrophy.
- Key clue: pelvic girdle weakness.
- Closest mimics: Trendelenburg gait, hip disease, cerebellar gait.
- Video segment: 00:00-01:00.
- One-line recall: Waddling gait points to proximal myopathy.

Source credit: Waddling gait in Myopathy; Department of Medicine; DMIMSU - Clinical Snippets-By Dr. Sourya Acharya-DMIHER.

Topic 06 - Trendelenburg gait / Trendelenburg test

Subject: Orthopedics

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Trendelenburg Gait; abnormal walking pattern caused by weakness of the hip abductor muscles - Abuja Orthopedics



What it is

Trendelenburg gait / Trendelenburg test: direct Trendelenburg sign/test or hip abductor weakness. High-yield anchors: gluteus medius weakness; superior gluteal nerve; hip abductor mechanism.

How to identify it in the video

direct Trendelenburg sign/test or hip abductor weakness. Watch for gluteus medius weakness, superior gluteal nerve, hip abductor mechanism. Video cues for Trendelenburg gait / Trendelenburg test: posture and trunk position, step height and stride length, base of support, arm swing symmetry, rhythm, turning, and freezing or imbalance.

Why it happens

The mechanism of Trendelenburg gait / Trendelenburg test starts with gluteus medius weakness and is expressed clinically through superior gluteal nerve. Related exam anchors are hip abductor mechanism. In Trendelenburg gait / Trendelenburg test, gluteus medius weakness drives the walking pattern and superior gluteal nerve explains the visible step, posture, or balance abnormality.

Associated conditions

- gluteus medius weakness
- superior gluteal nerve
- hip abductor mechanism
- traumatic injury
- sports injury
- chronic degenerative disease

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
waddling gait	Side-to-side trunk sway from bilateral proximal pelvic-girdle weakness.	Calf hypertrophy, CK, and myopathy history.
antalgic gait	Short stance phase caused by pain; the patient avoids weight bearing on the painful side.	Examine hip, knee, ankle, spine, and weight-bearing X-ray when needed.
hip osteoarthritis	Restricted internal rotation with groin pain and osteophytes/joint-space narrowing on X-ray.	Use the gluteus medius weakness endpoint to decide whether hip osteoarthritis explains the clip.

Clinical examination points

- Trendelenburg gait / Trendelenburg test: Observe posture, base, stride length, arm swing, turning, balance, and need for support.
- Trendelenburg gait / Trendelenburg test: Test power, tone, reflexes, plantar response, sensation, coordination, and Romberg.
- Trendelenburg gait / Trendelenburg test: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Trendelenburg gait / Trendelenburg test: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Trendelenburg gait / Trendelenburg test is usually tested through gluteus medius weakness; superior gluteal nerve; hip abductor mechanism. Separate it from waddling gait, antalgic gait, hip osteoarthritis.

Management relevance

Use the demonstrated technique only for the correct indication; the management decision hinges on gluteus medius weakness, patient safety, and the endpoint shown in the video.

Common traps

- Do not confuse with waddling gait: Side-to-side trunk sway from bilateral proximal pelvic-girdle weakness.
- Do not confuse with antalgic gait: Short stance phase caused by pain; the patient avoids weight bearing on the painful side.
- Do not confuse with hip osteoarthritis: Restricted internal rotation with groin pain and osteophytes/joint-space narrowing on X-ray.
- Trendelenburg gait / Trendelenburg test needs both gluteus medius weakness and superior gluteal nerve; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Trendelenburg gait / Trendelenburg test.
- Subject: Orthopedics.
- Key clue: gluteus medius weakness.
- Key clue: superior gluteal nerve.
- Key clue: hip abductor mechanism.
- Closest mimics: waddling gait, antalgic gait, hip osteoarthritis.
- Video segment: 00:00-01:00.
- One-line recall: Trendelenburg gait / Trendelenburg test points to gluteus medius weakness.

Source credit: Trendelenburg Gait; abnormal walking pattern caused by weakness of the hip abductor muscles - Abuja Orthopedics.

Topic 07 - Gower sign

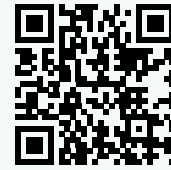
Subject: Pediatrics / Medicine

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:54

Source credit: Gower's sign in a child with DMD - Medchrome



What it is

Gower sign: child rising from floor using hands to climb up thighs. High-yield anchors: Duchenne muscular dystrophy; proximal muscle weakness; dystrophin; X-linked inheritance.

How to identify it in the video

child rising from floor using hands to climb up thighs. Watch for Duchenne muscular dystrophy, proximal muscle weakness, dystrophin, X-linked inheritance. Video cues for Gower sign: the visible endpoint of Gower sign, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Gower sign starts with Duchenne muscular dystrophy and is expressed clinically through proximal muscle weakness. Related exam anchors are dystrophin, X-linked inheritance.

Associated conditions

- Duchenne muscular dystrophy
- proximal muscle weakness
- dystrophin
- X-linked inheritance
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant
- developmental age-related normal variant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
normal toddler climbing	Milestones, symmetry.	Playfulness, and absence of proximal weakness separate normal behavior.
hip weakness	Manual hip abduction/flexion strength and pelvis drop localize weakness around the hip.	Use the Duchenne muscular dystrophy endpoint to decide whether hip weakness explains the clip.
myopathy	Symmetric proximal weakness, raised CK.	Myopathic EMG, and muscle/genetic testing are clues.

Clinical examination points

- Gower sign: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Gower sign: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Gower sign is usually tested through Duchenne muscular dystrophy; proximal muscle weakness; dystrophin. Separate it from normal toddler climbing, hip weakness, myopathy.

Management relevance

Refer for neuromuscular evaluation, CK testing, genetic confirmation, steroids where appropriate, and cardiac/respiratory follow-up.

Common traps

- Do not confuse with normal toddler climbing: Milestones, symmetry.
- Do not confuse with hip weakness: Manual hip abduction/flexion strength and pelvis drop localize weakness around the hip.
- Do not confuse with myopathy: Symmetric proximal weakness, raised CK.
- Gower sign needs both Duchenne muscular dystrophy and proximal muscle weakness; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Gower sign.
- Subject: Pediatrics / Medicine.
- Key clue: Duchenne muscular dystrophy.
- Key clue: proximal muscle weakness.
- Key clue: dystrophin.
- Closest mimics: normal toddler climbing, hip weakness, myopathy.
- Video segment: 00:00-00:54.
- One-line recall: Gower sign points to Duchenne muscular dystrophy.

Source credit: Gower's sign in a child with DMD - Medchrome.

Topic 08 - Resting tremor vs action tremor

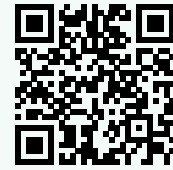
Subject: Medicine / Neurology

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:15

Source credit: Pill Rolling tremors in Parkinson's Disease - Clinical Snippets-By Dr. Sourya Acharya-DMIHER



What it is

Resting tremor vs action tremor: direct demonstration of resting tremor or comparison with action tremor. High-yield anchors: Parkinson resting tremor; essential tremor; cerebellar tremor.

How to identify it in the video

direct demonstration of resting tremor or comparison with action tremor. Watch for Parkinson resting tremor, essential tremor, cerebellar tremor. Video cues for Resting tremor vs action tremor: the visible endpoint of Resting tremor vs action tremor, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Resting tremor vs action tremor starts with Parkinson resting tremor and is expressed clinically through essential tremor. Related exam anchors are cerebellar tremor.

Associated conditions

- Parkinson resting tremor
- essential tremor
- cerebellar tremor
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
anxiety tremor	Fine tremor varies with stress and improves with reassurance.	Vitals and thyroid tests are normal.
thyrotoxic tremor	Fine postural tremor plus tachycardia, weight loss.	Goiter, and low TSH.
intention tremor	Finger-nose tremor worsens near the target.	Check dysmetria and cerebellar signs.

Clinical examination points

- Resting tremor vs action tremor: Compare rest, posture, and action; check symmetry, frequency, suppressibility, and associated rigidity or cerebellar signs.

NEET PG exam angle

Resting tremor vs action tremor is usually tested through Parkinson resting tremor; essential tremor; cerebellar tremor. Separate it from anxiety tremor, thyrotoxic tremor, intention tremor.

Management relevance

Treat Parkinson disease with dopaminergic therapy such as levodopa when indicated; review dopamine-blocking drugs.

Common traps

- Do not confuse with anxiety tremor: Fine tremor varies with stress and improves with reassurance.
- Do not confuse with thyrotoxic tremor: Fine postural tremor plus tachycardia, weight loss.
- Do not confuse with intention tremor: Finger-nose tremor worsens near the target.
- Resting tremor vs action tremor needs both Parkinson resting tremor and essential tremor; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Resting tremor vs action tremor.
- Subject: Medicine / Neurology.
- Key clue: Parkinson resting tremor.
- Key clue: essential tremor.
- Key clue: cerebellar tremor.
- Closest mimics: anxiety tremor, thyrotoxic tremor, intention tremor.
- Video segment: 00:00-00:15.
- One-line recall: Resting tremor vs action tremor points to Parkinson resting tremor.

Source credit: Pill Rolling tremors in Parkinson's Disease - Clinical Snippets-By Dr. Sourya Acharya-DMIHER.

Topic 09 - Intention tremor / finger-nose test

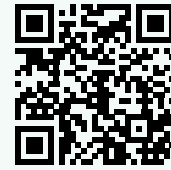
Subject: Medicine / Neurology

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:27

Source credit: Cerebellar intention tremors on finger nose finger test - Dr Gauhar Mahmood Azeem



What it is

Intention tremor / finger-nose test: finger-nose test showing dysmetria or intention tremor. High-yield anchors: cerebellar lesion; dysmetria; intention tremor.

How to identify it in the video

finger-nose test showing dysmetria or intention tremor. Watch for cerebellar lesion, dysmetria, intention tremor. Video cues for Intention tremor / finger-nose test: the visible endpoint of Intention tremor / finger-nose test, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Intention tremor / finger-nose test starts with cerebellar lesion and is expressed clinically through dysmetria. Related exam anchors are intention tremor.

Associated conditions

- cerebellar lesion
- dysmetria
- intention tremor
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
essential tremor	Postural/action tremor improves with alcohol.	Family history and no bradykinesia help.
Parkinson tremor	Resting pill-rolling tremor decreases with action.	Look for rigidity and bradykinesia.
sensory ataxia	Stomping gait that worsens with eye closure; Romberg is classically positive.	Cerebellar signs may be absent.

Clinical examination points

- Intention tremor / finger-nose test: Compare rest, posture, and action; check symmetry, frequency, suppressibility, and associated rigidity or cerebellar signs.
- Intention tremor / finger-nose test: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Intention tremor / finger-nose test: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Intention tremor / finger-nose test is usually tested through cerebellar lesion; dysmetria; intention tremor. Separate it from essential tremor, Parkinson tremor, sensory ataxia.

Management relevance

Treat the cause, exclude stroke/toxin/metabolic causes, and use fall-prevention rehabilitation.

Common traps

- Do not confuse with essential tremor: Postural/action tremor improves with alcohol.
- Do not confuse with Parkinson tremor: Resting pill-rolling tremor decreases with action.
- Do not confuse with sensory ataxia: Stomping gait that worsens with eye closure; Romberg is classically positive.
- Intention tremor / finger-nose test needs both cerebellar lesion and dysmetria; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Intention tremor / finger-nose test.
- Subject: Medicine / Neurology.
- Key clue: cerebellar lesion.
- Key clue: dysmetria.
- Key clue: intention tremor.
- Closest mimics: essential tremor, Parkinson tremor, sensory ataxia.
- Video segment: 00:00-00:27.
- One-line recall: Intention tremor / finger-nose test points to cerebellar lesion.

Source credit: Cerebellar intention tremors on finger nose finger test - Dr Gauhar Mahmood Azeem.

Topic 10 - Asterixis

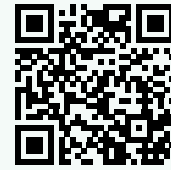
Subject: Medicine

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Demonstration of flapping tremors, Dept. Of Medicine, JNMC - Clinical Snippets-By Dr. Sourya Acharya-DMIHER



What it is

Asterixis: flapping tremor on wrist extension. High-yield anchors: hepatic encephalopathy; uremia; CO₂ retention; metabolic encephalopathy.

How to identify it in the video

flapping tremor on wrist extension. Watch for hepatic encephalopathy, uremia, CO₂ retention, metabolic encephalopathy. Video cues for Asterixis: the visible endpoint of Asterixis, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Asterixis starts with hepatic encephalopathy and is expressed clinically through uremia. Related exam anchors are CO₂ retention, metabolic encephalopathy.

Associated conditions

- hepatic encephalopathy
- uremia
- CO₂ retention
- metabolic encephalopathy
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
tremor	Classify as rest, postural.	Action, or intention before naming the disorder.
chorea	Random flowing non-rhythmic movements.	Ask about Huntington disease, Sydenham chorea, pregnancy, drugs.
myoclonus	Shock-like jerks suggest metabolic, epileptic, or post-hypoxic myoclonus.	EEG helps if epileptic.

Clinical examination points

- For Asterixis, document the visible abnormality before naming the diagnosis.
- For Asterixis, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Asterixis is usually tested through hepatic encephalopathy; uremia; CO₂ retention. Separate it from tremor, chorea, myoclonus.

Management relevance

Treat the metabolic driver: lactulose/rifaximin for hepatic encephalopathy, dialysis for uremia, ventilatory support for CO₂ retention.

Common traps

- Do not confuse with tremor: Classify as rest, postural.
- Do not confuse with chorea: Random flowing non-rhythmic movements.
- Do not confuse with myoclonus: Shock-like jerks suggest metabolic, epileptic, or post-hypoxic myoclonus.
- Asterixis needs both hepatic encephalopathy and uremia; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Asterixis.
- Subject: Medicine.
- Key clue: hepatic encephalopathy.
- Key clue: uremia.
- Key clue: CO₂ retention.
- Closest mimics: tremor, chorea, myoclonus.
- Video segment: 00:00-01:00.
- One-line recall: Asterixis points to hepatic encephalopathy.

Source credit: Demonstration of flapping tremors, Dept. Of Medicine, JNMC - Clinical Snippets-By Dr. Sourya Acharya-DMIHER.

Topic 11 - Chorea

Subject: Medicine / Neurology

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:21

Source credit: Huntington's Disease chorea - automatic analysis - Machine Medicine Technologies



What it is

Chorea: direct choreiform movements. High-yield anchors: Huntington disease; caudate nucleus; Sydenham chorea; rheumatic fever.

How to identify it in the video

direct choreiform movements. Watch for Huntington disease, caudate nucleus, Sydenham chorea, rheumatic fever. Video cues for Chorea: the visible endpoint of Chorea, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Chorea starts with Huntington disease and is expressed clinically through caudate nucleus. Related exam anchors are Sydenham chorea, rheumatic fever.

Associated conditions

- Huntington disease
- caudate nucleus
- Sydenham chorea
- rheumatic fever
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
dystonia	Sustained twisting posture suggests dystonia.	Check dopamine-blocker exposure and anticholinergic response.
athetosis	Slow writhing distal movements point to basal ganglia/perinatal injury rather than chorea.	Use the Huntington disease endpoint to decide whether athetosis explains the clip.
tardive dyskinesia	Chronic dopamine-blocker exposure plus orobuccolingual movements.	Use AIMS scale.
tics	Premonitory urge.	Partial suppressibility, and waxing-waning course suggest tic disorder.

Clinical examination points

- For Chorea, document the visible abnormality before naming the diagnosis.
- For Chorea, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Chorea is usually tested through Huntington disease; caudate nucleus; Sydenham chorea. Separate it from dystonia, athetosis, tardive dyskinesia.

Management relevance

Clinical action after recognizing Chorea: confirm Huntington disease, look for complications, and choose treatment according to the underlying diagnosis.

Common traps

- Do not confuse with dystonia: Sustained twisting posture suggests dystonia.
- Do not confuse with athetosis: Slow writhing distal movements point to basal ganglia/perinatal injury rather than chorea.
- Do not confuse with tardive dyskinesia: Chronic dopamine-blocker exposure plus orobuccolingual movements.
- Chorea needs both Huntington disease and caudate nucleus; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Chorea.
- Subject: Medicine / Neurology.
- Key clue: Huntington disease.
- Key clue: caudate nucleus.
- Key clue: Sydenham chorea.
- Closest mimics: dystonia, athetosis, tardive dyskinesia, tics.
- Video segment: 00:00-00:21.
- One-line recall: Chorea points to Huntington disease.

Source credit: Huntington's Disease chorea - automatic analysis - Machine Medicine Technologies.

Topic 12 - Dystonia / oculogyric crisis

Subject: Medicine / Psychiatry / Pharmacology

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Oculogyric Crisis - JAMA Network



What it is

Dystonia / oculogyric crisis: acute dystonia or oculogyric crisis demonstration. High-yield anchors: antipsychotic adverse effect; dopamine blockade; anticholinergic treatment.

How to identify it in the video

acute dystonia or oculogyric crisis demonstration. Watch for antipsychotic adverse effect, dopamine blockade, anticholinergic treatment. Video cues for Dystonia / oculogyric crisis: the visible endpoint of Dystonia / oculogyric crisis, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Dystonia / oculogyric crisis starts with antipsychotic adverse effect and is expressed clinically through dopamine blockade. Related exam anchors are anticholinergic treatment.

Associated conditions

- antipsychotic adverse effect
- dopamine blockade
- anticholinergic treatment
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
seizure	Stereotyped event, impaired awareness.	Postictal state, tongue bite, and EEG support seizure.
tardive dyskinesia	Chronic dopamine-blocker exposure plus orobuccolingual movements.	Use AIMS scale.
chorea	Random flowing non-rhythmic movements.	Ask about Huntington disease, Sydenham chorea, pregnancy, drugs.
tetany	Carpopedal spasm with low calcium.	Check Chvostek/Trousseau and serum calcium/magnesium.

Clinical examination points

- For Dystonia / oculogyric crisis, document the visible abnormality before naming the diagnosis.
- For Dystonia / oculogyric crisis, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Dystonia / oculogyric crisis is usually tested through antipsychotic adverse effect; dopamine blockade; anticholinergic treatment. Separate it from seizure, tardive dyskinesia, chorea.

Management relevance

Acute dystonia from dopamine blockade responds to benztropine, trihexyphenidyl, or promethazine/diphenhydramine.

Common traps

- Do not confuse with seizure: Stereotyped event, impaired awareness.
- Do not confuse with tardive dyskinesia: Chronic dopamine-blocker exposure plus orobuccolingual movements.
- Do not confuse with chorea: Random flowing non-rhythmic movements.
- Dystonia / oculogyric crisis needs both antipsychotic adverse effect and dopamine blockade; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Dystonia / oculogyric crisis.
- Subject: Medicine / Psychiatry / Pharmacology.
- Key clue: antipsychotic adverse effect.
- Key clue: dopamine blockade.
- Key clue: anticholinergic treatment.
- Closest mimics: seizure, tardive dyskinesia, chorea, tetany.
- Video segment: 00:00-01:00.
- One-line recall: Dystonia / oculogyric crisis points to antipsychotic adverse effect.

Source credit: Oculogyric Crisis - JAMA Network.

Topic 13 - Seizure semiology

Subject: Medicine / Pediatrics

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:25

Source credit: Example of a Tonic Clonic Seizure - Epilepsy Toronto



What it is

Seizure semiology: short educational clip showing seizure type. High-yield anchors: focal seizure; generalized seizure; absence seizure; GTCS; EEG correlation.

How to identify it in the video

short educational clip showing seizure type. Watch for focal seizure, generalized seizure, absence seizure, GTCS. Video cues for Seizure semiology: the visible endpoint of Seizure semiology, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Seizure semiology starts with focal seizure and is expressed clinically through generalized seizure. Related exam anchors are absence seizure, GTCS, EEG correlation.

Associated conditions

- focal seizure
- generalized seizure
- absence seizure
- GTCS
- EEG correlation
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
syncope	Prodrome, pallor.	Brief loss of tone, rapid recovery, ECG, and orthostatic vitals decide syncope.
psychogenic nonepileptic seizure	Stereotyped event, impaired awareness.	Postictal state, tongue bite, and EEG support seizure.
movement disorder	Describe rhythm, suppressibility.	Distribution, activation state, and medication exposure.

Clinical examination points

- For Seizure semiology, document the visible abnormality before naming the diagnosis.
- For Seizure semiology, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Seizure semiology is usually tested through focal seizure; generalized seizure; absence seizure. Separate it from syncope, psychogenic nonepileptic seizure, movement disorder.

Management relevance

Stabilize airway, glucose, electrolytes, and use benzodiazepines for ongoing convulsive seizure.

Common traps

- Do not confuse with syncope: Prodrome, pallor.
- Do not confuse with psychogenic nonepileptic seizure: Stereotyped event, impaired awareness.
- Do not confuse with movement disorder: Describe rhythm, suppressibility.
- Seizure semiology needs both focal seizure and generalized seizure; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Seizure semiology.
- Subject: Medicine / Pediatrics.
- Key clue: focal seizure.
- Key clue: generalized seizure.
- Key clue: absence seizure.
- Closest mimics: syncope, psychogenic nonepileptic seizure, movement disorder.
- Video segment: 00:00-00:25.
- One-line recall: Seizure semiology points to focal seizure.

Source credit: Example of a Tonic Clonic Seizure - Epilepsy Toronto.

Topic 14 - Infantile spasms

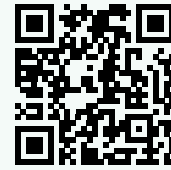
Subject: Pediatrics

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:21

Source credit: Infantile Spasms - megand1515



What it is

Infantile spasms: Salaam attacks or infantile spasm clip. High-yield anchors: West syndrome; hypsarrhythmia; ACTH; vigabatrin.

How to identify it in the video

Salaam attacks or infantile spasm clip. Watch for West syndrome, hypsarrhythmia, ACTH, vigabatrin. Video cues for Infantile spasms: the visible endpoint of Infantile spasms, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Infantile spasms starts with West syndrome and is expressed clinically through hypsarrhythmia. Related exam anchors are ACTH, vigabatrin.

Associated conditions

- West syndrome
- hypsarrhythmia
- ACTH
- vigabatrin
- developmental age-related normal variant
- congenital or inherited disorder
- acute infection or emergency presentation

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
benign myoclonus	Shock-like jerks suggest metabolic, epileptic, or post-hypoxic myoclonus.	EEG helps if epileptic.
startle response	Stimulus-linked Moro/startle response lacks clusters.	Regression, or hypsarrhythmia.
seizures	Stereotyped event, impaired awareness.	Postictal state, tongue bite, and EEG support seizure.

Clinical examination points

- For Infantile spasms, document the visible abnormality before naming the diagnosis.
- For Infantile spasms, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Infantile spasms is usually tested through West syndrome; hypsarrhythmia; ACTH. Separate it from benign myoclonus, startle response, seizures.

Management relevance

Urgent pediatric neurology care; ACTH or vigabatrin are high-yield treatments depending on cause.

Common traps

- Do not confuse with benign myoclonus: Shock-like jerks suggest metabolic, epileptic, or post-hypoxic myoclonus.
- Do not confuse with startle response: Stimulus-linked Moro/startle response lacks clusters.
- Do not confuse with seizures: Stereotyped event, impaired awareness.
- Infantile spasms needs both West syndrome and hypsarrhythmia; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Infantile spasms.
- Subject: Pediatrics.
- Key clue: West syndrome.
- Key clue: hypsarrhythmia.
- Key clue: ACTH.
- Closest mimics: benign myoclonus, startle response, seizures.
- Video segment: 00:00-00:21.
- One-line recall: Infantile spasms points to West syndrome.

Source credit: Infantile Spasms - megand1515.

Topic 15 - Nystagmus

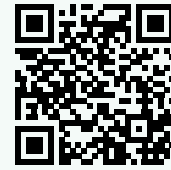
Subject: Medicine / Ophthalmology / ENT

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:20

Source credit: Nystagmus #clinical signs #Ophthalmology #shorts - THE WHITE ARMY



What it is

Nystagmus: direct eye movement video showing nystagmus. High-yield anchors: central vs peripheral nystagmus; vestibular cause; cerebellar cause.

How to identify it in the video

direct eye movement video showing nystagmus. Watch for central vs peripheral nystagmus, vestibular cause, cerebellar cause. Video cues for Nystagmus: pupil size and symmetry, direction of eye deviation or movement limitation, ptosis or lid asymmetry, nystagmus direction, light reflex or staining pattern.

Why it happens

The mechanism of Nystagmus starts with central vs peripheral nystagmus and is expressed clinically through vestibular cause. Related exam anchors are cerebellar cause. In Nystagmus, the bedside pattern localizes the relevant afferent, efferent, ocular motor, sympathetic, vestibular, or cerebellar pathway.

Associated conditions

- central vs peripheral nystagmus
- vestibular cause
- cerebellar cause
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
normal end-gaze nystagmus	Appears only at extreme gaze.	Is symmetric, and lacks vertigo or neurologic signs.
ocular flutter	Pure horizontal back-to-back saccades without intersaccadic interval.	Use the central vs peripheral nystagmus endpoint to decide whether ocular flutter explains the clip.
opsoclonus	Chaotic multidirectional saccades.	Think neuroblastoma or post-infectious/autoimmune causes.

Clinical examination points

- Nystagmus: Check pupils, ocular alignment, extraocular movements, ptosis, diplopia direction, and visual fields.
- Nystagmus: Localize afferent and efferent pathways before naming the lesion.

NEET PG exam angle

Nystagmus is usually tested through central vs peripheral nystagmus; vestibular cause; cerebellar cause. Separate it from normal end-gaze nystagmus, ocular flutter, opsoclonus.

Management relevance

Peripheral causes may need vestibular treatment; central nystagmus needs urgent neurologic evaluation.

Common traps

- Do not confuse with normal end-gaze nystagmus: Appears only at extreme gaze.
- Do not confuse with ocular flutter: Pure horizontal back-to-back saccades without intersaccadic interval.
- Do not confuse with opsoclonus: Chaotic multidirectional saccades.
- Nystagmus needs both central vs peripheral nystagmus and vestibular cause; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Nystagmus.
- Subject: Medicine / Ophthalmology / ENT.
- Key clue: central vs peripheral nystagmus.
- Key clue: vestibular cause.
- Key clue: cerebellar cause.
- Closest mimics: normal end-gaze nystagmus, ocular flutter, opsoclonus.
- Video segment: 00:00-00:20.
- One-line recall: Nystagmus points to central vs peripheral nystagmus.

Source credit: Nystagmus #clinical signs #Ophthalmology #shorts - THE WHITE ARMY.

Topic 16 - Third nerve palsy

Subject: Ophthalmology / Neurology

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:36

Source credit: 3rd Nerve Palsy (Oculomotor Nerve): Clinical findings simplified in 35 seconds - Dr. Aman Arora - Arora Medical Education



What it is

Third nerve palsy: ptosis and down-and-out eye movement. High-yield anchors: oculomotor nerve palsy; pupil involvement; posterior communicating artery aneurysm.

How to identify it in the video

ptosis and down-and-out eye movement. Watch for oculomotor nerve palsy, pupil involvement, posterior communicating artery aneurysm. Video cues for Third nerve palsy: pupil size and symmetry, direction of eye deviation or movement limitation, ptosis or lid asymmetry, nystagmus direction, light reflex or staining pattern.

Why it happens

The mechanism of Third nerve palsy starts with oculomotor nerve palsy and is expressed clinically through pupil involvement. Related exam anchors are posterior communicating artery aneurysm. In Third nerve palsy, the bedside pattern localizes the relevant afferent, efferent, ocular motor, sympathetic, vestibular, or cerebellar pathway.

Associated conditions

- oculomotor nerve palsy
- pupil involvement
- posterior communicating artery aneurysm
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
Horner syndrome	Mild ptosis, miosis, and possible anhidrosis from sympathetic pathway interruption.	Test anisocoria in dim light and consider carotid/Pancoast causes.
myasthenia gravis	Fatigable, variable weakness; improves with rest or ice-pack test.	Cogan lid twitch, ice-pack test, AChR antibodies.
fourth/sixth nerve palsy	Map diplopia direction.	Sixth limits abduction, fourth worsens on down-and-in gaze/head tilt.

Clinical examination points

- Third nerve palsy: Check pupils, ocular alignment, extraocular movements, ptosis, diplopia direction, and visual fields.
- Third nerve palsy: Localize afferent and efferent pathways before naming the lesion.

NEET PG exam angle

Third nerve palsy is usually tested through oculomotor nerve palsy; pupil involvement; posterior communicating artery aneurysm. Separate it from Horner syndrome, myasthenia gravis, fourth/sixth nerve palsy.

Management relevance

Painful pupil-involving third nerve palsy needs urgent aneurysm imaging.

Common traps

- Do not confuse with Horner syndrome: Mild ptosis, miosis, and possible anhidrosis from sympathetic pathway interruption.
- Do not confuse with myasthenia gravis: Fatigable, variable weakness; improves with rest or ice-pack test.
- Do not confuse with fourth/sixth nerve palsy: Map diplopia direction.
- Third nerve palsy needs both oculomotor nerve palsy and pupil involvement; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Third nerve palsy.
- Subject: Ophthalmology / Neurology.
- Key clue: oculomotor nerve palsy.
- Key clue: pupil involvement.
- Key clue: posterior communicating artery aneurysm.
- Closest mimics: Horner syndrome, myasthenia gravis, fourth/sixth nerve palsy.
- Video segment: 00:00-00:36.
- One-line recall: Third nerve palsy points to oculomotor nerve palsy.

Source credit: 3rd Nerve Palsy (Oculomotor Nerve): Clinical findings simplified in 35 seconds - Dr. Aman Arora - Arora Medical Education.

Topic 17 - Sixth nerve palsy

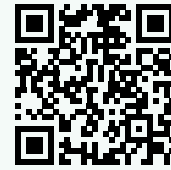
Subject: Ophthalmology / Neurology

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:25

Source credit: Sixth nerve palsy | video recognition series ophthalmology case-20 - EYE GUY



What it is

Sixth nerve palsy: abduction defect due to lateral rectus palsy. High-yield anchors: abducens nerve; lateral rectus; raised ICP; cavernous sinus.

How to identify it in the video

abduction defect due to lateral rectus palsy. Watch for abducens nerve, lateral rectus, raised ICP, cavernous sinus. Video cues for Sixth nerve palsy: pupil size and symmetry, direction of eye deviation or movement limitation, ptosis or lid asymmetry, nystagmus direction, light reflex or staining pattern.

Why it happens

The mechanism of Sixth nerve palsy starts with abducens nerve and is expressed clinically through lateral rectus. Related exam anchors are raised ICP, cavernous sinus. In Sixth nerve palsy, the bedside pattern localizes the relevant afferent, efferent, ocular motor, sympathetic, vestibular, or cerebellar pathway.

Associated conditions

- abducens nerve
- lateral rectus
- raised ICP
- cavernous sinus
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
third nerve palsy	Ptosis with down-and-out eye; pupil involvement suggests compressive aneurysm until proven otherwise.	Down-and-out eye, and pupil involvement require urgent aneurysm imaging.
internuclear ophthalmoplegia	Adduction failure with abducting nystagmus.	Convergence may be preserved.
restrictive strabismus	Forced-duction restriction or thyroid eye disease signs suggest mechanical restriction.	Use the abducens nerve endpoint to decide whether restrictive strabismus explains the clip.

Clinical examination points

- Sixth nerve palsy: Check pupils, ocular alignment, extraocular movements, ptosis, diplopia direction, and visual fields.
- Sixth nerve palsy: Localize afferent and efferent pathways before naming the lesion.

NEET PG exam angle

Sixth nerve palsy is usually tested through abducens nerve; lateral rectus; raised ICP. Separate it from third nerve palsy, internuclear ophthalmoplegia, restrictive strabismus.

Management relevance

Evaluate raised ICP, diabetes, trauma, and cavernous sinus causes; protect vision and refer.

Common traps

- Do not confuse with third nerve palsy: Ptosis with down-and-out eye; pupil involvement suggests compressive aneurysm until proven otherwise.
- Do not confuse with internuclear ophthalmoplegia: Adduction failure with abducting nystagmus.
- Do not confuse with restrictive strabismus: Forced-duction restriction or thyroid eye disease signs suggest mechanical restriction.
- Sixth nerve palsy needs both abducens nerve and lateral rectus; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Sixth nerve palsy.
- Subject: Ophthalmology / Neurology.
- Key clue: abducens nerve.
- Key clue: lateral rectus.
- Key clue: raised ICP.
- Closest mimics: third nerve palsy, internuclear ophthalmoplegia, restrictive strabismus.
- Video segment: 00:00-00:25.
- One-line recall: Sixth nerve palsy points to abducens nerve.

Source credit: Sixth nerve palsy | video recognition series ophthalmology case-20 - EYE GUY.

Topic 18 - Fourth nerve palsy

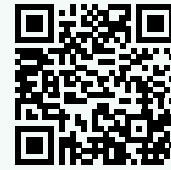
Subject: Ophthalmology / Neurology

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Fourth (4th) Cranial Nerve Palsy - U-M Kellogg Eye Center in Ann Arbor



What it is

Fourth nerve palsy: trochlear nerve palsy or superior oblique palsy demonstration. High-yield anchors: vertical diplopia; head tilt; superior oblique.

How to identify it in the video

trochlear nerve palsy or superior oblique palsy demonstration. Watch for vertical diplopia, head tilt, superior oblique. Video cues for Fourth nerve palsy: pupil size and symmetry, direction of eye deviation or movement limitation, ptosis or lid asymmetry, nystagmus direction, light reflex or staining pattern.

Why it happens

The mechanism of Fourth nerve palsy starts with vertical diplopia and is expressed clinically through head tilt. Related exam anchors are superior oblique. In Fourth nerve palsy, the bedside pattern localizes the relevant afferent, efferent, ocular motor, sympathetic, vestibular, or cerebellar pathway.

Associated conditions

- vertical diplopia
- head tilt
- superior oblique
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
third nerve palsy	Ptosis with down-and-out eye; pupil involvement suggests compressive aneurysm until proven otherwise.	Down-and-out eye, and pupil involvement require urgent aneurysm imaging.
skew deviation	Vertical misalignment with vestibular/brainstem signs.	Upright-supine test may help.
thyroid eye disease	Proptosis, lid retraction.	Restricted elevation, thyroid tests, and orbital imaging.

Clinical examination points

- Fourth nerve palsy: Check pupils, ocular alignment, extraocular movements, ptosis, diplopia direction, and visual fields.
- Fourth nerve palsy: Localize afferent and efferent pathways before naming the lesion.

NEET PG exam angle

Fourth nerve palsy is usually tested through vertical diplopia; head tilt; superior oblique. Separate it from third nerve palsy, skew deviation, thyroid eye disease.

Management relevance

Assess diplopia and head tilt; prism, observation, or surgery depends on cause.

Common traps

- Do not confuse with third nerve palsy: Ptosis with down-and-out eye; pupil involvement suggests compressive aneurysm until proven otherwise.
- Do not confuse with skew deviation: Vertical misalignment with vestibular/brainstem signs.
- Do not confuse with thyroid eye disease: Proptosis, lid retraction.
- Fourth nerve palsy needs both vertical diplopia and head tilt; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Fourth nerve palsy.
- Subject: Ophthalmology / Neurology.
- Key clue: vertical diplopia.
- Key clue: head tilt.
- Key clue: superior oblique.
- Closest mimics: third nerve palsy, skew deviation, thyroid eye disease.
- Video segment: 00:00-01:00.
- One-line recall: Fourth nerve palsy points to vertical diplopia.

Source credit: Fourth (4th) Cranial Nerve Palsy - U-M Kellogg Eye Center in Ann Arbor.

Topic 19 - Cover-uncover test / squint

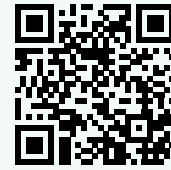
Subject: Ophthalmology

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:20

Source credit: Left eye Esotropia on Cover Uncover test/Squint Examination - Ophthalmology Unveiled



What it is

Cover-uncover test / squint: direct cover-uncover test demonstration. High-yield anchors: tropia; phoria; manifest squint; latent squint.

How to identify it in the video

direct cover-uncover test demonstration. Watch for tropia, phoria, manifest squint, latent squint. Video cues for Cover-uncover test / squint: the visible endpoint of Cover-uncover test / squint, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Cover-uncover test / squint starts with tropia and is expressed clinically through phoria. Related exam anchors are manifest squint, latent squint.

Associated conditions

- tropia
- phoria
- manifest squint
- latent squint

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
Hirschberg test	Corneal light reflex asymmetry estimates deviation.	Useful when cover test is difficult.
alternate cover test	Breaks fusion and measures total tropia plus phoria.	Use the tropia endpoint to decide whether alternate cover test explains the clip.
pseudo-strabismus	Symmetric corneal reflex and normal cover test despite broad epicanthal folds.	Use the tropia endpoint to decide whether pseudo-strabismus explains the clip.

Clinical examination points

- Cover-uncover test / squint: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Cover-uncover test / squint: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Cover-uncover test / squint is usually tested through tropia; phoria; manifest squint. Separate it from Hirschberg test, alternate cover test, pseudo-strabismus.

Management relevance

Use the demonstrated technique only for the correct indication; the management decision hinges on tropia, patient safety, and the endpoint shown in the video.

Common traps

- Do not confuse with Hirschberg test: Corneal light reflex asymmetry estimates deviation.
- Do not confuse with alternate cover test: Breaks fusion and measures total tropia plus phoria.
- Do not confuse with pseudo-strabismus: Symmetric corneal reflex and normal cover test despite broad epicanthal folds.
- Cover-uncover test / squint needs both tropia and phoria; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Cover-uncover test / squint.
- Subject: Ophthalmology.
- Key clue: tropia.
- Key clue: phoria.
- Key clue: manifest squint.
- Closest mimics: Hirschberg test, alternate cover test, pseudo-strabismus.
- Video segment: 00:00-00:20.
- One-line recall: Cover-uncover test / squint points to tropia.

Source credit: Left eye Esotropia on Cover Uncover test/Squint Examination - Ophthalmology Unveiled.

Topic 20 - Relative afferent pupillary defect / swinging flashlight test

Subject: Ophthalmology

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:20

Source credit: RAPD (Relative Afferent Pupillary Defect) Examination video for HKU medical students
s19.wmv - trin0345



What it is

Relative afferent pupillary defect / swinging flashlight test: RAPD or Marcus Gunn pupil demonstration. High-yield anchors: optic nerve lesion; afferent defect; swinging flashlight test.

How to identify it in the video

RAPD or Marcus Gunn pupil demonstration. Watch for optic nerve lesion, afferent defect, swinging flashlight test. Video cues for Relative afferent pupillary defect / swinging flashlight test: pupil size and symmetry, direction of eye deviation or movement limitation, ptosis or lid asymmetry, nystagmus direction, light reflex or staining pattern.

Why it happens

The mechanism of Relative afferent pupillary defect / swinging flashlight test starts with optic nerve lesion and is expressed clinically through afferent defect. Related exam anchors are swinging flashlight test. In Relative afferent pupillary defect / swinging flashlight test, the bedside pattern localizes the relevant afferent, efferent, ocular motor, sympathetic, vestibular, or cerebellar pathway.

Associated conditions

- optic nerve lesion
- afferent defect
- swinging flashlight test

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
anisocoria	Compare bright versus dim light.	Decide parasympathetic versus sympathetic lesion.
third nerve palsy	Ptosis with down-and-out eye; pupil involvement suggests compressive aneurysm until proven otherwise.	Down-and-out eye, and pupil involvement require urgent aneurysm imaging.
pharmacological pupil	Medication exposure.	No near response, and poor pilocarpine response can reveal drug effect.

Clinical examination points

- Relative afferent pupillary defect / swinging flashlight test: Check pupils, ocular alignment, extraocular movements, ptosis, diplopia direction, and visual fields.
- Relative afferent pupillary defect / swinging flashlight test: Localize afferent and efferent pathways before naming the lesion.
- Relative afferent pupillary defect / swinging flashlight test: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Relative afferent pupillary defect / swinging flashlight test: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Relative afferent pupillary defect / swinging flashlight test is usually tested through optic nerve lesion; afferent defect; swinging flashlight test. Separate it from anisocoria, third nerve palsy, pharmacological pupil.

Management relevance

Use the demonstrated technique only for the correct indication; the management decision hinges on optic nerve lesion, patient safety, and the endpoint shown in the video.

Common traps

- Do not confuse with anisocoria: Compare bright versus dim light.
- Do not confuse with third nerve palsy: Ptosis with down-and-out eye; pupil involvement suggests compressive aneurysm until proven otherwise.
- Do not confuse with pharmacological pupil: Medication exposure.
- Relative afferent pupillary defect / swinging flashlight test needs both optic nerve lesion and afferent defect; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Relative afferent pupillary defect / swinging flashlight test.
- Subject: Ophthalmology.
- Key clue: optic nerve lesion.
- Key clue: afferent defect.
- Key clue: swinging flashlight test.
- Closest mimics: anisocoria, third nerve palsy, pharmacological pupil.
- Video segment: 00:00-00:20.
- One-line recall: Relative afferent pupillary defect / swinging flashlight test points to optic nerve lesion.

Source credit: RAPD (Relative Afferent Pupillary Defect) Examination video for HKU medical students s19.wmv - trin0345.

Topic 21 - Otoscopy: acute otitis media

Subject: ENT

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:16

Source credit: Otitis Media - AGX Hearing



What it is

Otoscopy: acute otitis media: otoscopy showing bulging erythematous tympanic membrane. High-yield anchors: AOM; bulging TM; middle ear effusion; treatment basics.

How to identify it in the video

otoscopy showing bulging erythematous tympanic membrane. Watch for AOM, bulging TM, middle ear effusion, treatment basics. Video cues for Otoscopy: acute otitis media: visible mucosa or membrane, swelling, mass, perforation, or discharge, movement during phonation, swallowing, or breathing, side and symmetry.

Why it happens

The mechanism of Otoscopy: acute otitis media starts with AOM and is expressed clinically through bulging TM. Related exam anchors are middle ear effusion, treatment basics.

Associated conditions

- bulging TM
- middle ear effusion
- treatment basics

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
otitis externa	Canal tenderness and edema; tragal/pinna movement is painful.	Use the AOM endpoint to decide whether otitis externa explains the clip.
OME	Retracted or dull tympanic membrane with air-fluid level but no acute fever or bulging.	Use the AOM endpoint to decide whether OME explains the clip.
CSOM	Chronic discharge through tympanic membrane perforation.	Document safe versus unsafe perforation.

Clinical examination points

- Otoscopy: acute otitis media: Inspect pinna/canal, test tragal tenderness, visualize tympanic membrane landmarks, mobility, perforation, and discharge.

NEET PG exam angle

Otoscopy: acute otitis media is usually tested through AOM; bulging TM; middle ear effusion. Separate it from otitis externa, OME, CSOM.

Management relevance

Analgesia, observation or antibiotics by age/severity, and review for complications.

Common traps

- Do not confuse with otitis externa: Canal tenderness and edema; tragal/pinna movement is painful.
- Do not confuse with OME: Retracted or dull tympanic membrane with air-fluid level but no acute fever or bulging.
- Do not confuse with CSOM: Chronic discharge through tympanic membrane perforation.

- Otoscopy: acute otitis media needs both AOM and bulging TM; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Otoscopy: acute otitis media.
- Subject: ENT.
- Key clue: AOM.
- Key clue: bulging TM.
- Key clue: middle ear effusion.
- Closest mimics: otitis externa, OME, CSOM.
- Video segment: 00:00-00:16.
- One-line recall: Otoscopy: acute otitis media points to AOM.

Source credit: Otitis Media - AGX Hearing.

Topic 22 - Otoscopy: CSOM / perforation / cholesteatoma

Subject: ENT

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:47

Source credit: Cholesteatoma from a Tympanic Perforation - earsite1



What it is

Otoscopy: CSOM / perforation / cholesteatoma: otoscopy clip showing perforation or cholesteatoma. High-yield anchors: central perforation; attic perforation; cholesteatoma; complications.

How to identify it in the video

otoscopy clip showing perforation or cholesteatoma. Watch for central perforation, attic perforation, cholesteatoma, complications. Video cues for Otoscopy: CSOM / perforation / cholesteatoma: visible mucosa or membrane, swelling, mass, perforation, or discharge, movement during phonation, swallowing, or breathing, side and symmetry.

Why it happens

The mechanism of Otoscopy: CSOM / perforation / cholesteatoma starts with central perforation and is expressed clinically through attic perforation. Related exam anchors are cholesteatoma, complications.

Associated conditions

- central perforation
- attic perforation
- cholesteatoma
- complications

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
AOM	Bulging erythematous tympanic membrane with acute pain and fever.	Use the central perforation endpoint to decide whether AOM explains the clip.
otitis externa	Canal tenderness and edema; tragal/pinna movement is painful.	Use the central perforation endpoint to decide whether otitis externa explains the clip.
wax	Cerumen blocks canal view.	Remove wax before diagnosing middle-ear disease.

Clinical examination points

- Otoscopy: CSOM / perforation / cholesteatoma: Inspect pinna/canal, test tragal tenderness, visualize tympanic membrane landmarks, mobility, perforation, and discharge.

NEET PG exam angle

Otoscopy: CSOM / perforation / cholesteatoma is usually tested through central perforation; attic perforation; cholesteatoma. Separate it from AOM, otitis externa, wax.

Management relevance

ENT referral is essential because cholesteatoma erodes bone and can cause intracranial complications.

Common traps

- Do not confuse with AOM: Bulging erythematous tympanic membrane with acute pain and fever.
- Do not confuse with otitis externa: Canal tenderness and edema; tragal/pinna movement is painful.

- Do not confuse with wax: Cerumen blocks canal view.
- Otoscopy: CSOM / perforation / cholesteatoma needs both central perforation and attic perforation; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Otoscopy: CSOM / perforation / cholesteatoma.
- Subject: ENT.
- Key clue: central perforation.
- Key clue: attic perforation.
- Key clue: cholesteatoma.
- Closest mimics: AOM, otitis externa, wax.
- Video segment: 00:00-00:47.
- One-line recall: Otoscopy: CSOM / perforation / cholesteatoma points to central perforation.

Source credit: Cholesteatoma from a Tympanic Perforation - earsite1.

Topic 23 - Vocal cord palsy on laryngoscopy

Subject: ENT

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Vocal Cord Paralysis: 6 Different Types of Paralysis and How It Looks/Moves - Fauquier ENT



What it is

Vocal cord palsy on laryngoscopy: vocal fold paralysis on laryngoscopy. High-yield anchors: recurrent laryngeal nerve palsy; thyroid surgery; hoarseness.

How to identify it in the video

vocal fold paralysis on laryngoscopy. Watch for recurrent laryngeal nerve palsy, thyroid surgery, hoarseness. Video cues for Vocal cord palsy on laryngoscopy: pupil size and symmetry, direction of eye deviation or movement limitation, ptosis or lid asymmetry, nystagmus direction, light reflex or staining pattern.

Why it happens

The mechanism of Vocal cord palsy on laryngoscopy starts with recurrent laryngeal nerve palsy and is expressed clinically through thyroid surgery. Related exam anchors are hoarseness. In Vocal cord palsy on laryngoscopy, the bedside pattern localizes the relevant afferent, efferent, ocular motor, sympathetic, vestibular, or cerebellar pathway.

Associated conditions

- recurrent laryngeal nerve palsy
- thyroid surgery
- hoarseness

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
vocal cord nodule	Usually bilateral at the junction of anterior and middle thirds.	Voice abuse history.
laryngeal carcinoma	Irregular unilateral lesion, smoking history.	Persistent hoarseness, biopsy required.
functional dysphonia	Normal laryngoscopy with abnormal voice use pattern.	Speech therapy assessment helps.

Clinical examination points

- Vocal cord palsy on laryngoscopy: Check pupils, ocular alignment, extraocular movements, ptosis, diplopia direction, and visual fields.
- Vocal cord palsy on laryngoscopy: Localize afferent and efferent pathways before naming the lesion.

NEET PG exam angle

Vocal cord palsy on laryngoscopy is usually tested through recurrent laryngeal nerve palsy; thyroid surgery; hoarseness. Separate it from vocal cord nodule, laryngeal carcinoma, functional dysphonia.

Management relevance

Evaluate recurrent laryngeal nerve course, thyroid surgery history, malignancy, and aspiration risk.

Common traps

- Do not confuse with vocal cord nodule: Usually bilateral at the junction of anterior and middle thirds.
- Do not confuse with laryngeal carcinoma: Irregular unilateral lesion, smoking history.

- Do not confuse with functional dysphonia: Normal laryngoscopy with abnormal voice use pattern.
- Vocal cord palsy on laryngoscopy needs both recurrent laryngeal nerve palsy and thyroid surgery; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Vocal cord palsy on laryngoscopy.
- Subject: ENT.
- Key clue: recurrent laryngeal nerve palsy.
- Key clue: thyroid surgery.
- Key clue: hoarseness.
- Closest mimics: vocal cord nodule, laryngeal carcinoma, functional dysphonia.
- Video segment: 00:00-01:00.
- One-line recall: Vocal cord palsy on laryngoscopy points to recurrent laryngeal nerve palsy.

Source credit: Vocal Cord Paralysis: 6 Different Types of Paralysis and How It Looks/Moves - Fauquier ENT.

Topic 24 - Vocal cord nodule / polyp

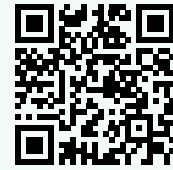
Subject: ENT

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:53

Source credit: How Vocal Polyp Removal Surgery Helped a Patient Regain Her Voice - Mid-Atlantic Permanente Medicine



What it is

Vocal cord nodule / polyp: laryngoscopy clip of vocal cord nodule/polyp. High-yield anchors: singer's nodule; hoarseness; bilateral nodules; voice abuse.

How to identify it in the video

laryngoscopy clip of vocal cord nodule/polyp. Watch for singer's nodule, hoarseness, bilateral nodules, voice abuse. Video cues for Vocal cord nodule / polyp: visible mucosa or membrane, swelling, mass, perforation, or discharge, movement during phonation, swallowing, or breathing, side and symmetry.

Why it happens

The mechanism of Vocal cord nodule / polyp starts with singer's nodule and is expressed clinically through hoarseness. Related exam anchors are bilateral nodules, voice abuse.

Associated conditions

- singer's nodule
- hoarseness
- bilateral nodules
- voice abuse

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
vocal cord palsy	Immobile cord on phonation/respiration.	Image recurrent laryngeal nerve pathway.
malignancy	Irregular, ulcerated.	Vascular, fixed, or progressive lesion needs biopsy.
polyp	Usually unilateral pedunculated lesion with voice abuse or acute phonotrauma history.	Use the singer's nodule endpoint to decide whether polyp explains the clip.

Clinical examination points

- For Vocal cord nodule / polyp, document the visible abnormality before naming the diagnosis.
- For Vocal cord nodule / polyp, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Vocal cord nodule / polyp is usually tested through singer's nodule; hoarseness; bilateral nodules. Separate it from vocal cord palsy, malignancy, polyp.

Management relevance

Clinical action after recognizing Vocal cord nodule / polyp: confirm singer's nodule, look for complications, and choose treatment according to the underlying diagnosis.

Common traps

- Do not confuse with vocal cord palsy: Immobile cord on phonation/respiration.

- Do not confuse with malignancy: Irregular, ulcerated.
- Do not confuse with polyp: Usually unilateral pedunculated lesion with voice abuse or acute phonotrauma history.
- Vocal cord nodule / polyp needs both singer's nodule and hoarseness; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Vocal cord nodule / polyp.
- Subject: ENT.
- Key clue: singer's nodule.
- Key clue: hoarseness.
- Key clue: bilateral nodules.
- Closest mimics: vocal cord palsy, malignancy, polyp.
- Video segment: 00:00-00:53.
- One-line recall: Vocal cord nodule / polyp points to singer's nodule.

Source credit: How Vocal Polyp Removal Surgery Helped a Patient Regain Her Voice - Mid-Atlantic Permanente Medicine.

Topic 25 - Dix-Hallpike test

Subject: ENT / Medicine

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:28

Source credit: Dix-Hallpike Test for Vertigo (BPPV) #ent #meded #physiotherapy #vertigo - Med Education



What it is

Dix-Hallpike test: positional test for BPPV showing nystagmus. High-yield anchors: posterior canal BPPV; vertigo; torsional nystagmus.

How to identify it in the video

positional test for BPPV showing nystagmus. Watch for posterior canal BPPV, vertigo, torsional nystagmus. Video cues for Dix-Hallpike test: the visible endpoint of Dix-Hallpike test, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Dix-Hallpike test starts with posterior canal BPPV and is expressed clinically through vertigo. Related exam anchors are torsional nystagmus.

Associated conditions

- posterior canal BPPV
- vertigo
- torsional nystagmus
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
vestibular neuritis	Acute prolonged vertigo after viral illness with no hearing loss.	Abnormal head impulse, no hearing loss.
Ménière disease	Episodic vertigo with fluctuating hearing loss.	Tinnitus, and aural fullness.
central vertigo	Direction-changing nystagmus, skew deviation.	Ataxia, or neurologic signs need stroke workup.

Clinical examination points

- Dix-Hallpike test: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Dix-Hallpike test: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Dix-Hallpike test is usually tested through posterior canal BPPV; vertigo; torsional nystagmus. Separate it from vestibular neuritis, Ménière disease, central vertigo.

Management relevance

Peripheral causes may need vestibular treatment; central nystagmus needs urgent neurologic evaluation.

Common traps

- Do not confuse with vestibular neuritis: Acute prolonged vertigo after viral illness with no hearing loss.
- Do not confuse with Ménière disease: Episodic vertigo with fluctuating hearing loss.
- Do not confuse with central vertigo: Direction-changing nystagmus, skew deviation.
- Dix-Hallpike test needs both posterior canal BPPV and vertigo; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Dix-Hallpike test.
- Subject: ENT / Medicine.
- Key clue: posterior canal BPPV.
- Key clue: vertigo.
- Key clue: torsional nystagmus.
- Closest mimics: vestibular neuritis, Ménière disease, central vertigo.
- Video segment: 00:00-00:28.
- One-line recall: Dix-Hallpike test points to posterior canal BPPV.

Source credit: Dix-Hallpike Test for Vertigo (BPPV) #ent #meded #physiotherapy #vertigo - Med Education.

Topic 26 - Epley maneuver

Subject: ENT / Medicine

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Epley Maneuver: Performed on a Patient Suffering from Vertigo - EM:RAP Medical Education



What it is

Epley maneuver: canalith repositioning procedure. High-yield anchors: BPPV treatment; posterior canal; canalith repositioning.

How to identify it in the video

canalith repositioning procedure. Watch for BPPV treatment, posterior canal, canalith repositioning. Video cues for Epley maneuver: the visible endpoint of Epley maneuver, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Epley maneuver starts with BPPV treatment and is expressed clinically through posterior canal. Related exam anchors are canalith repositioning.

Associated conditions

- BPPV treatment
- posterior canal
- canalith repositioning
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
Dix-Hallpike test	Diagnostic positional test.	Torsional upbeat nystagmus supports posterior canal BPPV.
Semont maneuver	Treatment maneuver with rapid side-to-side movement.	Not the diagnostic test.
vestibular rehab	Exercise-based compensation strategy after acute diagnosis is established.	Use the BPPV treatment endpoint to decide whether vestibular rehab explains the clip.

Clinical examination points

- Epley maneuver: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Epley maneuver: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Epley maneuver is usually tested through BPPV treatment; posterior canal; canalith repositioning. Separate it from Dix-Hallpike test, Semont maneuver, vestibular rehab.

Management relevance

Posterior canal BPPV is treated with Epley maneuver after diagnostic Dix-Hallpike.

Common traps

- Do not confuse with Dix-Hallpike test: Diagnostic positional test.
- Do not confuse with Semont maneuver: Treatment maneuver with rapid side-to-side movement.
- Do not confuse with vestibular rehab: Exercise-based compensation strategy after acute diagnosis is established.
- Epley maneuver needs both BPPV treatment and posterior canal; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Epley maneuver.
- Subject: ENT / Medicine.
- Key clue: BPPV treatment.
- Key clue: posterior canal.
- Key clue: canalith repositioning.
- Closest mimics: Dix-Hallpike test, Semont maneuver, vestibular rehab.
- Video segment: 00:00-01:00.
- One-line recall: Epley maneuver points to BPPV treatment.

Source credit: Epley Maneuver: Performed on a Patient Suffering from Vertigo - EM:RAP Medical Education.

Topic 27 - Rinne and Weber tests

Subject: ENT

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Hearing Test (Rinne and Weber Examinations) - ENT - Oxford Medical Education



What it is

Rinne and Weber tests: tuning fork test demonstration. High-yield anchors: conductive vs sensorineural hearing loss; Rinne positive/negative; Weber lateralization.

How to identify it in the video

tuning fork test demonstration. Watch for conductive vs sensorineural hearing loss, Rinne positive/negative, Weber lateralization. Video cues for Rinne and Weber tests: the visible endpoint of Rinne and Weber tests, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Rinne and Weber tests starts with conductive vs sensorineural hearing loss and is expressed clinically through Rinne positive/negative. Related exam anchors are Weber lateralization.

Associated conditions

- conductive vs sensorineural hearing loss
- Rinne positive/negative
- Weber lateralization

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
mixed hearing loss	Audiometry shows air-bone gap plus elevated bone-conduction thresholds.	Use the conductive vs sensorineural hearing loss endpoint to decide whether mixed hearing loss explains the clip.
malinger	Inconsistent responses.	Stenger test or objective audiometry may be needed.
severe SNHL	Rinne can appear falsely negative.	Audiometry confirms profound sensorineural loss.

Clinical examination points

- Rinne and Weber tests: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Rinne and Weber tests: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Rinne and Weber tests is usually tested through conductive vs sensorineural hearing loss; Rinne positive/negative; Weber lateralization. Separate it from mixed hearing loss, malingering, severe SNHL.

Management relevance

Use the demonstrated technique only for the correct indication; the management decision hinges on conductive vs sensorineural hearing loss, patient safety, and the endpoint shown in the video.

Common traps

- Do not confuse with mixed hearing loss: Audiometry shows air-bone gap plus elevated bone-conduction thresholds.

- Do not confuse with malingering: Inconsistent responses.
- Do not confuse with severe SNHL: Rinne can appear falsely negative.
- Rinne and Weber tests needs both conductive vs sensorineural hearing loss and Rinne positive/negative; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Rinne and Weber tests.
- Subject: ENT.
- Key clue: conductive vs sensorineural hearing loss.
- Key clue: Rinne positive/negative.
- Key clue: Weber lateralization.
- Closest mimics: mixed hearing loss, malingering, severe SNHL.
- Video segment: 00:00-01:00.
- One-line recall: Rinne and Weber tests points to conductive vs sensorineural hearing loss.

Source credit: Hearing Test (Rinne and Weber Examinations) - ENT - Oxford Medical Education.

Topic 28 - Lachman test

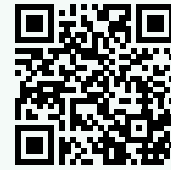
Subject: Orthopedics

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:36

Source credit: Knee Exam: Lachman Test - UW - Department of Family Medicine and Community Health



What it is

Lachman test: ACL examination with Lachman test. High-yield anchors: ACL tear; anterior tibial translation; endpoint.

How to identify it in the video

ACL examination with Lachman test. Watch for ACL tear, anterior tibial translation, endpoint. Video cues for Lachman test: the visible endpoint of Lachman test, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Lachman test starts with ACL tear and is expressed clinically through anterior tibial translation. Related exam anchors are endpoint.

Associated conditions

- ACL tear
- anterior tibial translation
- endpoint
- traumatic injury
- sports injury
- chronic degenerative disease

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
anterior drawer	Knee at 90 degrees.	Anterior tibial translation is less sensitive than Lachman acutely.
PCL injury	Posterior sag or posterior translation, often after dashboard injury.	Dashboard injury is classic.
meniscal tear	Joint-line tenderness, locking, clicking, and positive McMurray/Thessaly.	McMurray/Thessaly, and MRI if needed.

Clinical examination points

- Lachman test: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Lachman test: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Lachman test is usually tested through ACL tear; anterior tibial translation; endpoint. Separate it from anterior drawer, PCL injury, meniscal tear.

Management relevance

Acute care includes RICE, imaging where needed, physiotherapy, and orthopedic planning for instability.

Common traps

- Do not confuse with anterior drawer: Knee at 90 degrees.

- Do not confuse with PCL injury: Posterior sag or posterior translation, often after dashboard injury.
- Do not confuse with meniscal tear: Joint-line tenderness, locking, clicking, and positive McMurray/Thessaly.
- Lachman test needs both ACL tear and anterior tibial translation; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Lachman test.
- Subject: Orthopedics.
- Key clue: ACL tear.
- Key clue: anterior tibial translation.
- Key clue: endpoint.
- Closest mimics: anterior drawer, PCL injury, meniscal tear.
- Video segment: 00:00-00:36.
- One-line recall: Lachman test points to ACL tear.

Source credit: Knee Exam: Lachman Test - UW - Department of Family Medicine and Community Health.

Topic 29 - Anterior drawer test knee

Subject: Orthopedics

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:58

Source credit: Anterior Drawer Test Knee | ACL Tear - Med Education



What it is

Anterior drawer test knee: anterior drawer demonstration. High-yield anchors: ACL injury; anterior translation; knee flexion position.

How to identify it in the video

anterior drawer demonstration. Watch for ACL injury, anterior translation, knee flexion position. Video cues for Anterior drawer test knee: the visible endpoint of Anterior drawer test knee, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Anterior drawer test knee starts with ACL injury and is expressed clinically through anterior translation. Related exam anchors are knee flexion position.

Associated conditions

- ACL injury
- anterior translation
- knee flexion position
- traumatic injury
- sports injury
- chronic degenerative disease

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
Lachman test	Knee at 20-30 degrees.	Soft endpoint is the ACL clue.
posterior drawer	Posterior tibial translation at 90 degrees confirms PCL laxity.	Use the ACL injury endpoint to decide whether posterior drawer explains the clip.
ligament laxity	Compare both sides and look for generalized hypermobility rather than focal injury.	Use the ACL injury endpoint to decide whether ligament laxity explains the clip.

Clinical examination points

- Anterior drawer test knee: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Anterior drawer test knee: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Anterior drawer test knee is usually tested through ACL injury; anterior translation; knee flexion position. Separate it from Lachman test, posterior drawer, ligament laxity.

Management relevance

Acute care includes RICE, imaging where needed, physiotherapy, and orthopedic planning for instability.

Common traps

- Do not confuse with Lachman test: Knee at 20-30 degrees.
- Do not confuse with posterior drawer: Posterior tibial translation at 90 degrees confirms PCL laxity.
- Do not confuse with ligament laxity: Compare both sides and look for generalized hypermobility rather than focal injury.
- Anterior drawer test knee needs both ACL injury and anterior translation; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Anterior drawer test knee.
- Subject: Orthopedics.
- Key clue: ACL injury.
- Key clue: anterior translation.
- Key clue: knee flexion position.
- Closest mimics: Lachman test, posterior drawer, ligament laxity.
- Video segment: 00:00-00:58.
- One-line recall: Anterior drawer test knee points to ACL injury.

Source credit: Anterior Drawer Test Knee | ACL Tear - Med Education.

Topic 30 - Posterior drawer test

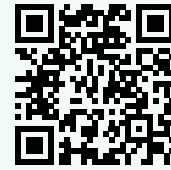
Subject: Orthopedics

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Posterior Sag Test - Clinical Examination - AMBOSS: Medical Knowledge Distilled



What it is

Posterior drawer test: PCL examination or posterior sag sign. High-yield anchors: PCL tear; dashboard injury; posterior tibial translation.

How to identify it in the video

PCL examination or posterior sag sign. Watch for PCL tear, dashboard injury, posterior tibial translation. Video cues for Posterior drawer test: the visible endpoint of Posterior drawer test, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Posterior drawer test starts with PCL tear and is expressed clinically through dashboard injury. Related exam anchors are posterior tibial translation.

Associated conditions

- PCL tear
- dashboard injury
- posterior tibial translation
- traumatic injury
- sports injury
- chronic degenerative disease

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
ACL tear	Pivot injury, immediate swelling.	Lachman/pivot shift, and MRI confirmation.
meniscal injury	Mechanical locking and joint-line tenderness point away from isolated ligament injury.	Use the PCL tear endpoint to decide whether meniscal injury explains the clip.
collateral ligament injury	Varus/valgus opening at 30 degrees localizes LCL/MCL injury.	Use the PCL tear endpoint to decide whether collateral ligament injury explains the clip.

Clinical examination points

- Posterior drawer test: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Posterior drawer test: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Posterior drawer test is usually tested through PCL tear; dashboard injury; posterior tibial translation. Separate it from ACL tear, meniscal injury, collateral ligament injury.

Management relevance

Use the demonstrated technique only for the correct indication; the management decision hinges on PCL tear, patient safety, and the endpoint shown in the video.

Common traps

- Do not confuse with ACL tear: Pivot injury, immediate swelling.
- Do not confuse with meniscal injury: Mechanical locking and joint-line tenderness point away from isolated ligament injury.
- Do not confuse with collateral ligament injury: Varus/valgus opening at 30 degrees localizes LCL/MCL injury.
- Posterior drawer test needs both PCL tear and dashboard injury; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Posterior drawer test.
- Subject: Orthopedics.
- Key clue: PCL tear.
- Key clue: dashboard injury.
- Key clue: posterior tibial translation.
- Closest mimics: ACL tear, meniscal injury, collateral ligament injury.
- Video segment: 00:00-01:00.
- One-line recall: Posterior drawer test points to PCL tear.

Source credit: Posterior Sag Test - Clinical Examination - AMBOSS: Medical Knowledge Distilled.

Topic 31 - McMurray test

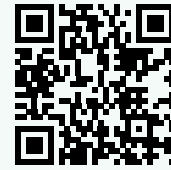
Subject: Orthopedics

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:57

Source credit: McMurray Special Test for Meniscus Tear - Knee Clinical Examination - Mike Reinold



What it is

McMurray test: meniscal test demonstration. High-yield anchors: meniscal tear; locking; clicking; joint line tenderness.

How to identify it in the video

meniscal test demonstration. Watch for meniscal tear, locking, clicking, joint line tenderness. Video cues for McMurray test: the visible endpoint of McMurray test, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of McMurray test starts with meniscal tear and is expressed clinically through locking. Related exam anchors are clicking, joint line tenderness.

Associated conditions

- meniscal tear
- locking
- clicking
- joint line tenderness
- traumatic injury
- sports injury
- chronic degenerative disease

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
ligament injury	Stress the specific ligament and assess endpoint.	Swelling, and instability direction.
osteoarthritis	Crepitus, bony enlargement.	Activity pain, and weight-bearing X-ray changes.
patellofemoral pain	Anterior knee pain with stairs/squatting and patellar grind/apprehension signs.	Use the meniscal tear endpoint to decide whether patellofemoral pain explains the clip.

Clinical examination points

- McMurray test: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- McMurray test: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

McMurray test is usually tested through meniscal tear; locking; clicking. Separate it from ligament injury, osteoarthritis, patellofemoral pain.

Management relevance

Use the demonstrated technique only for the correct indication; the management decision hinges on meniscal tear, patient safety, and the endpoint shown in the video.

Common traps

- Do not confuse with ligament injury: Stress the specific ligament and assess endpoint.
- Do not confuse with osteoarthritis: Crepitus, bony enlargement.
- Do not confuse with patellofemoral pain: Anterior knee pain with stairs/squatting and patellar grind/apprehension signs.
- McMurray test needs both meniscal tear and locking; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: McMurray test.
- Subject: Orthopedics.
- Key clue: meniscal tear.
- Key clue: locking.
- Key clue: clicking.
- Closest mimics: ligament injury, osteoarthritis, patellofemoral pain.
- Video segment: 00:00-00:57.
- One-line recall: McMurray test points to meniscal tear.

Source credit: McMurray Special Test for Meniscus Tear - Knee Clinical Examination - Mike Reinold.

Topic 32 - Varus and valgus stress tests

Subject: Orthopedics

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:26

Source credit: Knee Valgus and Varus Stress Tests - MSK Medicine



What it is

Varus and valgus stress tests: MCL/LCL stress test demonstration. High-yield anchors: MCL injury; LCL injury; varus/valgus stress.

How to identify it in the video

MCL/LCL stress test demonstration. Watch for MCL injury, LCL injury, varus/valgus stress. Video cues for Varus and valgus stress tests: the visible endpoint of Varus and valgus stress tests, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Varus and valgus stress tests starts with MCL injury and is expressed clinically through LCL injury. Related exam anchors are varus/valgus stress.

Associated conditions

- MCL injury
- LCL injury
- varus/valgus stress
- traumatic injury
- sports injury
- chronic degenerative disease

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
ACL injury	Hemarthrosis after twisting injury with anterior translation and soft endpoint.	Use the MCL injury endpoint to decide whether ACL injury explains the clip.
meniscal tear	Joint-line tenderness, locking, clicking, and positive McMurray/Thessaly.	McMurray/Thessaly, and MRI if needed.
generalized laxity	Beighton score and bilateral symmetry separate laxity from acute injury.	Use the MCL injury endpoint to decide whether generalized laxity explains the clip.

Clinical examination points

- Varus and valgus stress tests: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Varus and valgus stress tests: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Varus and valgus stress tests is usually tested through MCL injury; LCL injury; varus/valgus stress. Separate it from ACL injury, meniscal tear, generalized laxity.

Management relevance

Use the demonstrated technique only for the correct indication; the management decision hinges on MCL injury, patient safety, and the endpoint shown in the video.

Common traps

- Do not confuse with ACL injury: Hemarthrosis after twisting injury with anterior translation and soft endpoint.
- Do not confuse with meniscal tear: Joint-line tenderness, locking, clicking, and positive McMurray/Thessaly.
- Do not confuse with generalized laxity: Beighton score and bilateral symmetry separate laxity from acute injury.
- Varus and valgus stress tests needs both MCL injury and LCL injury; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Varus and valgus stress tests.
- Subject: Orthopedics.
- Key clue: MCL injury.
- Key clue: LCL injury.
- Key clue: varus/valgus stress.
- Closest mimics: ACL injury, meniscal tear, generalized laxity.
- Video segment: 00:00-00:26.
- One-line recall: Varus and valgus stress tests points to MCL injury.

Source credit: Knee Valgus and Varus Stress Tests - MSK Medicine.

Topic 33 - Phalen and Tinel signs

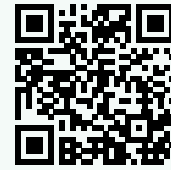
Subject: Orthopedics / Medicine

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:30

Source credit: Carpal Tunnel Tests - Phalen's and Tinel Tests - Advanced Massage Techniques School



What it is

Phalen and Tinel signs: carpal tunnel examination. High-yield anchors: median nerve compression; carpal tunnel syndrome; paresthesia.

How to identify it in the video

carpal tunnel examination. Watch for median nerve compression, carpal tunnel syndrome, paresthesia. Video cues for Phalen and Tinel signs: the visible endpoint of Phalen and Tinel signs, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Phalen and Tinel signs starts with median nerve compression and is expressed clinically through carpal tunnel syndrome. Related exam anchors are paresthesia.

Associated conditions

- median nerve compression
- carpal tunnel syndrome
- paresthesia
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant
- traumatic injury
- sports injury

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
cervical radiculopathy	Dermatomal pain, Spurling test.	Reflex changes, and cervical imaging/EMG.
ulnar neuropathy	Fourth/fifth digit symptoms.	Froment sign, cubital tunnel/Guyon canal localization.
pronator syndrome	Forearm pain with no nocturnal symptoms; Tinel is positive over the proximal forearm, not the wrist.	Use the median nerve compression endpoint to decide whether pronator syndrome explains the clip.

Clinical examination points

- Phalen and Tinel signs: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Phalen and Tinel signs: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Phalen and Tinel signs is usually tested through median nerve compression; carpal tunnel syndrome; paresthesia. Separate it from cervical radiculopathy, ulnar neuropathy, pronator syndrome.

Management relevance

Use the demonstrated technique only for the correct indication; the management decision hinges on median nerve compression, patient safety, and the endpoint shown in the video.

Common traps

- Do not confuse with cervical radiculopathy: Dermatomal pain, Spurling test.
- Do not confuse with ulnar neuropathy: Fourth/fifth digit symptoms.
- Do not confuse with pronator syndrome: Retracted or dull tympanic membrane with air-fluid level but no acute fever or bulging.
- Phalen and Tinel signs needs both median nerve compression and carpal tunnel syndrome; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Phalen and Tinel signs.
- Subject: Orthopedics / Medicine.
- Key clue: median nerve compression.
- Key clue: carpal tunnel syndrome.
- Key clue: paresthesia.
- Closest mimics: cervical radiculopathy, ulnar neuropathy, pronator syndrome.
- Video segment: 00:00-00:30.
- One-line recall: Phalen and Tinel signs points to median nerve compression.

Source credit: Carpal Tunnel Tests - Phalen's and Tinel Tests - Advanced Massage Techniques School.

Topic 34 - Finkelstein test

Subject: Orthopedics

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:27

Source credit: Finkelstein's Test - MSK Medicine



What it is

Finkelstein test: de Quervain tenosynovitis test. High-yield anchors: abductor pollicis longus; extensor pollicis brevis; radial styloid pain.

How to identify it in the video

de Quervain tenosynovitis test. Watch for abductor pollicis longus, extensor pollicis brevis, radial styloid pain. Video cues for Finkelstein test: the visible endpoint of Finkelstein test, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Finkelstein test starts with abductor pollicis longus and is expressed clinically through extensor pollicis brevis. Related exam anchors are radial styloid pain.

Associated conditions

- abductor pollicis longus
- extensor pollicis brevis
- radial styloid pain
- traumatic injury
- sports injury
- chronic degenerative disease

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
CMC arthritis	Thumb-base pain.	Grind test, and first CMC joint X-ray changes.
scaphoid fracture	Anatomic snuffbox tenderness after fall.	Immobilize and repeat imaging/MRI.
intersection syndrome	Pain and crepitus 4-6 cm proximal to the wrist, not at the radial styloid as in de Quervain.	Use the abductor pollicis longus endpoint to decide whether intersection syndrome explains the clip.

Clinical examination points

- Finkelstein test: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Finkelstein test: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Finkelstein test is usually tested through abductor pollicis longus; extensor pollicis brevis; radial styloid pain. Separate it from CMC arthritis, scaphoid fracture, intersection syndrome.

Management relevance

Use the demonstrated technique only for the correct indication; the management decision hinges on abductor pollicis longus, patient safety, and the endpoint shown in the video.

Common traps

- Do not confuse with CMC arthritis: Thumb-base pain.
- Do not confuse with scaphoid fracture: Anatomic snuffbox tenderness after fall.
- Do not confuse with intersection syndrome: Retracted or dull tympanic membrane with air-fluid level but no acute fever or bulging.
- Finkelstein test needs both abductor pollicis longus and extensor pollicis brevis; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Finkelstein test.
- Subject: Orthopedics.
- Key clue: abductor pollicis longus.
- Key clue: extensor pollicis brevis.
- Key clue: radial styloid pain.
- Closest mimics: CMC arthritis, scaphoid fracture, intersection syndrome.
- Video segment: 00:00-00:27.
- One-line recall: Finkelstein test points to abductor pollicis longus.

Source credit: Finkelstein's Test - MSK Medicine.

Topic 35 - Thomas test

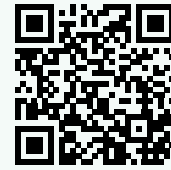
Subject: Orthopedics

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Thomas Test - OSCE Guide | Clip | UKMLA | CPSA | PLAB 2 - Geeky Medics



What it is

Thomas test: hip fixed flexion deformity test. High-yield anchors: hip flexion contracture; psoas tightness; lumbar lordosis correction.

How to identify it in the video

hip fixed flexion deformity test. Watch for hip flexion contracture, psoas tightness, lumbar lordosis correction. Video cues for Thomas test: the visible endpoint of Thomas test, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Thomas test starts with hip flexion contracture and is expressed clinically through psoas tightness. Related exam anchors are lumbar lordosis correction.

Associated conditions

- hip flexion contracture
- psoas tightness
- lumbar lordosis correction
- traumatic injury
- sports injury
- chronic degenerative disease

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
FABER test	Pain location matters: groin suggests hip, posterior pain suggests SI joint.	Use the hip flexion contracture endpoint to decide whether FABER test explains the clip.
hip OA	Loss of internal rotation and groin pain.	Pelvis/hip X-ray confirms.
spine pathology	Neurologic deficit, radicular pain.	Red flags, and spine imaging guide diagnosis.

Clinical examination points

- Thomas test: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Thomas test: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Thomas test is usually tested through hip flexion contracture; psoas tightness; lumbar lordosis correction. Separate it from FABER test, hip OA, spine pathology.

Management relevance

Use the demonstrated technique only for the correct indication; the management decision hinges on hip flexion contracture, patient safety, and the endpoint shown in the video.

Common traps

- Do not confuse with FABER test: Pain location matters: groin suggests hip, posterior pain suggests SI joint.
- Do not confuse with hip OA: Loss of internal rotation and groin pain.
- Do not confuse with spine pathology: Neurologic deficit, radicular pain.
- Thomas test needs both hip flexion contracture and psoas tightness; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Thomas test.
- Subject: Orthopedics.
- Key clue: hip flexion contracture.
- Key clue: psoas tightness.
- Key clue: lumbar lordosis correction.
- Closest mimics: FABER test, hip OA, spine pathology.
- Video segment: 00:00-01:00.
- One-line recall: Thomas test points to hip flexion contracture.

Source credit: Thomas Test - OSCE Guide | Clip | UKMLA | CPSA | PLAB 2 - Geeky Medics.

Topic 36 - Shoulder apprehension / relocation test

Subject: Orthopedics

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:24

Source credit: Apprehension Test (Shoulder) - MSK Medicine



What it is

Shoulder apprehension / relocation test: anterior shoulder instability demonstration. High-yield anchors: anterior shoulder dislocation; apprehension; relocation.

How to identify it in the video

anterior shoulder instability demonstration. Watch for anterior shoulder dislocation, apprehension, relocation. Video cues for Shoulder apprehension / relocation test: the visible endpoint of Shoulder apprehension / relocation test, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Shoulder apprehension / relocation test starts with anterior shoulder dislocation and is expressed clinically through apprehension. Related exam anchors are relocation.

Associated conditions

- anterior shoulder dislocation
- apprehension
- relocation
- traumatic injury
- sports injury
- chronic degenerative disease

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
rotator cuff tear	Weak abduction/external rotation.	Drop-arm test, ultrasound/MRI.
impingement	Painful arc.	Neer/Hawkins tests, preserved passive range.
adhesive capsulitis	Global restriction of active and passive shoulder movement, especially external rotation.	Use the anterior shoulder dislocation endpoint to decide whether adhesive capsulitis explains the clip.

Clinical examination points

- Shoulder apprehension / relocation test: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Shoulder apprehension / relocation test: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Shoulder apprehension / relocation test is usually tested through anterior shoulder dislocation; apprehension; relocation. Separate it from rotator cuff tear, impingement, adhesive capsulitis.

Management relevance

Use the demonstrated technique only for the correct indication; the management decision hinges on anterior shoulder dislocation, patient safety, and the endpoint shown in the video.

Common traps

- Do not confuse with rotator cuff tear: Weak abduction/external rotation.
- Do not confuse with impingement: Painful arc.
- Do not confuse with adhesive capsulitis: Global restriction of active and passive shoulder movement, especially external rotation.
- Shoulder apprehension / relocation test needs both anterior shoulder dislocation and apprehension; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Shoulder apprehension / relocation test.
- Subject: Orthopedics.
- Key clue: anterior shoulder dislocation.
- Key clue: apprehension.
- Key clue: relocation.
- Closest mimics: rotator cuff tear, impingement, adhesive capsulitis.
- Video segment: 00:00-00:24.
- One-line recall: Shoulder apprehension / relocation test points to anterior shoulder dislocation.

Source credit: Apprehension Test (Shoulder) - MSK Medicine.

Topic 37 - FAST scan: free fluid

Subject: Surgery / Emergency / Radiology

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Spotting Fluid on Ultrasound: FAST RUQ Edition-Focused Assessment with Sonography in Trauma Sample - Gulfcoast Ultrasound Institute



What it is

FAST scan: free fluid: FAST ultrasound showing free fluid. High-yield anchors: trauma; hemoperitoneum; Morrison pouch; unstable patient.

How to identify it in the video

FAST ultrasound showing free fluid. Watch for trauma, hemoperitoneum, Morrison pouch, unstable patient. Video cues for FAST scan: free fluid: probe view and anatomic landmark, moving structure or compressibility, fluid, shadowing, or artifact, dynamic change with pressure or respiration.

Why it happens

The mechanism of FAST scan: free fluid starts with trauma and is expressed clinically through hemoperitoneum. Related exam anchors are Morrison pouch, unstable patient. In FAST scan: free fluid, the decisive clue is dynamic: compressibility, fluid movement, valve motion, shadowing, or probe position changes the diagnosis.

Associated conditions

- trauma
- hemoperitoneum
- Morrison pouch
- unstable patient
- acute emergency presentation
- mimic on imaging
- operator or probe-position artifact

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
ascites	Free fluid is diffuse and non-traumatic.	Correlate with chronic liver disease and FAST context.
cystic structures	Trace anatomy in two planes.	Cysts have walls and fixed organ relationships.
pericardial fluid	Subxiphoid cardiac view distinguishes pericardial fluid from intra-abdominal free fluid.	Use the trauma endpoint to decide whether pericardial fluid explains the clip.

Clinical examination points

- FAST scan: free fluid: Confirm the correct probe view, look for dynamic movement, repeat in orthogonal planes, and correlate with hemodynamics.
- FAST scan: free fluid: Start with ABCs, oxygenation/ventilation, circulation, monitoring, and escalation triggers.

NEET PG exam angle

FAST scan: free fluid is usually tested through trauma; hemoperitoneum; Morrison pouch. Separate it from ascites, cystic structures, pericardial fluid.

Management relevance

For FAST scan: free fluid, stabilize airway, breathing, circulation, and hemodynamics before definitive treatment for trauma.

Common traps

- Do not confuse with ascites: Free fluid is diffuse and non-traumatic.
- Do not confuse with cystic structures: Trace anatomy in two planes.
- Do not confuse with pericardial fluid: Subxiphoid cardiac view distinguishes pericardial fluid from intra-abdominal free fluid.
- FAST scan: free fluid needs both trauma and hemoperitoneum; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: FAST scan: free fluid.
- Subject: Surgery / Emergency / Radiology.
- Key clue: trauma.
- Key clue: hemoperitoneum.
- Key clue: Morrison pouch.
- Closest mimics: ascites, cystic structures, pericardial fluid.
- Video segment: 00:00-01:00.
- One-line recall: FAST scan: free fluid points to trauma.

Source credit: Spotting Fluid on Ultrasound: FAST RUQ Edition-Focused Assessment with Sonography in Trauma Sample - Gulfcoast Ultrasound Institute.

Topic 38 - Lung ultrasound: pneumothorax / lung sliding

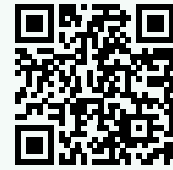
Subject: Radiology / Emergency / Anesthesia

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:20

Source credit: Lung Ultrasound: Absent Lung Sliding #pocus #ultrasound #lungultrasound #pneumothorax #lungsliding - pocusstuff



What it is

Lung ultrasound: pneumothorax / lung sliding: lung sliding, absent lung sliding, lung point. High-yield anchors: pneumothorax; eFAST; pleural line; M-mode.

How to identify it in the video

lung sliding, absent lung sliding, lung point. Watch for pneumothorax, eFAST, pleural line, M-mode. Video cues for Lung ultrasound: pneumothorax / lung sliding: probe view and anatomic landmark, moving structure or compressibility, fluid, shadowing, or artifact, dynamic change with pressure or respiration.

Why it happens

The mechanism of Lung ultrasound: pneumothorax / lung sliding starts with pneumothorax and is expressed clinically through eFAST. Related exam anchors are pleural line, M-mode. In Lung ultrasound: pneumothorax / lung sliding, the decisive clue is dynamic: compressibility, fluid movement, valve motion, shadowing, or probe position changes the diagnosis.

Associated conditions

- pneumothorax
- eFAST
- pleural line
- M-mode
- acute emergency presentation
- mimic on imaging
- operator or probe-position artifact

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
pleural effusion	Anechoic fluid above the diaphragm with lung floating or compressed.	Spine sign and lung compression help.
lung consolidation	Tissue-like lung with air bronchograms.	Lung sliding may persist.
artifacts	Change probe angle, depth, gain, and view.	Artifacts do not respect anatomy.

Clinical examination points

- Lung ultrasound: pneumothorax / lung sliding: Confirm the correct probe view, look for dynamic movement, repeat in orthogonal planes, and correlate with hemodynamics.
- Lung ultrasound: pneumothorax / lung sliding: Start with ABCs, oxygenation/ventilation, circulation, monitoring, and escalation triggers.

NEET PG exam angle

Lung ultrasound: pneumothorax / lung sliding is usually tested through pneumothorax; eFAST; pleural line. Separate it from pleural effusion, lung consolidation, artifacts.

Management relevance

For Lung ultrasound: pneumothorax / lung sliding, stabilize airway, breathing, circulation, and hemodynamics before definitive treatment for pneumothorax.

Common traps

- Do not confuse with pleural effusion: Anechoic fluid above the diaphragm with lung floating or compressed.
- Do not confuse with lung consolidation: Tissue-like lung with air bronchograms.
- Do not confuse with artifacts: Change probe angle, depth, gain, and view.
- Lung ultrasound: pneumothorax / lung sliding needs both pneumothorax and eFAST; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Lung ultrasound: pneumothorax / lung sliding.
- Subject: Radiology / Emergency / Anesthesia.
- Key clue: pneumothorax.
- Key clue: eFAST.
- Key clue: pleural line.
- Closest mimics: pleural effusion, lung consolidation, artifacts.
- Video segment: 00:00-00:20.
- One-line recall: Lung ultrasound: pneumothorax / lung sliding points to pneumothorax.

Source credit: Lung Ultrasound: Absent Lung Sliding #pocus #ultrasound #lungultrasound #pneumothorax #lungsliding - pocusstuff.

Topic 39 - Echo: pericardial effusion / tamponade

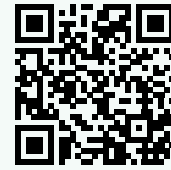
Subject: Medicine / Emergency / Radiology

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:20

Source credit: Echocardiogram, pericardial effusion compressing RV and RA, signs of impending cardiac tamponade - Cureus Journal of Medical Science



What it is

Echo: pericardial effusion / tamponade: echo showing pericardial effusion or tamponade. High-yield anchors: tamponade; right atrial/right ventricular collapse; Beck triad; pericardiocentesis.

How to identify it in the video

echo showing pericardial effusion or tamponade. Watch for tamponade, right atrial/right ventricular collapse, Beck triad, pericardiocentesis. Video cues for Echo: pericardial effusion / tamponade: probe view and anatomic landmark, moving structure or compressibility, fluid, shadowing, or artifact, dynamic change with pressure or respiration.

Why it happens

The mechanism of Echo: pericardial effusion / tamponade starts with tamponade and is expressed clinically through right atrial/right ventricular collapse. Related exam anchors are Beck triad, pericardiocentesis. In Echo: pericardial effusion / tamponade, the decisive clue is dynamic: compressibility, fluid movement, valve motion, shadowing, or probe position changes the diagnosis.

Associated conditions

- tamponade
- right atrial/right ventricular collapse
- Beck triad
- pericardiocentesis
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant
- acute emergency presentation

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
pleural effusion	Anechoic fluid above the diaphragm with lung floating or compressed.	Spine sign and lung compression help.
epicardial fat	Usually anterior.	More echogenic, and moves with the heart unlike effusion.
cardiomyopathy	Chamber dilation or wall motion defect, no effusion.	Echo shows structural change without pericardial fluid or chamber collapse.

Clinical examination points

- Echo: pericardial effusion / tamponade: Confirm the correct probe view, look for dynamic movement, repeat in orthogonal planes, and correlate with hemodynamics.
- Echo: pericardial effusion / tamponade: Start with ABCs, oxygenation/ventilation, circulation, monitoring, and escalation triggers.

NEET PG exam angle

Echo: pericardial effusion / tamponade is usually tested through tamponade; right atrial/right ventricular collapse; Beck triad. Separate it from pleural effusion, epicardial fat, cardiomyopathy.

Management relevance

For Echo: pericardial effusion / tamponade, stabilize airway, breathing, circulation, and hemodynamics before definitive treatment for tamponade.

Common traps

- Do not confuse with pleural effusion: Anechoic fluid above the diaphragm with lung floating or compressed.
- Do not confuse with epicardial fat: Usually anterior.
- Do not confuse with cardiomyopathy: Symmetric proximal weakness, raised CK.
- Echo: pericardial effusion / tamponade needs both tamponade and right atrial/right ventricular collapse; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Echo: pericardial effusion / tamponade.
- Subject: Medicine / Emergency / Radiology.
- Key clue: tamponade.
- Key clue: right atrial/right ventricular collapse.
- Key clue: Beck triad.
- Closest mimics: pleural effusion, epicardial fat, cardiomyopathy.
- Video segment: 00:00-00:20.
- One-line recall: Echo: pericardial effusion / tamponade points to tamponade.

Source credit: Echocardiogram, pericardial effusion compressing RV and RA, signs of impending cardiac tamponade - Cureus Journal of Medical Science.

Topic 40 - Echo: mitral stenosis / valvular lesion

Subject: Medicine / Radiology

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Echocardiographic Evaluation of the Mitral Valve SAMPLE - Gulfcoast Ultrasound Institute



What it is

Echo: mitral stenosis / valvular lesion: echocardiography of rheumatic mitral stenosis. High-yield anchors: rheumatic heart disease; hockey-stick mitral valve; LA enlargement.

How to identify it in the video

echocardiography of rheumatic mitral stenosis. Watch for rheumatic heart disease, hockey-stick mitral valve, LA enlargement. Video cues for Echo: mitral stenosis / valvular lesion: probe view and anatomic landmark, moving structure or compressibility, fluid, shadowing, or artifact, dynamic change with pressure or respiration.

Why it happens

The mechanism of Echo: mitral stenosis / valvular lesion starts with rheumatic heart disease and is expressed clinically through hockey-stick mitral valve. Related exam anchors are LA enlargement. In Echo: mitral stenosis / valvular lesion, the decisive clue is dynamic: compressibility, fluid movement, valve motion, shadowing, or probe position changes the diagnosis.

Associated conditions

- rheumatic heart disease
- hockey-stick mitral valve
- LA enlargement
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant
- acute emergency presentation
- mimic on imaging

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
mitral regurgitation	Color Doppler regurgitant jet and systolic murmur.	Valve does not show classic stenotic doming.
aortic stenosis	Calcified aortic valve and systolic gradient.	Murmur radiates to carotids.
atrial myxoma	Mobile left atrial mass attached to septum.	May prolapse through mitral valve.

Clinical examination points

- Echo: mitral stenosis / valvular lesion: Confirm the correct probe view, look for dynamic movement, repeat in orthogonal planes, and correlate with hemodynamics.

NEET PG exam angle

Echo: mitral stenosis / valvular lesion is usually tested through rheumatic heart disease; hockey-stick mitral valve; LA enlargement. Separate it from mitral regurgitation, aortic stenosis, atrial myxoma.

Management relevance

Clinical action after recognizing Echo: mitral stenosis / valvular lesion: confirm rheumatic heart disease, look for complications, and choose treatment according to the underlying diagnosis.

Common traps

- Do not confuse with mitral regurgitation: Color Doppler regurgitant jet and systolic murmur.
- Do not confuse with aortic stenosis: Calcified aortic valve and systolic gradient.
- Do not confuse with atrial myxoma: Mobile left atrial mass attached to septum.
- Echo: mitral stenosis / valvular lesion needs both rheumatic heart disease and hockey-stick mitral valve; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Echo: mitral stenosis / valvular lesion.
- Subject: Medicine / Radiology.
- Key clue: rheumatic heart disease.
- Key clue: hockey-stick mitral valve.
- Key clue: LA enlargement.
- Closest mimics: mitral regurgitation, aortic stenosis, atrial myxoma.
- Video segment: 00:00-01:00.
- One-line recall: Echo: mitral stenosis / valvular lesion points to rheumatic heart disease.

Source credit: Echocardiographic Evaluation of the Mitral Valve SAMPLE - Gulfcoast Ultrasound Institute.

Topic 41 - Gallbladder ultrasound: stones / cholecystitis

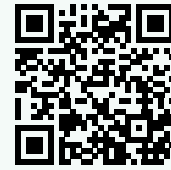
Subject: Surgery / Radiology

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Ultrasound of Gallbladder Stones with Chronic Cholecystitis: Full Clinical Case - ECHOMAP English Ultrasound



What it is

Gallbladder ultrasound: stones / cholecystitis: ultrasound clip showing gallstones or cholecystitis. High-yield anchors: cholelithiasis; acoustic shadowing; sonographic Murphy sign; wall thickening.

How to identify it in the video

ultrasound clip showing gallstones or cholecystitis. Watch for cholelithiasis, acoustic shadowing, sonographic Murphy sign, wall thickening. Video cues for Gallbladder ultrasound: stones / cholecystitis: probe view and anatomic landmark, moving structure or compressibility, fluid, shadowing, or artifact, dynamic change with pressure or respiration.

Why it happens

The mechanism of Gallbladder ultrasound: stones / cholecystitis starts with cholelithiasis and is expressed clinically through acoustic shadowing. Related exam anchors are sonographic Murphy sign, wall thickening. In Gallbladder ultrasound: stones / cholecystitis, the decisive clue is dynamic: compressibility, fluid movement, valve motion, shadowing, or probe position changes the diagnosis.

Associated conditions

- cholelithiasis
- acoustic shadowing
- sonographic Murphy sign
- wall thickening
- acute emergency presentation
- mimic on imaging
- operator or probe-position artifact

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
gallbladder polyp	Fixed, non-shadowing echogenic projection from the wall; does not move or cast a shadow like a stone.	Use the cholelithiasis endpoint to decide whether gallbladder polyp explains the clip.
sludge	Low-level echoes layer dependently without clean acoustic shadowing.	Use the cholelithiasis endpoint to decide whether sludge explains the clip.
porcelain gallbladder	Calcified echogenic wall with shadowing.	Malignancy association matters.

Clinical examination points

- Gallbladder ultrasound: stones / cholecystitis: Confirm the correct probe view, look for dynamic movement, repeat in orthogonal planes, and correlate with hemodynamics.

NEET PG exam angle

Gallbladder ultrasound: stones / cholecystitis is usually tested through cholelithiasis; acoustic shadowing; sonographic Murphy sign. Separate it from gallbladder polyp, sludge, porcelain gallbladder.

Management relevance

Clinical action after recognizing Gallbladder ultrasound: stones / cholecystitis: confirm cholelithiasis, look for complications, and choose treatment according to the underlying diagnosis.

Common traps

- Do not confuse with gallbladder polyp: Usually unilateral pedunculated lesion with voice abuse or acute phonotrauma history.
- Do not confuse with sludge: Low-level echoes layer dependently without clean acoustic shadowing.
- Do not confuse with porcelain gallbladder: Calcified echogenic wall with shadowing.
- Gallbladder ultrasound: stones / cholecystitis needs both cholelithiasis and acoustic shadowing; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Gallbladder ultrasound: stones / cholecystitis.
- Subject: Surgery / Radiology.
- Key clue: cholelithiasis.
- Key clue: acoustic shadowing.
- Key clue: sonographic Murphy sign.
- Closest mimics: gallbladder polyp, sludge, porcelain gallbladder.
- Video segment: 00:00-01:00.
- One-line recall: Gallbladder ultrasound: stones / cholecystitis points to cholelithiasis.

Source credit: Ultrasound of Gallbladder Stones with Chronic Cholecystitis: Full Clinical Case - ECHOMAP English Ultrasound.

Topic 42 - Hydronephrosis ultrasound

Subject: Surgery / Radiology

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:20

Source credit: Mild hydronephrosis | ultrasound case - Radiology Practice



What it is

Hydronephrosis ultrasound: renal ultrasound showing hydronephrosis. High-yield anchors: renal pelvis dilation; obstruction; renal colic; grading.

How to identify it in the video

renal ultrasound showing hydronephrosis. Watch for renal pelvis dilation, obstruction, renal colic, grading. Video cues for Hydronephrosis ultrasound: probe view and anatomic landmark, moving structure or compressibility, fluid, shadowing, or artifact, dynamic change with pressure or respiration.

Why it happens

The mechanism of Hydronephrosis ultrasound starts with renal pelvis dilation and is expressed clinically through obstruction. Related exam anchors are renal colic, grading. In Hydronephrosis ultrasound, the decisive clue is dynamic: compressibility, fluid movement, valve motion, shadowing, or probe position changes the diagnosis.

Associated conditions

- renal pelvis dilation
- obstruction
- renal colic
- grading
- acute emergency presentation
- mimic on imaging
- operator or probe-position artifact

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
renal cyst	Round anechoic lesion with posterior enhancement, not branching collecting-system dilation.	Use the renal pelvis dilation endpoint to decide whether renal cyst explains the clip.
parapelvic cyst	Cystic lesion near sinus but not connected to calyces.	CT/urogram clarifies.
extrarenal pelvis	Pelvic fullness without calyceal dilation or cortical thinning.	Use the renal pelvis dilation endpoint to decide whether extrarenal pelvis explains the clip.

Clinical examination points

- Hydronephrosis ultrasound: Confirm the correct probe view, look for dynamic movement, repeat in orthogonal planes, and correlate with hemodynamics.

NEET PG exam angle

Hydronephrosis ultrasound is usually tested through renal pelvis dilation; obstruction; renal colic. Separate it from renal cyst, parapelvic cyst, extrarenal pelvis.

Management relevance

Clinical action after recognizing Hydronephrosis ultrasound: confirm renal pelvis dilation, look for complications, and choose treatment according to the underlying diagnosis.

Common traps

- Do not confuse with renal cyst: Round anechoic lesion with posterior enhancement, not branching collecting-system dilation.
- Do not confuse with parapelvic cyst: Cystic lesion near sinus but not connected to calyces.
- Do not confuse with extrarenal pelvis: Pelvic fullness without calyceal dilation or cortical thinning.
- Hydronephrosis ultrasound needs both renal pelvis dilation and obstruction; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Hydronephrosis ultrasound.
- Subject: Surgery / Radiology.
- Key clue: renal pelvis dilation.
- Key clue: obstruction.
- Key clue: renal colic.
- Closest mimics: renal cyst, parapelvic cyst, extrarenal pelvis.
- Video segment: 00:00-00:20.
- One-line recall: Hydronephrosis ultrasound points to renal pelvis dilation.

Source credit: Mild hydronephrosis | ultrasound case - Radiology Practice.

Topic 43 - DVT compression ultrasound

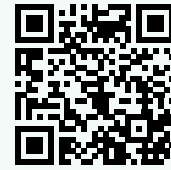
Subject: Medicine / Surgery / Radiology

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Deep Vein Thrombosis (DVT) - Ultrasound Scanning Technique - Clarius Mobile Health



What it is

DVT compression ultrasound: non-compressible vein on compression ultrasound. High-yield anchors: DVT diagnosis; compression ultrasound; anticoagulation.

How to identify it in the video

non-compressible vein on compression ultrasound. Watch for DVT diagnosis, compression ultrasound, anticoagulation. Video cues for DVT compression ultrasound: probe view and anatomic landmark, moving structure or compressibility, fluid, shadowing, or artifact, dynamic change with pressure or respiration.

Why it happens

The mechanism of DVT compression ultrasound starts with DVT diagnosis and is expressed clinically through compression ultrasound. Related exam anchors are anticoagulation. In DVT compression ultrasound, the decisive clue is dynamic: compressibility, fluid movement, valve motion, shadowing, or probe position changes the diagnosis.

Associated conditions

- DVT diagnosis
- compression ultrasound
- anticoagulation
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant
- acute emergency presentation
- mimic on imaging

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
Baker cyst	Popliteal cystic swelling.	Ultrasound shows cyst rather than non-compressible vein.
superficial thrombophlebitis	Tender superficial cord.	Deep veins remain compressible.
lymphadenopathy	Nodal mass has hilum/vascularity and is separate from venous lumen.	Use the DVT diagnosis endpoint to decide whether lymphadenopathy explains the clip.

Clinical examination points

- DVT compression ultrasound: Confirm the correct probe view, look for dynamic movement, repeat in orthogonal planes, and correlate with hemodynamics.

NEET PG exam angle

DVT compression ultrasound is usually tested through DVT diagnosis; compression ultrasound; anticoagulation. Separate it from Baker cyst, superficial thrombophlebitis, lymphadenopathy.

Management relevance

Compression ultrasound diagnosis is followed by anticoagulation unless contraindicated.

Common traps

- Do not confuse with Baker cyst: Popliteal cystic swelling.
- Do not confuse with superficial thrombophlebitis: Tender superficial cord.
- Do not confuse with lymphadenopathy: Nodal mass has hilum/vascularity and is separate from venous lumen.
- DVT compression ultrasound needs both DVT diagnosis and compression ultrasound; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: DVT compression ultrasound.
- Subject: Medicine / Surgery / Radiology.
- Key clue: DVT diagnosis.
- Key clue: compression ultrasound.
- Key clue: anticoagulation.
- Closest mimics: Baker cyst, superficial thrombophlebitis, lymphadenopathy.
- Video segment: 00:00-01:00.
- One-line recall: DVT compression ultrasound points to DVT diagnosis.

Source credit: Deep Vein Thrombosis (DVT) - Ultrasound Scanning Technique - Clarius Mobile Health.

Topic 44 - Obstetric ultrasound: ectopic pregnancy

Subject: OBGY / Radiology

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Ectopic Pregnancy || Ultrasound || Case 151 - Imaging Study



What it is

Obstetric ultrasound: ectopic pregnancy: ultrasound showing ectopic pregnancy features. High-yield anchors: empty uterus; adnexal mass; beta-hCG; management.

How to identify it in the video

ultrasound showing ectopic pregnancy features. Watch for empty uterus, adnexal mass, beta-hCG, management. Video cues for Obstetric ultrasound: ectopic pregnancy: probe view and anatomic landmark, moving structure or compressibility, fluid, shadowing, or artifact, dynamic change with pressure or respiration.

Why it happens

The mechanism of Obstetric ultrasound: ectopic pregnancy starts with empty uterus and is expressed clinically through adnexal mass. Related exam anchors are beta-hCG, management. In Obstetric ultrasound: ectopic pregnancy, the decisive clue is dynamic: compressibility, fluid movement, valve motion, shadowing, or probe position changes the diagnosis.

Associated conditions

- empty uterus
- adnexal mass
- beta-hCG
- management
- pregnancy-related emergency
- procedure-related complication
- maternal-fetal safety issue
- acute emergency presentation

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
early intrauterine pregnancy	Look for intrauterine gestational sac/yolk sac and serial beta-hCG trend.	Use the empty uterus endpoint to decide whether early intrauterine pregnancy explains the clip.
corpus luteum cyst	Ovarian ring of fire can mimic ectopic.	Identify intrauterine pregnancy and adnexal relation.
miscarriage	Open cervix.	Falling beta-hCG, and intrauterine products distinguish pregnancy loss.

Clinical examination points

- Obstetric ultrasound: ectopic pregnancy: Confirm the correct probe view, look for dynamic movement, repeat in orthogonal planes, and correlate with hemodynamics.

NEET PG exam angle

Obstetric ultrasound: ectopic pregnancy is usually tested through empty uterus; adnexal mass; beta-hCG. Separate it from early intrauterine pregnancy, corpus luteum cyst, miscarriage.

Management relevance

Unstable ectopic pregnancy needs urgent surgery; stable selected cases may receive methotrexate.

Common traps

- Do not confuse with early intrauterine pregnancy: Look for intrauterine gestational sac/yolk sac and serial beta-hCG trend.
- Do not confuse with corpus luteum cyst: Ovarian ring of fire can mimic ectopic.
- Do not confuse with miscarriage: Open cervix.
- Obstetric ultrasound: ectopic pregnancy needs both empty uterus and adnexal mass; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Obstetric ultrasound: ectopic pregnancy.
- Subject: OBGY / Radiology.
- Key clue: empty uterus.
- Key clue: adnexal mass.
- Key clue: beta-hCG.
- Closest mimics: early intrauterine pregnancy, corpus luteum cyst, miscarriage.
- Video segment: 00:00-01:00.
- One-line recall: Obstetric ultrasound: ectopic pregnancy points to empty uterus.

Source credit: Ectopic Pregnancy || Ultrasound || Case 151 - Imaging Study.

Topic 45 - Obstetric ultrasound: fetal cardiac activity

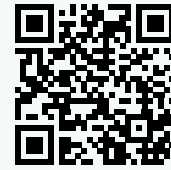
Subject: OBGY / Radiology

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:46

Source credit: How to Measure a Fetal Heart Rate in the 1st Trimester - MetroHealth Emergency Ultrasound



What it is

Obstetric ultrasound: fetal cardiac activity: fetal heartbeat on early pregnancy ultrasound. High-yield anchors: viability; fetal cardiac activity; early pregnancy scan.

How to identify it in the video

fetal heartbeat on early pregnancy ultrasound. Watch for viability, fetal cardiac activity, early pregnancy scan. Video cues for Obstetric ultrasound: fetal cardiac activity: probe view and anatomic landmark, moving structure or compressibility, fluid, shadowing, or artifact, dynamic change with pressure or respiration.

Why it happens

The mechanism of Obstetric ultrasound: fetal cardiac activity starts with viability and is expressed clinically through fetal cardiac activity. Related exam anchors are early pregnancy scan. In Obstetric ultrasound: fetal cardiac activity, the decisive clue is dynamic: compressibility, fluid movement, valve motion, shadowing, or probe position changes the diagnosis.

Associated conditions

- viability
- fetal cardiac activity
- early pregnancy scan
- pregnancy-related emergency
- procedure-related complication
- maternal-fetal safety issue
- acute emergency presentation
- mimic on imaging

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
missed abortion	Absent cardiac activity at diagnostic CRL threshold.	Repeat scan if below threshold.
early pregnancy loss	Use CRL/MSD criteria and repeat ultrasound timing to avoid false diagnosis.	Use the viability endpoint to decide whether early pregnancy loss explains the clip.
blighted ovum	Empty gestational sac above MSD threshold.	Repeat ultrasound if uncertain.

Clinical examination points

- Obstetric ultrasound: fetal cardiac activity: Confirm the correct probe view, look for dynamic movement, repeat in orthogonal planes, and correlate with hemodynamics.

NEET PG exam angle

Obstetric ultrasound: fetal cardiac activity is usually tested through viability; fetal cardiac activity; early pregnancy scan. Separate it from missed abortion, early pregnancy loss, blighted ovum.

Management relevance

Clinical action after recognizing Obstetric ultrasound: fetal cardiac activity: confirm viability, look for complications, and choose treatment according to the underlying diagnosis.

Common traps

- Do not confuse with missed abortion: Absent cardiac activity at diagnostic CRL threshold.
- Do not confuse with early pregnancy loss: Use CRL/MSD criteria and repeat ultrasound timing to avoid false diagnosis.
- Do not confuse with blighted ovum: Empty gestational sac above MSD threshold.
- Obstetric ultrasound: fetal cardiac activity needs both viability and fetal cardiac activity; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Obstetric ultrasound: fetal cardiac activity.
- Subject: OBGY / Radiology.
- Key clue: viability.
- Key clue: fetal cardiac activity.
- Key clue: early pregnancy scan.
- Closest mimics: missed abortion, early pregnancy loss, blighted ovum.
- Video segment: 00:00-00:46.
- One-line recall: Obstetric ultrasound: fetal cardiac activity points to viability.

Source credit: How to Measure a Fetal Heart Rate in the 1st Trimester - MetroHealth Emergency Ultrasound.

Topic 46 - Neonatal reflexes: Moro / rooting / grasp

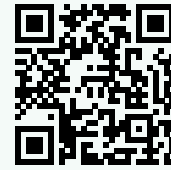
Subject: Pediatrics

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Assessing Newborn Primitive Reflexes - OPENPediatrics



What it is

Neonatal reflexes: Moro / rooting / grasp: direct neonatal reflex demonstration. High-yield anchors: primitive reflexes; developmental assessment; disappearance age.

How to identify it in the video

direct neonatal reflex demonstration. Watch for primitive reflexes, developmental assessment, disappearance age. Video cues for Neonatal reflexes: Moro / rooting / grasp: the visible endpoint of Neonatal reflexes: Moro / rooting / grasp, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Neonatal reflexes: Moro / rooting / grasp starts with primitive reflexes and is expressed clinically through developmental assessment. Related exam anchors are disappearance age.

Associated conditions

- primitive reflexes
- developmental assessment
- disappearance age
- developmental age-related normal variant
- congenital or inherited disorder
- acute infection or emergency presentation

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
seizure	Stereotyped event, impaired awareness.	Postictal state, tongue bite, and EEG support seizure.
startle	Stimulus-linked single startle follows noise or touch.	No clustering, regression, or hypsarrhythmia.
abnormal persistence	Primitive reflex persisting beyond expected age suggests UMN/developmental pathology.	Use the primitive reflexes endpoint to decide whether abnormal persistence explains the clip.

Clinical examination points

- For Neonatal reflexes: Moro / rooting / grasp, document the visible abnormality before naming the diagnosis.
- For Neonatal reflexes: Moro / rooting / grasp, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Neonatal reflexes: Moro / rooting / grasp is usually tested through primitive reflexes; developmental assessment; disappearance age. Separate it from seizure, startle, abnormal persistence.

Management relevance

Clinical action after recognizing Neonatal reflexes: Moro / rooting / grasp: confirm primitive reflexes, look for complications, and choose treatment according to the underlying diagnosis.

Common traps

- Do not confuse with seizure: Stereotyped event, impaired awareness.
- Do not confuse with startle: Stimulus-linked single startle follows noise or touch.
- Do not confuse with abnormal persistence: Primitive reflex persisting beyond expected age suggests UMN/developmental pathology.
- Neonatal reflexes: Moro / rooting / grasp needs both primitive reflexes and developmental assessment; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Neonatal reflexes: Moro / rooting / grasp.
- Subject: Pediatrics.
- Key clue: primitive reflexes.
- Key clue: developmental assessment.
- Key clue: disappearance age.
- Closest mimics: seizure, startle, abnormal persistence.
- Video segment: 00:00-01:00.
- One-line recall: Neonatal reflexes: Moro / rooting / grasp points to primitive reflexes.

Source credit: Assessing Newborn Primitive Reflexes - OPENPediatrics.

Topic 47 - Pediatric respiratory distress

Subject: Pediatrics / Emergency

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:18

Source credit: Pediatric Retractions 3 - JETem



What it is

Pediatric respiratory distress: retractions, nasal flaring, grunting. High-yield anchors: respiratory distress signs; bronchiolitis; pneumonia; severity assessment.

How to identify it in the video

retractions, nasal flaring, grunting. Watch for respiratory distress signs, bronchiolitis, pneumonia, severity assessment. Video cues for Pediatric respiratory distress: the visible endpoint of Pediatric respiratory distress, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Pediatric respiratory distress starts with respiratory distress signs and is expressed clinically through bronchiolitis. Related exam anchors are pneumonia, severity assessment.

Associated conditions

- respiratory distress signs
- bronchiolitis
- pneumonia
- severity assessment
- developmental age-related normal variant
- congenital or inherited disorder
- acute infection or emergency presentation

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
croup	Barking cough and inspiratory stridor.	Steeple sign may support.
asthma	Expiratory wheeze and prolonged expiration dominate rather than stridor.	Use the respiratory distress signs endpoint to decide whether asthma explains the clip.
foreign body aspiration	Sudden choking, asymmetric breath sounds.	Air trapping, and bronchoscopy clue.

Clinical examination points

- Pediatric respiratory distress: Start with ABCs, oxygenation/ventilation, circulation, monitoring, and escalation triggers.

NEET PG exam angle

Pediatric respiratory distress is usually tested through respiratory distress signs; bronchiolitis; pneumonia. Separate it from croup, asthma, foreign body aspiration.

Management relevance

For Pediatric respiratory distress, stabilize airway, breathing, circulation, and hemodynamics before definitive treatment for respiratory distress signs.

Common traps

- Do not confuse with croup: Barking cough and inspiratory stridor.
- Do not confuse with asthma: Expiratory wheeze and prolonged expiration dominate rather than stridor.
- Do not confuse with foreign body aspiration: Sudden choking, asymmetric breath sounds.
- Pediatric respiratory distress needs both respiratory distress signs and bronchiolitis; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Pediatric respiratory distress.
- Subject: Pediatrics / Emergency.
- Key clue: respiratory distress signs.
- Key clue: bronchiolitis.
- Key clue: pneumonia.
- Closest mimics: croup, asthma, foreign body aspiration.
- Video segment: 00:00-00:18.
- One-line recall: Pediatric respiratory distress points to respiratory distress signs.

Source credit: Pediatric Retractions 3 - JETem.

Topic 48 - Croup stridor

Subject: Pediatrics / ENT

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:36

Source credit: Croup - Healthier Together



What it is

Croup stridor: barking cough or inspiratory stridor. High-yield anchors: croup; steeple sign; nebulized epinephrine; steroids.

How to identify it in the video

barking cough or inspiratory stridor. Watch for croup, steeple sign, nebulized epinephrine, steroids. Video cues for Croup stridor: the visible endpoint of Croup stridor, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Croup stridor starts with croup and is expressed clinically through steeple sign. Related exam anchors are nebulized epinephrine, steroids.

Associated conditions

- croup
- steeple sign
- nebulized epinephrine
- steroids
- developmental age-related normal variant
- congenital or inherited disorder
- acute infection or emergency presentation

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
epiglottitis	Toxic child, drooling, tripod posture.	Avoid throat examination and secure airway.
foreign body aspiration	Sudden choking, asymmetric breath sounds.	Air trapping, and bronchoscopy clue.
laryngomalacia	Chronic positional infant stridor since early infancy, worse supine/feeding.	Use the croup endpoint to decide whether laryngomalacia explains the clip.

Clinical examination points

- For Croup stridor, document the visible abnormality before naming the diagnosis.
- For Croup stridor, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Croup stridor is usually tested through croup; steeple sign; nebulized epinephrine. Separate it from epiglottitis, foreign body aspiration, laryngomalacia.

Management relevance

Steroids for all clinically significant croup; nebulized epinephrine for moderate-severe distress.

Common traps

- Do not confuse with epiglottitis: Toxic child, drooling, tripod posture.
- Do not confuse with foreign body aspiration: Sudden choking, asymmetric breath sounds.
- Do not confuse with laryngomalacia: Chronic positional infant stridor since early infancy, worse supine/feeding.
- Croup stridor needs both croup and steeple sign; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Croup stridor.
- Subject: Pediatrics / ENT.
- Key clue: croup.
- Key clue: steeple sign.
- Key clue: nebulized epinephrine.
- Closest mimics: epiglottitis, foreign body aspiration, laryngomalacia.
- Video segment: 00:00-00:36.
- One-line recall: Croup stridor points to croup.

Source credit: Croup - Healthier Together.

Topic 49 - Bag-mask ventilation / airway positioning

Subject: Anesthesia / Emergency

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Bag-Valve-Mask Ventilation Technique - Medscape



What it is

Bag-mask ventilation / airway positioning: BVM technique or airway opening maneuver. High-yield anchors: head tilt-chin lift; jaw thrust; mask seal; ventilation.

How to identify it in the video

BVM technique or airway opening maneuver. Watch for head tilt-chin lift, jaw thrust, mask seal, ventilation. Video cues for Bag-mask ventilation / airway positioning: patient or mannequin position, hand and instrument placement, sequence of steps, endpoint and safety checks.

Why it happens

The mechanism of Bag-mask ventilation / airway positioning starts with head tilt-chin lift and is expressed clinically through jaw thrust. Related exam anchors are mask seal, ventilation. In Bag-mask ventilation / airway positioning, the outcome depends on sequence, hand position, endpoint recognition, and avoiding the unsafe step.

Associated conditions

- head tilt-chin lift
- jaw thrust
- mask seal
- ventilation

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
intubation	Endotracheal tube placement with capnography.	Different from bag-mask ventilation.
LMA insertion	Supraglottic airway device.	Does not pass through vocal cords.
CPR compressions	Compression depth/rate and pulse check, not airway seal or ventilation technique.	Use the head tilt-chin lift endpoint to decide whether CPR compressions explains the clip.

Clinical examination points

- Bag-mask ventilation / airway positioning: Start with ABCs, oxygenation/ventilation, circulation, monitoring, and escalation triggers.

NEET PG exam angle

Bag-mask ventilation / airway positioning is usually tested through head tilt-chin lift; jaw thrust; mask seal. Separate it from intubation, LMA insertion, CPR compressions.

Management relevance

Correct airway position, mask seal, and ventilation before escalating to advanced airway.

Common traps

- Do not confuse with intubation: Endotracheal tube placement with capnography.
- Do not confuse with LMA insertion: Supraglottic airway device.

- Do not confuse with CPR compressions: Compression depth/rate and pulse check, not airway seal or ventilation technique.
- Safety trap in Bag-mask ventilation / airway positioning: sequence, hand position, and endpoint matter as much as naming the step.

Quick revision box

- Topic: Bag-mask ventilation / airway positioning.
- Subject: Anesthesia / Emergency.
- Key clue: head tilt-chin lift.
- Key clue: jaw thrust.
- Key clue: mask seal.
- Closest mimics: intubation, LMA insertion, CPR compressions.
- Video segment: 00:00-01:00.
- One-line recall: Bag-mask ventilation / airway positioning points to head tilt-chin lift.

Source credit: Bag-Valve-Mask Ventilation Technique - Medscape.

Topic 50 - CPR chest compressions / defibrillation sequence

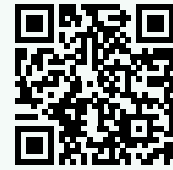
Subject: Emergency Medicine / Anesthesia

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: How to use an AED - Geeky Medics



What it is

CPR chest compressions / defibrillation sequence: BLS/ACLS demonstration. High-yield anchors: compression rate/depth; shockable rhythm; defibrillation sequence.

How to identify it in the video

BLS/ACLS demonstration. Watch for compression rate/depth, shockable rhythm, defibrillation sequence. Video cues for CPR chest compressions / defibrillation sequence: patient or mannequin position, hand and instrument placement, sequence of steps, endpoint and safety checks.

Why it happens

The mechanism of CPR chest compressions / defibrillation sequence starts with compression rate/depth and is expressed clinically through shockable rhythm. Related exam anchors are defibrillation sequence. In CPR chest compressions / defibrillation sequence, the outcome depends on sequence, hand position, endpoint recognition, and avoiding the unsafe step.

Associated conditions

- compression rate/depth
- shockable rhythm
- defibrillation sequence
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
synchronized cardioversion	Uses sync mode for organized tachyarrhythmia with pulse.	Use the compression rate/depth endpoint to decide whether synchronized cardioversion explains the clip.
rescue breathing	Ventilation-only support when pulse is present.	Compression decision depends on pulse/HR.
neonatal CPR	Start compressions only if HR remains below 60/min after effective ventilation.	Use the compression rate/depth endpoint to decide whether neonatal CPR explains the clip.

Clinical examination points

- CPR chest compressions / defibrillation sequence: Start with ABCs, oxygenation/ventilation, circulation, monitoring, and escalation triggers.

NEET PG exam angle

CPR chest compressions / defibrillation sequence is usually tested through compression rate/depth; shockable rhythm; defibrillation sequence. Separate it from synchronized cardioversion, rescue breathing, neonatal CPR.

Management relevance

High-quality compressions, rhythm check, defibrillation for VF/pulseless VT, and minimize pauses.

Common traps

- Do not confuse with synchronized cardioversion: Uses sync mode for organized tachyarrhythmia with pulse.
- Do not confuse with rescue breathing: Ventilation-only support when pulse is present.
- Do not confuse with neonatal CPR: Start compressions only if HR remains below 60/min after effective ventilation.
- Safety trap in CPR chest compressions / defibrillation sequence: sequence, hand position, and endpoint matter as much as naming the step.

Quick revision box

- Topic: CPR chest compressions / defibrillation sequence.
- Subject: Emergency Medicine / Anesthesia.
- Key clue: compression rate/depth.
- Key clue: shockable rhythm.
- Key clue: defibrillation sequence.
- Closest mimics: synchronized cardioversion, rescue breathing, neonatal CPR.
- Video segment: 00:00-01:00.
- One-line recall: CPR chest compressions / defibrillation sequence points to compression rate/depth.

Source credit: How to use an AED - Geeky Medics.

Topic 51 - Myasthenia gravis: fatigable ptosis

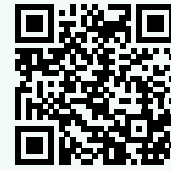
Subject: Medicine / Neurology

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:25

Source credit: Fatigue test - Ocular Myasthenia GRAVIS - Clinical Peanuts in Ophthalmology



What it is

Myasthenia gravis: fatigable ptosis: short video showing ptosis worsening on sustained upgaze or fatigability. High-yield anchors: fatigable weakness; ocular MG; ice-pack test; ACh receptor antibodies; thymoma; myasthenic crisis.

How to identify it in the video

short video showing ptosis worsening on sustained upgaze or fatigability. Watch for fatigable weakness, ocular MG, ice-pack test, ACh receptor antibodies. Video cues for Myasthenia gravis: fatigable ptosis: pupil size and symmetry, direction of eye deviation or movement limitation, ptosis or lid asymmetry, nystagmus direction, light reflex or staining pattern.

Why it happens

The mechanism of Myasthenia gravis: fatigable ptosis starts with fatigable weakness and is expressed clinically through ocular MG. Related exam anchors are ice-pack test, ACh receptor antibodies, thymoma, myasthenic crisis. In Myasthenia gravis: fatigable ptosis, the bedside pattern localizes the relevant afferent, efferent, ocular motor, sympathetic, vestibular, or cerebellar pathway.

Associated conditions

- fatigable weakness
- ocular MG
- ice-pack test
- ACh receptor antibodies
- thymoma
- myasthenic crisis
- stroke or structural CNS lesion
- metabolic/toxic disturbance

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
third nerve palsy	Ptosis with down-and-out eye; pupil involvement suggests compressive aneurysm until proven otherwise.	Down-and-out eye, and pupil involvement require urgent aneurysm imaging.
Horner syndrome	Mild ptosis, miosis, and possible anhidrosis from sympathetic pathway interruption.	Test anisocoria in dim light and consider carotid/Pancoast causes.
congenital ptosis	Present since childhood.	Poor levator function, no diplopia or pupil abnormality.
chronic progressive external ophthalmoplegia	Slow bilateral ophthalmoplegia with ptosis and mitochondrial clues.	Use the fatigable weakness endpoint to decide whether chronic progressive external ophthalmoplegia explains the clip.

Clinical examination points

- Myasthenia gravis: fatigable ptosis: Check pupils, ocular alignment, extraocular movements, ptosis, diplopia direction, and visual fields.
- Myasthenia gravis: fatigable ptosis: Localize afferent and efferent pathways before naming the lesion.

NEET PG exam angle

Myasthenia gravis: fatigable ptosis is usually tested through fatigable weakness; ocular MG; ice-pack test. Separate it from third nerve palsy, Horner syndrome, congenital ptosis.

Management relevance

Clinical action after recognizing Myasthenia gravis: fatigable ptosis: confirm fatigable weakness, look for complications, and choose treatment according to the underlying diagnosis.

Common traps

- Do not confuse with third nerve palsy: Ptosis with down-and-out eye; pupil involvement suggests compressive aneurysm until proven otherwise.
- Do not confuse with Horner syndrome: Mild ptosis, miosis, and possible anhidrosis from sympathetic pathway interruption.
- Do not confuse with congenital ptosis: Present since childhood.
- Myasthenia gravis: fatigable ptosis needs both fatigable weakness and ocular MG; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Myasthenia gravis: fatigable ptosis.
- Subject: Medicine / Neurology.
- Key clue: fatigable weakness.
- Key clue: ocular MG.
- Key clue: ice-pack test.
- Closest mimics: third nerve palsy, Horner syndrome, congenital ptosis, chronic progressive external ophthalmoplegia.
- Video segment: 00:00-00:25.
- One-line recall: Myasthenia gravis: fatigable ptosis points to fatigable weakness.

Source credit: Fatigue test - Ocular Myasthenia GRAVIS - Clinical Peanuts in Ophthalmology.

Topic 52 - Bell's palsy / LMN facial nerve palsy

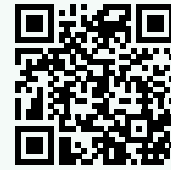
Subject: Medicine / Neurology

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Bell's Palsy (One Sided Facial Paralysis) - Fauquier ENT



What it is

Bell's palsy / LMN facial nerve palsy: direct demonstration of complete unilateral facial weakness. High-yield anchors: forehead involvement; inability to close eye; Bell's phenomenon; corneal protection.

How to identify it in the video

direct demonstration of complete unilateral facial weakness. Watch for forehead involvement, inability to close eye, Bell's phenomenon, corneal protection. Video cues for Bell's palsy / LMN facial nerve palsy: pupil size and symmetry, direction of eye deviation or movement limitation, ptosis or lid asymmetry, nystagmus direction, light reflex or staining pattern.

Why it happens

The mechanism of Bell's palsy / LMN facial nerve palsy starts with forehead involvement and is expressed clinically through inability to close eye. Related exam anchors are Bell's phenomenon, corneal protection. In Bell's palsy / LMN facial nerve palsy, the bedside pattern localizes the relevant afferent, efferent, ocular motor, sympathetic, vestibular, or cerebellar pathway.

Associated conditions

- forehead involvement
- inability to close eye
- Bell's phenomenon
- corneal protection
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
UMN facial palsy	Forehead sparing with contralateral lower facial weakness points to supranuclear lesion.	Use the forehead involvement endpoint to decide whether UMN facial palsy explains the clip.
stroke	Acute focal deficit.	CT/MRI brain and vascular risk profile guide emergency care.
parotid lesion	Facial weakness with parotid mass or pain suggests extracranial facial nerve involvement.	Use the forehead involvement endpoint to decide whether parotid lesion explains the clip.
Ramsay Hunt syndrome	Facial palsy with painful vesicles in the ear canal or pinna, often with hearing loss and vertigo.	Use the forehead involvement endpoint to decide whether Ramsay Hunt syndrome explains the clip.

Clinical examination points

- Bell's palsy / LMN facial nerve palsy: Check pupils, ocular alignment, extraocular movements, ptosis, diplopia direction, and visual fields.
- Bell's palsy / LMN facial nerve palsy: Localize afferent and efferent pathways before naming the lesion.

NEET PG exam angle

Bell's palsy / LMN facial nerve palsy is usually tested through forehead involvement; inability to close eye; Bell's phenomenon. Separate it from UMN facial palsy, stroke, parotid lesion.

Management relevance

Clinical action after recognizing Bell's palsy / LMN facial nerve palsy: confirm forehead involvement, look for complications, and choose treatment according to the underlying diagnosis.

Common traps

- Do not confuse with UMN facial palsy: Forehead sparing with contralateral lower facial weakness points to supranuclear lesion.
- Do not confuse with stroke: Acute focal deficit.
- Do not confuse with parotid lesion: Facial weakness with parotid mass or pain suggests extracranial facial nerve involvement.
- Bell's palsy / LMN facial nerve palsy needs both forehead involvement and inability to close eye; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Bell's palsy / LMN facial nerve palsy.
- Subject: Medicine / Neurology.
- Key clue: forehead involvement.
- Key clue: inability to close eye.
- Key clue: Bell's phenomenon.
- Closest mimics: UMN facial palsy, stroke, parotid lesion, Ramsay Hunt syndrome.
- Video segment: 00:00-01:00.
- One-line recall: Bell's palsy / LMN facial nerve palsy points to forehead involvement.

Source credit: Bell's Palsy (One Sided Facial Paralysis) - Fauquier ENT.

Topic 53 - UMN facial palsy

Subject: Medicine / Neurology

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Bell Palsy vs Stroke - EM:RAP Medical Education



What it is

UMN facial palsy: video showing forehead sparing with lower facial weakness. High-yield anchors: contralateral lower facial weakness; forehead sparing; corticobulbar tract.

How to identify it in the video

video showing forehead sparing with lower facial weakness. Watch for contralateral lower facial weakness, forehead sparing, corticobulbar tract. Video cues for UMN facial palsy: pupil size and symmetry, direction of eye deviation or movement limitation, ptosis or lid asymmetry, nystagmus direction, light reflex or staining pattern.

Why it happens

The mechanism of UMN facial palsy starts with contralateral lower facial weakness and is expressed clinically through forehead sparing. Related exam anchors are corticobulbar tract. In UMN facial palsy, the bedside pattern localizes the relevant afferent, efferent, ocular motor, sympathetic, vestibular, or cerebellar pathway.

Associated conditions

- contralateral lower facial weakness
- forehead sparing
- corticobulbar tract
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
Bell's palsy	LMN whole-face weakness without forehead sparing.	Exclude vesicles/stroke red flags.
brainstem lesion	Crossed signs, gaze palsy.	Ataxia, or long-tract signs localize to brainstem.
facial nerve palsy	Whole ipsilateral face, taste/lacrimation/hyperacusis help localize facial nerve level.	Use the contralateral lower facial weakness endpoint to decide whether facial nerve palsy explains the clip.

Clinical examination points

- UMN facial palsy: Check pupils, ocular alignment, extraocular movements, ptosis, diplopia direction, and visual fields.
- UMN facial palsy: Localize afferent and efferent pathways before naming the lesion.

NEET PG exam angle

UMN facial palsy is usually tested through contralateral lower facial weakness; forehead sparing; corticobulbar tract. Separate it from Bell's palsy, brainstem lesion, facial nerve palsy.

Management relevance

Clinical action after recognizing UMN facial palsy: confirm contralateral lower facial weakness, look for complications, and choose treatment according to the underlying diagnosis.

Common traps

- Do not confuse with Bell's palsy: LMN whole-face weakness without forehead sparing.
- Do not confuse with brainstem lesion: Crossed signs, gaze palsy.
- Do not confuse with facial nerve palsy: Whole ipsilateral face, taste/lacrimation/hyperacusis help localize facial nerve level.
- UMN facial palsy needs both contralateral lower facial weakness and forehead sparing; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: UMN facial palsy.
- Subject: Medicine / Neurology.
- Key clue: contralateral lower facial weakness.
- Key clue: forehead sparing.
- Key clue: corticobulbar tract.
- Closest mimics: Bell's palsy, brainstem lesion, facial nerve palsy.
- Video segment: 00:00-01:00.
- One-line recall: UMN facial palsy points to contralateral lower facial weakness.

Source credit: Bell Palsy vs Stroke - EM:RAP Medical Education.

Topic 54 - Babinski sign / extensor plantar response

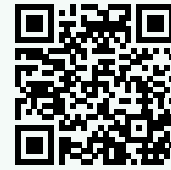
Subject: Medicine / Neurology

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Babinski Sign Positive, Plantar Reflex, #mbbs #medicine #neet #aiims #cns #neetpg - MBBS Physiology - Dr. Waqas Khan



What it is

Babinski sign / extensor plantar response: direct plantar reflex demonstration. High-yield anchors: extensor plantar response; UMN lesion; corticospinal tract.

How to identify it in the video

direct plantar reflex demonstration. Watch for extensor plantar response, UMN lesion, corticospinal tract. Video cues for Babinski sign / extensor plantar response: the visible endpoint of Babinski sign / extensor plantar response, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Babinski sign / extensor plantar response starts with extensor plantar response and is expressed clinically through UMN lesion. Related exam anchors are corticospinal tract.

Associated conditions

- extensor plantar response
- UMN lesion
- corticospinal tract
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
withdrawal response	Whole-leg withdrawal to pain lacks reproducible extensor great-toe response.	Use the extensor plantar response endpoint to decide whether withdrawal response explains the clip.
normal infant plantar response	Extensor plantar can be normal below walking age.	Interpret by age.
equivocal plantar	Repeat with correct lateral-sole stimulus.	Avoid painful over-stimulation.

Clinical examination points

- Babinski sign / extensor plantar response: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Babinski sign / extensor plantar response: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Babinski sign / extensor plantar response is usually tested through extensor plantar response; UMN lesion; corticospinal tract. Separate it from withdrawal response, normal infant plantar response, equivocal plantar.

Management relevance

Use the demonstrated technique only for the correct indication; the management decision hinges on extensor plantar response, patient safety, and the endpoint shown in the video.

Common traps

- Do not confuse with withdrawal response: Whole-leg withdrawal to pain lacks reproducible extensor great-toe response.
- Do not confuse with normal infant plantar response: Extensor plantar can be normal below walking age.
- Do not confuse with equivocal plantar: Repeat with correct lateral-sole stimulus.
- Babinski sign / extensor plantar response needs both extensor plantar response and UMN lesion; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Babinski sign / extensor plantar response.
- Subject: Medicine / Neurology.
- Key clue: extensor plantar response.
- Key clue: UMN lesion.
- Key clue: corticospinal tract.
- Closest mimics: withdrawal response, normal infant plantar response, equivocal plantar.
- Video segment: 00:00-01:00.
- One-line recall: Babinski sign / extensor plantar response points to extensor plantar response.

Source credit: Babinski Sign Positive, Plantar Reflex, #mbbs #medicine #neet #aiims #cns #neetpg - MBBS Physiology - Dr. Waqas Khan.

Topic 55 - Ankle clonus

Subject: Medicine / Neurology

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Ankle Clonus Decoded @DrHardeepPeads #medicine #neetpg #learnmedicine #pediatrics #clinicalsigns - Dr Hardeep Khatri



What it is

Ankle clonus: clonus elicitation at ankle. High-yield anchors: rhythmic contractions; hyperreflexia; UMN lesion; spasticity.

How to identify it in the video

clonus elicitation at ankle. Watch for rhythmic contractions, hyperreflexia, UMN lesion, spasticity. Video cues for Ankle clonus: the visible endpoint of Ankle clonus, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Ankle clonus starts with rhythmic contractions and is expressed clinically through hyperreflexia. Related exam anchors are UMN lesion, spasticity.

Associated conditions

- rhythmic contractions
- hyperreflexia
- UMN lesion
- spasticity
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
tremor	Classify as rest, postural.	Action, or intention before naming the disorder.
voluntary movement	Irregular voluntary movement lacks rhythmic stretch-induced clonus.	Use the rhythmic contractions endpoint to decide whether voluntary movement explains the clip.
rigidity	Velocity-independent resistance without rhythmic beats.	Compare with spasticity/clonus.

Clinical examination points

- For Ankle clonus, document the visible abnormality before naming the diagnosis.
- For Ankle clonus, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Ankle clonus is usually tested through rhythmic contractions; hyperreflexia; UMN lesion. Separate it from tremor, voluntary movement, rigidity.

Management relevance

Clinical action after recognizing Ankle clonus: confirm rhythmic contractions, look for complications, and choose treatment according to the underlying diagnosis.

Common traps

- Do not confuse with tremor: Classify as rest, postural.
- Do not confuse with voluntary movement: Irregular voluntary movement lacks rhythmic stretch-induced clonus.
- Do not confuse with rigidity: Velocity-independent resistance without rhythmic beats.
- Ankle clonus needs both rhythmic contractions and hyperreflexia; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Ankle clonus.
- Subject: Medicine / Neurology.
- Key clue: rhythmic contractions.
- Key clue: hyperreflexia.
- Key clue: UMN lesion.
- Closest mimics: tremor, voluntary movement, rigidity.
- Video segment: 00:00-01:00.
- One-line recall: Ankle clonus points to rhythmic contractions.

Source credit: Ankle Clonus Decoded @DrHardeepPeads #medicine #neetpg #learnmedicine #pediatrics #clinicalsigns - Dr Hardeep Khatri.

Topic 56 - Romberg test

Subject: Medicine / Neurology

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:26

Source credit: Abnormal Sensory Exam : Romberg Test - onlinemedicalvideo



What it is

Romberg test: direct Romberg test demonstration. High-yield anchors: sensory ataxia; posterior column; worsening with eye closure.

How to identify it in the video

direct Romberg test demonstration. Watch for sensory ataxia, posterior column, worsening with eye closure. Video cues for Romberg test: the visible endpoint of Romberg test, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Romberg test starts with sensory ataxia and is expressed clinically through posterior column. Related exam anchors are worsening with eye closure.

Associated conditions

- sensory ataxia
- posterior column
- worsening with eye closure
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
cerebellar ataxia	Dysmetria and nystagmus persist with eyes open.	Romberg is not the main driver.
vestibular ataxia	Veering with vertigo, nausea, and nystagmus rather than limb dysmetria.	Vertigo, nausea, and hearing symptoms guide the diagnosis.
functional imbalance	Inconsistency.	Distractibility, and mismatch between severe sway and preserved function.

Clinical examination points

- Romberg test: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Romberg test: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Romberg test is usually tested through sensory ataxia; posterior column; worsening with eye closure. Separate it from cerebellar ataxia, vestibular ataxia, functional imbalance.

Management relevance

Use the demonstrated technique only for the correct indication; the management decision hinges on sensory ataxia, patient safety, and the endpoint shown in the video.

Common traps

- Do not confuse with cerebellar ataxia: Dysmetria and nystagmus persist with eyes open.
- Do not confuse with vestibular ataxia: Veering with vertigo, nausea, and nystagmus rather than limb dysmetria.
- Do not confuse with functional imbalance: Inconsistency.
- Romberg test needs both sensory ataxia and posterior column; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Romberg test.
- Subject: Medicine / Neurology.
- Key clue: sensory ataxia.
- Key clue: posterior column.
- Key clue: worsening with eye closure.
- Closest mimics: cerebellar ataxia, vestibular ataxia, functional imbalance.
- Video segment: 00:00-00:26.
- One-line recall: Romberg test points to sensory ataxia.

Source credit: Abnormal Sensory Exam : Romberg Test - onlinemedicalvideo.

Topic 57 - Kernig and Brudzinski signs

Subject: Medicine / Neurology

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:20

Source credit: Kernig's sign in meningitis patient... - Learn MBBS



What it is

Kernig and Brudzinski signs: meningeal sign demonstration. High-yield anchors: meningeal irritation; meningitis; subarachnoid hemorrhage.

How to identify it in the video

meningeal sign demonstration. Watch for meningeal irritation, meningitis, subarachnoid hemorrhage. Video cues for Kernig and Brudzinski signs: the visible endpoint of Kernig and Brudzinski signs, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Kernig and Brudzinski signs starts with meningeal irritation and is expressed clinically through meningitis. Related exam anchors are subarachnoid hemorrhage.

Associated conditions

- meningeal irritation
- meningitis
- subarachnoid hemorrhage
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
hamstring tightness	Pain/tightness without meningeal irritation.	Straight-leg discomfort may be symmetric.
cervical stiffness	Neck movement limitation alone lacks Kernig/Brudzinski and infection/SAH context.	Use the meningeal irritation endpoint to decide whether cervical stiffness explains the clip.
raised ICP without meningitis	Papilledema, vomiting, focal deficit.	Meningeal signs may be absent.

Clinical examination points

- Kernig and Brudzinski signs: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Kernig and Brudzinski signs: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Kernig and Brudzinski signs is usually tested through meningeal irritation; meningitis; subarachnoid hemorrhage. Separate it from hamstring tightness, cervical stiffness, raised ICP without meningitis.

Management relevance

Use the demonstrated technique only for the correct indication; the management decision hinges on meningeal irritation, patient safety, and the endpoint shown in the video.

Common traps

- Do not confuse with hamstring tightness: Pain/tightness without meningeal irritation.
- Do not confuse with cervical stiffness: Neck movement limitation alone lacks Kernig/Brudzinski and infection/SAH context.
- Do not confuse with raised ICP without meningitis: Papilledema, vomiting, focal deficit.
- Kernig and Brudzinski signs needs both meningeal irritation and meningitis; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Kernig and Brudzinski signs.
- Subject: Medicine / Neurology.
- Key clue: meningeal irritation.
- Key clue: meningitis.
- Key clue: subarachnoid hemorrhage.
- Closest mimics: hamstring tightness, cervical stiffness, raised ICP without meningitis.
- Video segment: 00:00-00:20.
- One-line recall: Kernig and Brudzinski signs points to meningeal irritation.

Source credit: Kernig's sign in meningitis patient... - Learn MBBS.

Topic 58 - Hemifacial spasm / facial myokymia

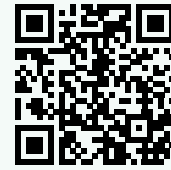
Subject: Medicine / Neurology

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:31

Source credit: Hemifacial Spasm Symptoms and Cure - Before and After Microvascular Decompression - MVD Surgery - TheRenderQ



What it is

Hemifacial spasm / facial myokymia: short clip of involuntary unilateral facial twitching. High-yield anchors: VII nerve irritation; vascular compression; cerebellopontine angle lesion.

How to identify it in the video

short clip of involuntary unilateral facial twitching. Watch for VII nerve irritation, vascular compression, cerebellopontine angle lesion. Video cues for Hemifacial spasm / facial myokymia: the visible endpoint of Hemifacial spasm / facial myokymia, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Hemifacial spasm / facial myokymia starts with VII nerve irritation and is expressed clinically through vascular compression. Related exam anchors are cerebellopontine angle lesion.

Associated conditions

- VII nerve irritation
- vascular compression
- cerebellopontine angle lesion
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
focal seizure	Stereotyped event, impaired awareness.	Postictal state, tongue bite, and EEG support seizure.
tics	Premonitory urge.	Partial suppressibility, and waxing-waning course suggest tic disorder.
Bell's palsy recovery synkinesis	LMN whole-face weakness without forehead sparing.	Exclude vesicles/stroke red flags.

Clinical examination points

- For Hemifacial spasm / facial myokymia, document the visible abnormality before naming the diagnosis.
- For Hemifacial spasm / facial myokymia, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Hemifacial spasm / facial myokymia is usually tested through VII nerve irritation; vascular compression; cerebellopontine angle lesion. Separate it from focal seizure, tics, Bell's palsy recovery synkinesis.

Management relevance

Clinical action after recognizing Hemifacial spasm / facial myokymia: confirm VII nerve irritation, look for complications, and choose treatment according to the underlying diagnosis.

Common traps

- Do not confuse with focal seizure: Stereotyped event, impaired awareness.
- Do not confuse with tics: Premonitory urge.
- Do not confuse with Bell's palsy recovery synkinesis: LMN whole-face weakness without forehead sparing.
- Hemifacial spasm / facial myokymia needs both VII nerve irritation and vascular compression; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Hemifacial spasm / facial myokymia.
- Subject: Medicine / Neurology.
- Key clue: VII nerve irritation.
- Key clue: vascular compression.
- Key clue: cerebellopontine angle lesion.
- Closest mimics: focal seizure, tics, Bell's palsy recovery synkinesis.
- Video segment: 00:00-00:31.
- One-line recall: Hemifacial spasm / facial myokymia points to VII nerve irritation.

Source credit: Hemifacial Spasm Symptoms and Cure - Before and After Microvascular Decompression - MVD Surgery - TheRenderQ.

Topic 59 - Fasciculations

Subject: Medicine / Neurology

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:20

Source credit: visible fasciculations in a patient of ALS #Neurology - Dr Muhammad Hashir
Neurologist & Stroke Physician



What it is

Fasciculations: visible muscle fasciculations. High-yield anchors: LMN sign; ALS; anterior horn cell disease; benign fasciculation syndrome.

How to identify it in the video

visible muscle fasciculations. Watch for LMN sign, ALS, anterior horn cell disease, benign fasciculation syndrome. Video cues for Fasciculations: the visible endpoint of Fasciculations, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Fasciculations starts with LMN sign and is expressed clinically through ALS. Related exam anchors are anterior horn cell disease, benign fasciculation syndrome.

Associated conditions

- LMN sign
- anterior horn cell disease
- benign fasciculation syndrome
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
tremor	Classify as rest, postural.	Action, or intention before naming the disorder.
myokymia	Rippling worm-like muscle activity.	EMG shows myokymic discharges.
fibrillation	Not visible clinically.	EMG finding, unlike fasciculations.
cramps	Painful sustained contraction relieved by stretch, not random fasciculation.	Use the LMN sign endpoint to decide whether cramps explains the clip.

Clinical examination points

- For Fasciculations, document the visible abnormality before naming the diagnosis.
- For Fasciculations, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Fasciculations is usually tested through LMN sign; ALS; anterior horn cell disease. Separate it from tremor, myokymia, fibrillation.

Management relevance

Clinical action after recognizing Fasciculations: confirm LMN sign, look for complications, and choose treatment according to the underlying diagnosis.

Common traps

- Do not confuse with tremor: Classify as rest, postural.
- Do not confuse with myokymia: Rippling worm-like muscle activity.
- Do not confuse with fibrillation: Not visible clinically.
- Fasciculations needs both LMN sign and ALS; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Fasciculations.
- Subject: Medicine / Neurology.
- Key clue: LMN sign.
- Key clue: ALS.
- Key clue: anterior horn cell disease.
- Closest mimics: tremor, myokymia, fibrillation, cramps.
- Video segment: 00:00-00:20.
- One-line recall: Fasciculations points to LMN sign.

Source credit: visible fasciculations in a patient of ALS #Neurology - Dr Muhammad Hashir Neurologist & Stroke Physician.

Topic 60 - Tardive dyskinesia

Subject: Psychiatry / Pharmacology / Neurology

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:27

Source credit: Patient With TD: Orofacial Movements - AUSTEDO XR educational channel



What it is

Tardive dyskinesia: orofacial dyskinesia or choreoathetoid movements. High-yield anchors: chronic antipsychotic use; dopamine receptor supersensitivity; VMAT2 inhibitors.

How to identify it in the video

orofacial dyskinesia or choreoathetoid movements. Watch for chronic antipsychotic use, dopamine receptor supersensitivity, VMAT2 inhibitors. Video cues for Tardive dyskinesia: the visible endpoint of Tardive dyskinesia, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Tardive dyskinesia starts with chronic antipsychotic use and is expressed clinically through dopamine receptor supersensitivity. Related exam anchors are VMAT2 inhibitors.

Associated conditions

- chronic antipsychotic use
- dopamine receptor supersensitivity
- VMAT2 inhibitors
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
acute dystonia	Sustained twisting posture suggests dystonia.	Check dopamine-blocker exposure and anticholinergic response.
akathisia	Subjective inner restlessness with pacing.	No choreiform orofacial movements.
chorea	Random flowing non-rhythmic movements.	Ask about Huntington disease, Sydenham chorea, pregnancy, drugs.
drug-induced parkinsonism	Symmetric parkinsonism after dopamine blockers.	Improve after stopping drug.

Clinical examination points

- For Tardive dyskinesia, document the visible abnormality before naming the diagnosis.
- For Tardive dyskinesia, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Tardive dyskinesia is usually tested through chronic antipsychotic use; dopamine receptor supersensitivity; VMAT2 inhibitors. Separate it from acute dystonia, akathisia, chorea.

Management relevance

Reduce/offending antipsychotic if feasible and consider VMAT2 inhibitors.

Common traps

- Do not confuse with acute dystonia: Sustained twisting posture suggests dystonia.
- Do not confuse with akathisia: Subjective inner restlessness with pacing.
- Do not confuse with chorea: Random flowing non-rhythmic movements.
- Tardive dyskinesia needs both chronic antipsychotic use and dopamine receptor supersensitivity; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Tardive dyskinesia.
- Subject: Psychiatry / Pharmacology / Neurology.
- Key clue: chronic antipsychotic use.
- Key clue: dopamine receptor supersensitivity.
- Key clue: VMAT2 inhibitors.
- Closest mimics: acute dystonia, akathisia, chorea, drug-induced parkinsonism.
- Video segment: 00:00-00:27.
- One-line recall: Tardive dyskinesia points to chronic antipsychotic use.

Source credit: Patient With TD: Orofacial Movements - AUSTEDO XR educational channel.

Topic 61 - Akathisia

Subject: Psychiatry / Pharmacology

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:17

Source credit: Akathisia - Aimee & Josh Jensen



What it is

Akathisia: restless pacing, inability to sit still. High-yield anchors: antipsychotic adverse effect; subjective inner restlessness; propranolol.

How to identify it in the video

restless pacing, inability to sit still. Watch for antipsychotic adverse effect, subjective inner restlessness, propranolol. Video cues for Akathisia: the visible endpoint of Akathisia, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Akathisia starts with antipsychotic adverse effect and is expressed clinically through subjective inner restlessness. Related exam anchors are propranolol.

Associated conditions

- antipsychotic adverse effect
- subjective inner restlessness
- propranolol

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
anxiety	Restlessness tied to worry/autonomic arousal.	No objective EPS pattern.
agitation	Goal-directed overactivity from delirium/mania/psychosis.	Not medication-induced inner restlessness alone.
mania	Elevated mood, decreased sleep.	Pressured speech, grandiosity.
restless leg syndrome	Urge to move legs, worse at rest and evening, relieved by movement, not antipsychotic-linked.	Use the antipsychotic adverse effect endpoint to decide whether restless leg syndrome explains the clip.

Clinical examination points

- For Akathisia, document the visible abnormality before naming the diagnosis.
- For Akathisia, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Akathisia is usually tested through antipsychotic adverse effect; subjective inner restlessness; propranolol. Separate it from anxiety, agitation, mania.

Management relevance

Dose reduction, switch antipsychotic, and propranolol are classic management steps.

Common traps

- Do not confuse with anxiety: Restlessness tied to worry/autonomic arousal.
- Do not confuse with agitation: Goal-directed overactivity from delirium/mania/psychosis.

- Do not confuse with mania: Elevated mood, decreased sleep.
- Akathisia needs both antipsychotic adverse effect and subjective inner restlessness; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Akathisia.
- Subject: Psychiatry / Pharmacology.
- Key clue: antipsychotic adverse effect.
- Key clue: subjective inner restlessness.
- Key clue: propranolol.
- Closest mimics: anxiety, agitation, mania, restless leg syndrome.
- Video segment: 00:00-00:17.
- One-line recall: Akathisia points to antipsychotic adverse effect.

Source credit: Akathisia - Aimee & Josh Jensen.

Topic 62 - Catatonia

Subject: Psychiatry

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Catatonia - negativism, waxy flexibility, catalepsy - Mental Health Treatment



What it is

Catatonia: waxy flexibility, posturing, negativism, mutism, or stupor. High-yield anchors: catatonia signs; lorazepam challenge; ECT.

How to identify it in the video

waxy flexibility, posturing, negativism, mutism, or stupor. Watch for catatonia signs, lorazepam challenge, ECT. Video cues for Catatonia: the visible endpoint of Catatonia, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Catatonia starts with catatonia signs and is expressed clinically through lorazepam challenge. Related exam anchors are ECT.

Associated conditions

- catatonia signs
- lorazepam challenge
- ECT

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
severe depression	Psychomotor retardation without waxy flexibility or posturing.	Use the catatonia signs endpoint to decide whether severe depression explains the clip.
psychosis	Delusions/hallucinations alone do not explain catalepsy.	Mutism, or negativism.
neuroleptic malignant syndrome	Fever, rigidity, autonomic instability, and raised CK after antipsychotic exposure.	Use the catatonia signs endpoint to decide whether neuroleptic malignant syndrome explains the clip.
delirium	Fluctuating attention and consciousness.	Search infection/metabolic/drug causes.

Clinical examination points

- For Catatonia, document the visible abnormality before naming the diagnosis.
- For Catatonia, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Catatonia is usually tested through catatonia signs; lorazepam challenge; ECT. Separate it from severe depression, psychosis, neuroleptic malignant syndrome.

Management relevance

Lorazepam challenge is diagnostic and therapeutic; ECT for malignant or resistant cases.

Common traps

- Do not confuse with severe depression: Psychomotor retardation without waxy flexibility or posturing.
- Do not confuse with psychosis: Delusions/hallucinations alone do not explain catalepsy.

- Do not confuse with neuroleptic malignant syndrome: Retracted or dull tympanic membrane with air-fluid level but no acute fever or bulging.
- Catatonia needs both catatonia signs and lorazepam challenge; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Catatonia.
- Subject: Psychiatry.
- Key clue: catatonia signs.
- Key clue: lorazepam challenge.
- Key clue: ECT.
- Closest mimics: severe depression, psychosis, neuroleptic malignant syndrome, delirium.
- Video segment: 00:00-01:00.
- One-line recall: Catatonia points to catatonia signs.

Source credit: Catatonia - negativism, waxy flexibility, catalepsy - Mental Health Treatment.

Topic 63 - Trousseau sign of hypocalcemia

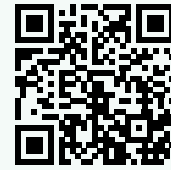
Subject: Medicine / Endocrinology

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Trousseau's Sign of Latent Tetany During Hypocalcemia: Nursing Clinical Skills - RegisteredNurseRN



What it is

Trousseau sign of hypocalcemia: carpopedal spasm after BP cuff inflation. High-yield anchors: hypocalcemia; latent tetany; post-thyroidectomy hypocalcemia.

How to identify it in the video

carpopedal spasm after BP cuff inflation. Watch for hypocalcemia, latent tetany, post-thyroidectomy hypocalcemia. Video cues for Trousseau sign of hypocalcemia: the visible endpoint of Trousseau sign of hypocalcemia, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Trousseau sign of hypocalcemia starts with hypocalcemia and is expressed clinically through latent tetany. Related exam anchors are post-thyroidectomy hypocalcemia.

Associated conditions

- hypocalcemia
- latent tetany
- post-thyroidectomy hypocalcemia
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
seizure	Stereotyped event, impaired awareness.	Postictal state, tongue bite, and EEG support seizure.
dystonia	Sustained twisting posture suggests dystonia.	Check dopamine-blocker exposure and anticholinergic response.
carpopedal spasm from alkalosis	Respiratory alkalosis lowers ionized calcium.	ABG and anxiety/hyperventilation clue.

Clinical examination points

- Trousseau sign of hypocalcemia: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Trousseau sign of hypocalcemia: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Trousseau sign of hypocalcemia is usually tested through hypocalcemia; latent tetany; post-thyroidectomy hypocalcemia. Separate it from seizure, dystonia, carpopedal spasm from alkalosis.

Management relevance

Confirm calcium/magnesium; symptomatic hypocalcemia needs IV calcium gluconate.

Common traps

- Do not confuse with seizure: Stereotyped event, impaired awareness.
- Do not confuse with dystonia: Sustained twisting posture suggests dystonia.
- Do not confuse with carpopedal spasm from alkalosis: Respiratory alkalosis lowers ionized calcium.
- Trousseau sign of hypocalcemia needs both hypocalcemia and latent tetany; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Trousseau sign of hypocalcemia.
- Subject: Medicine / Endocrinology.
- Key clue: hypocalcemia.
- Key clue: latent tetany.
- Key clue: post-thyroidectomy hypocalcemia.
- Closest mimics: seizure, dystonia, carpopedal spasm from alkalosis.
- Video segment: 00:00-01:00.
- One-line recall: Trousseau sign of hypocalcemia points to hypocalcemia.

Source credit: Trousseau's Sign of Latent Tetany During Hypocalcemia: Nursing Clinical Skills - RegisteredNurseRN.

Topic 64 - Chvostek sign

Subject: Medicine / Endocrinology

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Chvostek's Sign: Positive vs Negative Sign Assessment - Nursing Clinical Skills - RegisteredNurseRN



What it is

Chvostek sign: facial twitch after tapping facial nerve. High-yield anchors: hypocalcemia; neuromuscular irritability; tetany.

How to identify it in the video

facial twitch after tapping facial nerve. Watch for hypocalcemia, neuromuscular irritability, tetany. Video cues for Chvostek sign: the visible endpoint of Chvostek sign, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Chvostek sign starts with hypocalcemia and is expressed clinically through neuromuscular irritability. Related exam anchors are tetany.

Associated conditions

- hypocalcemia
- neuromuscular irritability
- tetany
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
nonspecific facial twitch	Brief twitch without hypocalcemia signs.	Check calcium only if context fits.
hemifacial spasm	Unilateral facial contractions from CN VII irritation.	Persists during sleep.

Clinical examination points

- Chvostek sign: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Chvostek sign: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Chvostek sign is usually tested through hypocalcemia; neuromuscular irritability; tetany. Separate it from nonspecific facial twitch, hemifacial spasm.

Management relevance

Confirm calcium/magnesium; symptomatic hypocalcemia needs IV calcium gluconate.

Common traps

- Do not confuse with nonspecific facial twitch: Brief twitch without hypocalcemia signs.
- Do not confuse with hemifacial spasm: Unilateral facial contractions from CN VII irritation.
- Chvostek sign needs both hypocalcemia and neuromuscular irritability; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Chvostek sign.
- Subject: Medicine / Endocrinology.
- Key clue: hypocalcemia.
- Key clue: neuromuscular irritability.
- Key clue: tetany.
- Closest mimics: nonspecific facial twitch, hemifacial spasm.
- Video segment: 00:00-01:00.
- One-line recall: Chvostek sign points to hypocalcemia.

Source credit: Chvostek's Sign: Positive vs Negative Sign Assessment - Nursing Clinical Skills - RegisteredNurseRN.

Topic 65 - Kussmaul breathing

Subject: Medicine / Emergency

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:27

Source credit: Kussmaul Breathing in Diabetic Ketoacidosis (DKA) - Jeff Suchard



What it is

Kussmaul breathing: deep labored breathing. High-yield anchors: DKA; metabolic acidosis; compensatory hyperventilation.

How to identify it in the video

deep labored breathing. Watch for DKA, metabolic acidosis, compensatory hyperventilation. Video cues for Kussmaul breathing: the visible endpoint of Kussmaul breathing, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Kussmaul breathing starts with DKA and is expressed clinically through metabolic acidosis. Related exam anchors are compensatory hyperventilation.

Associated conditions

- metabolic acidosis
- compensatory hyperventilation
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
Cheyne-Stokes breathing	Crescendo-decrescendo breathing with apneas.	Heart failure/stroke context.
panic hyperventilation	Fast shallow breathing with anxiety and normal metabolic workup.	Use the DKA endpoint to decide whether panic hyperventilation explains the clip.
respiratory distress	Accessory muscle use, hypoxia.	Retractions, and auscultation decide cause.

Clinical examination points

- Kussmaul breathing: Start with ABCs, oxygenation/ventilation, circulation, monitoring, and escalation triggers.

NEET PG exam angle

Kussmaul breathing is usually tested through DKA; metabolic acidosis; compensatory hyperventilation. Separate it from Cheyne-Stokes breathing, panic hyperventilation, respiratory distress.

Management relevance

Treat the metabolic acidosis, especially DKA with fluids, insulin, potassium, and monitoring.

Common traps

- Do not confuse with Cheyne-Stokes breathing: Crescendo-decrescendo breathing with apneas.
- Do not confuse with panic hyperventilation: Fast shallow breathing with anxiety and normal metabolic workup.
- Do not confuse with respiratory distress: Accessory muscle use, hypoxia.

- Kussmaul breathing needs both DKA and metabolic acidosis; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Kussmaul breathing.
- Subject: Medicine / Emergency.
- Key clue: DKA.
- Key clue: metabolic acidosis.
- Key clue: compensatory hyperventilation.
- Closest mimics: Cheyne-Stokes breathing, panic hyperventilation, respiratory distress.
- Video segment: 00:00-00:27.
- One-line recall: Kussmaul breathing points to DKA.

Source credit: Kussmaul Breathing in Diabetic Ketoacidosis (DKA) - Jeff Suchard.

Topic 66 - Cheyne-Stokes respiration

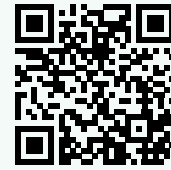
Subject: Medicine / Respiratory / Neurology

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Cheyne-Stokes Respirations: Real Clinical Examples - André Mansoor



What it is

Cheyne-Stokes respiration: crescendo-decrescendo periodic breathing. High-yield anchors: heart failure; stroke; raised ICP; periodic breathing.

How to identify it in the video

crescendo-decrescendo periodic breathing. Watch for heart failure, stroke, raised ICP, periodic breathing. Video cues for Cheyne-Stokes respiration: the visible endpoint of Cheyne-Stokes respiration, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Cheyne-Stokes respiration starts with heart failure and is expressed clinically through stroke. Related exam anchors are raised ICP, periodic breathing.

Associated conditions

- heart failure
- stroke
- raised ICP
- periodic breathing
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
Kussmaul breathing	Deep regular acidotic breathing.	Check glucose, ketones, ABG.
Biot breathing	Irregular clusters of breaths and apnea.	Medullary lesion/raised ICP context.
obstructive sleep apnea	Sleep-linked snoring/apneas.	Not continuous periodic breathing while awake.

Clinical examination points

- For Cheyne-Stokes respiration, document the visible abnormality before naming the diagnosis.
- For Cheyne-Stokes respiration, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Cheyne-Stokes respiration is usually tested through heart failure; stroke; raised ICP. Separate it from Kussmaul breathing, Biot breathing, obstructive sleep apnea.

Management relevance

Clinical action after recognizing Cheyne-Stokes respiration: confirm heart failure, look for complications, and choose treatment according to the underlying diagnosis.

Common traps

- Do not confuse with Kussmaul breathing: Deep regular acidotic breathing.
- Do not confuse with Biot breathing: Irregular clusters of breaths and apnea.
- Do not confuse with obstructive sleep apnea: Sleep-linked snoring/apneas.
- Cheyne-Stokes respiration needs both heart failure and stroke; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Cheyne-Stokes respiration.
- Subject: Medicine / Respiratory / Neurology.
- Key clue: heart failure.
- Key clue: stroke.
- Key clue: raised ICP.
- Closest mimics: Kussmaul breathing, Biot breathing, obstructive sleep apnea.
- Video segment: 00:00-01:00.
- One-line recall: Cheyne-Stokes respiration points to heart failure.

Source credit: Cheyne-Stokes Respirations: Real Clinical Examples - André Mansoor.

Topic 67 - Pitting edema demonstration

Subject: Medicine

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:20

Source credit: Pitting Pedal Edema #Edema #imwell - Dr Shashikant Mind Body Integrative Medicine



What it is

Pitting edema demonstration: direct pedal edema pitting demonstration. High-yield anchors: grading; heart failure; nephrotic syndrome; liver disease; venous insufficiency.

How to identify it in the video

direct pedal edema pitting demonstration. Watch for grading, heart failure, nephrotic syndrome, liver disease. Video cues for Pitting edema demonstration: the visible endpoint of Pitting edema demonstration, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Pitting edema demonstration starts with grading and is expressed clinically through heart failure. Related exam anchors are nephrotic syndrome, liver disease, venous insufficiency.

Associated conditions

- grading
- heart failure
- nephrotic syndrome
- liver disease
- venous insufficiency
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
lymphedema	Non-pitting later, thick skin.	Stemmer sign, and chronic lymphatic history.
myxedema	Non-pitting doughy edema with hypothyroid features and high TSH.	Use the grading endpoint to decide whether myxedema explains the clip.
cellulitis swelling	Warmth, erythema.	Tenderness, fever, and raised inflammatory markers.

Clinical examination points

- For Pitting edema demonstration, document the visible abnormality before naming the diagnosis.
- For Pitting edema demonstration, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Pitting edema demonstration is usually tested through grading; heart failure; nephrotic syndrome. Separate it from lymphedema, myxedema, cellulitis swelling.

Management relevance

Clinical action after recognizing Pitting edema demonstration: confirm grading, look for complications, and choose treatment according to the underlying diagnosis.

Common traps

- Do not confuse with lymphedema: Non-pitting later, thick skin.
- Do not confuse with myxedema: Non-pitting doughy edema with hypothyroid features and high TSH.
- Do not confuse with cellulitis swelling: Warmth, erythema.
- Pitting edema demonstration needs both grading and heart failure; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Pitting edema demonstration.
- Subject: Medicine.
- Key clue: grading.
- Key clue: heart failure.
- Key clue: nephrotic syndrome.
- Closest mimics: lymphedema, myxedema, cellulitis swelling.
- Video segment: 00:00-00:20.
- One-line recall: Pitting edema demonstration points to grading.

Source credit: Pitting Pedal Edema #Edema #imwell - Dr Shashikant Mind Body Integrative Medicine.

Topic 68 - Jugular venous pressure examination

Subject: Medicine / Cardiology

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:40

Source credit: Jugular Venous Distention - JVD | Dr. Rifai | The Virtual Nephrologist - The Virtual Nephrologist



What it is

Jugular venous pressure examination: visible JVP or JVP examination. High-yield anchors: right heart failure; tamponade; constrictive pericarditis; Kussmaul sign.

How to identify it in the video

visible JVP or JVP examination. Watch for right heart failure, tamponade, constrictive pericarditis, Kussmaul sign. Video cues for Jugular venous pressure examination: the visible endpoint of Jugular venous pressure examination, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Jugular venous pressure examination starts with right heart failure and is expressed clinically through tamponade. Related exam anchors are constrictive pericarditis, Kussmaul sign.

Associated conditions

- right heart failure
- tamponade
- constrictive pericarditis
- Kussmaul sign
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
carotid pulsation	Palpable, single brisk pulsation not obliterated by pressure.	No venous waveform.
external jugular vein	Superficial and variable.	Internal JVP is measured at right atrium reference.
prominent neck veins	Confirm venous waveform, height.	Hepatojugular reflux, and respiratory change.

Clinical examination points

- For Jugular venous pressure examination, document the visible abnormality before naming the diagnosis.
- For Jugular venous pressure examination, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Jugular venous pressure examination is usually tested through right heart failure; tamponade; constrictive pericarditis. Separate it from carotid pulsation, external jugular vein, prominent neck veins.

Management relevance

Treat the metabolic acidosis, especially DKA with fluids, insulin, potassium, and monitoring.

Common traps

- Do not confuse with carotid pulsation: Palpable, single brisk pulsation not obliterated by pressure.
- Do not confuse with external jugular vein: Superficial and variable.
- Do not confuse with prominent neck veins: Confirm venous waveform, height.
- Jugular venous pressure examination needs both right heart failure and tamponade; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Jugular venous pressure examination.
- Subject: Medicine / Cardiology.
- Key clue: right heart failure.
- Key clue: tamponade.
- Key clue: constrictive pericarditis.
- Closest mimics: carotid pulsation, external jugular vein, prominent neck veins.
- Video segment: 00:00-00:40.
- One-line recall: Jugular venous pressure examination points to right heart failure.

Source credit: Jugular Venous Distention - JVD | Dr. Rifai | The Virtual Nephrologist - The Virtual Nephrologist.

Topic 69 - Clubbing examination / Schamroth sign

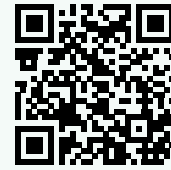
Subject: Medicine

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:58

Source credit: Schamroth Window Test for Diagnosis of Clubbing || Causes of Clubbing - Med-Clinical



What it is

Clubbing examination / Schamroth sign: direct finger clubbing or Schamroth window test. High-yield anchors: lung cancer; bronchiectasis; cyanotic heart disease; IBD.

How to identify it in the video

direct finger clubbing or Schamroth window test. Watch for lung cancer, bronchiectasis, cyanotic heart disease, IBD. Video cues for Clubbing examination / Schamroth sign: the visible endpoint of Clubbing examination / Schamroth sign, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Clubbing examination / Schamroth sign starts with lung cancer and is expressed clinically through bronchiectasis. Related exam anchors are cyanotic heart disease, IBD.

Associated conditions

- lung cancer
- bronchiectasis
- cyanotic heart disease
- IBD
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
pseudoclubbing	Asymmetric nail change often local trauma.	Schamroth window may be preserved.
paronychia	Tender inflamed nail fold rather than bulbous distal phalanx.	Use the lung cancer endpoint to decide whether paronychia explains the clip.
acromegaly	Enlarged hands/feet and coarse facial features from GH excess; the Schamroth window stays normal.	Use the lung cancer endpoint to decide whether acromegaly explains the clip.

Clinical examination points

- Clubbing examination / Schamroth sign: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Clubbing examination / Schamroth sign: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Clubbing examination / Schamroth sign is usually tested through lung cancer; bronchiectasis; cyanotic heart disease. Separate it from pseudoclubbing, paronychia, acromegaly.

Management relevance

Use the demonstrated technique only for the correct indication; the management decision hinges on lung cancer, patient safety, and the endpoint shown in the video.

Common traps

- Do not confuse with pseudoclubbing: Asymmetric nail change often local trauma.
- Do not confuse with paronychia: Tender inflamed nail fold rather than bulbous distal phalanx.
- Do not confuse with acromegaly: Retracted or dull tympanic membrane with air-fluid level but no acute fever or bulging.
- Clubbing examination / Schamroth sign needs both lung cancer and bronchiectasis; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Clubbing examination / Schamroth sign.
- Subject: Medicine.
- Key clue: lung cancer.
- Key clue: bronchiectasis.
- Key clue: cyanotic heart disease.
- Closest mimics: pseudoclubbing, paronychia, acromegaly.
- Video segment: 00:00-00:58.
- One-line recall: Clubbing examination / Schamroth sign points to lung cancer.

Source credit: Schamroth Window Test for Diagnosis of Clubbing || Causes of Clubbing - Med-Clinical.

Topic 70 - Applied asterix: hepatic encephalopathy / CO2 retention

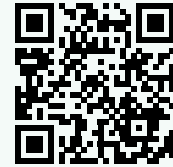
Subject: Medicine

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: My Patient is Confused! (Hepatic Encephalopathy) - Doc Schmidt



What it is

Applied asterix: hepatic encephalopathy / CO2 retention: flapping tremor in real clinical context. High-yield anchors: hepatic encephalopathy; uremia; CO2 retention; metabolic encephalopathy.

How to identify it in the video

flapping tremor in real clinical context. Watch for hepatic encephalopathy, uremia, CO2 retention, metabolic encephalopathy. Video cues for Applied asterix: hepatic encephalopathy / CO2 retention: the visible endpoint of Applied asterix: hepatic encephalopathy / CO2 retention, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Applied asterix: hepatic encephalopathy / CO2 retention starts with hepatic encephalopathy and is expressed clinically through uremia. Related exam anchors are CO2 retention, metabolic encephalopathy.

Associated conditions

- hepatic encephalopathy
- uremia
- CO2 retention
- metabolic encephalopathy
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
tremor	Classify as rest, postural.	Action, or intention before naming the disorder.
chorea	Random flowing non-rhythmic movements.	Ask about Huntington disease, Sydenham chorea, pregnancy, drugs.
myoclonus	Shock-like jerks suggest metabolic, epileptic, or post-hypoxic myoclonus.	EEG helps if epileptic.

Clinical examination points

- For Applied asterix: hepatic encephalopathy / CO2 retention, document the visible abnormality before naming the diagnosis.
- For Applied asterix: hepatic encephalopathy / CO2 retention, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Applied asterixis: hepatic encephalopathy / CO₂ retention is usually tested through hepatic encephalopathy; uremia; CO₂ retention. Separate it from tremor, chorea, myoclonus.

Management relevance

Treat the metabolic driver: lactulose/rifaximin for hepatic encephalopathy, dialysis for uremia, ventilatory support for CO₂ retention.

Common traps

- Do not confuse with tremor: Classify as rest, postural.
- Do not confuse with chorea: Random flowing non-rhythmic movements.
- Do not confuse with myoclonus: Shock-like jerks suggest metabolic, epileptic, or post-hypoxic myoclonus.
- Applied asterixis: hepatic encephalopathy / CO₂ retention needs both hepatic encephalopathy and uremia; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Applied asterixis: hepatic encephalopathy / CO₂ retention.
- Subject: Medicine.
- Key clue: hepatic encephalopathy.
- Key clue: uremia.
- Key clue: CO₂ retention.
- Closest mimics: tremor, chorea, myoclonus.
- Video segment: 00:00-01:00.
- One-line recall: Applied asterixis: hepatic encephalopathy / CO₂ retention points to hepatic encephalopathy.

Source credit: My Patient is Confused! (Hepatic Encephalopathy) - Doc Schmidt.

Topic 71 - Direct and consensual light reflex

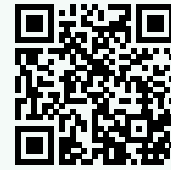
Subject: Ophthalmology / Neurology

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:24

Source credit: Direct and Consensual Light Reflex Test - Dr. Ray Pulmano DC



What it is

Direct and consensual light reflex: pupil light reflex demonstration. High-yield anchors: afferent optic nerve; efferent oculomotor nerve; direct vs consensual response.

How to identify it in the video

pupil light reflex demonstration. Watch for afferent optic nerve, efferent oculomotor nerve, direct vs consensual response. Video cues for Direct and consensual light reflex: the visible endpoint of Direct and consensual light reflex, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Direct and consensual light reflex starts with afferent optic nerve and is expressed clinically through efferent oculomotor nerve. Related exam anchors are direct vs consensual response.

Associated conditions

- afferent optic nerve
- efferent oculomotor nerve
- direct vs consensual response
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
RAPD	Swinging flashlight shows paradoxical dilation in affected eye.	Use the afferent optic nerve endpoint to decide whether RAPD explains the clip.
third nerve palsy	Ptosis with down-and-out eye; pupil involvement suggests compressive aneurysm until proven otherwise.	Down-and-out eye, and pupil involvement require urgent aneurysm imaging.
pharmacological pupil	Medication exposure.	No near response, and poor pilocarpine response can reveal drug effect.

Clinical examination points

- For Direct and consensual light reflex, document the visible abnormality before naming the diagnosis.
- For Direct and consensual light reflex, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Direct and consensual light reflex is usually tested through afferent optic nerve; efferent oculomotor nerve; direct vs consensual response. Separate it from RAPD, third nerve palsy, pharmacological pupil.

Management relevance

Clinical action after recognizing Direct and consensual light reflex: confirm afferent optic nerve, look for complications, and choose treatment according to the underlying diagnosis.

Common traps

- Do not confuse with RAPD: Swinging flashlight shows paradoxical dilation in affected eye.
- Do not confuse with third nerve palsy: Ptosis with down-and-out eye; pupil involvement suggests compressive aneurysm until proven otherwise.
- Do not confuse with pharmacological pupil: Medication exposure.
- Direct and consensual light reflex needs both afferent optic nerve and efferent oculomotor nerve; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Direct and consensual light reflex.
- Subject: Ophthalmology / Neurology.
- Key clue: afferent optic nerve.
- Key clue: efferent oculomotor nerve.
- Key clue: direct vs consensual response.
- Closest mimics: RAPD, third nerve palsy, pharmacological pupil.
- Video segment: 00:00-00:24.
- One-line recall: Direct and consensual light reflex points to afferent optic nerve.

Source credit: Direct and Consensual Light Reflex Test - Dr. Ray Pulmano DC.

Topic 72 - Argyll Robertson pupil

Subject: Ophthalmology / Medicine

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:59

Source credit: Argyll-Robertson Pupil - key points in 60 seconds - Dr. Aman Arora - Arora Medical Education



What it is

Argyll Robertson pupil: light-near dissociation if available. High-yield anchors: accommodates but does not react; neurosyphilis; diabetes as association.

How to identify it in the video

light-near dissociation if available. Watch for accommodates but does not react, neurosyphilis, diabetes as association. Video cues for Argyll Robertson pupil: pupil size and symmetry, direction of eye deviation or movement limitation, ptosis or lid asymmetry, nystagmus direction, light reflex or staining pattern.

Why it happens

The mechanism of Argyll Robertson pupil starts with accommodates but does not react and is expressed clinically through neurosyphilis. Related exam anchors are diabetes as association. In Argyll Robertson pupil, the bedside pattern localizes the relevant afferent, efferent, ocular motor, sympathetic, vestibular, or cerebellar pathway.

Associated conditions

- accommodates but does not react
- neurosyphilis
- diabetes as association
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
Adie tonic pupil	Large pupil with light-near dissociation and dilute pilocarpine supersensitivity.	Use the accommodates but does not react endpoint to decide whether Adie tonic pupil explains the clip.
RAPD	Swinging flashlight shows paradoxical dilation in affected eye.	Use the accommodates but does not react endpoint to decide whether RAPD explains the clip.
third nerve palsy	Ptosis with down-and-out eye; pupil involvement suggests compressive aneurysm until proven otherwise.	Down-and-out eye, and pupil involvement require urgent aneurysm imaging.

Clinical examination points

- Argyll Robertson pupil: Check pupils, ocular alignment, extraocular movements, ptosis, diplopia direction, and visual fields.
- Argyll Robertson pupil: Localize afferent and efferent pathways before naming the lesion.

NEET PG exam angle

Argyll Robertson pupil is usually tested through accommodates but does not react; neurosyphilis; diabetes as association. Separate it from Adie tonic pupil, RAPD, third nerve palsy.

Management relevance

Clinical action after recognizing Argyll Robertson pupil: confirm accommodates but does not react, look for complications, and choose treatment according to the underlying diagnosis.

Common traps

- Do not confuse with Adie tonic pupil: Large pupil with light-near dissociation and dilute pilocarpine supersensitivity.
- Do not confuse with RAPD: Swinging flashlight shows paradoxical dilation in affected eye.
- Do not confuse with third nerve palsy: Ptosis with down-and-out eye; pupil involvement suggests compressive aneurysm until proven otherwise.
- Argyll Robertson pupil needs both accommodates but does not react and neurosyphilis; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Argyll Robertson pupil.
- Subject: Ophthalmology / Medicine.
- Key clue: accommodates but does not react.
- Key clue: neurosyphilis.
- Key clue: diabetes as association.
- Closest mimics: Adie tonic pupil, RAPD, third nerve palsy.
- Video segment: 00:00-00:59.
- One-line recall: Argyll Robertson pupil points to accommodates but does not react.

Source credit: Argyll-Robertson Pupil - key points in 60 seconds - Dr. Aman Arora - Arora Medical Education.

Topic 73 - Horner syndrome

Subject: Ophthalmology / Neurology

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:40

Source credit: Horner's syndrome or oculosympathetic paresis - classic triad - Dr. Rakesh Ukey MD



What it is

Horner syndrome: ptosis and miosis demonstration. High-yield anchors: ptosis; miosis; anhidrosis; sympathetic pathway; Pancoast tumor; carotid dissection.

How to identify it in the video

ptosis and miosis demonstration. Watch for ptosis, miosis, anhidrosis, sympathetic pathway. Video cues for Horner syndrome: the visible endpoint of Horner syndrome, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Horner syndrome starts with ptosis and is expressed clinically through miosis. Related exam anchors are anhidrosis, sympathetic pathway, Pancoast tumor, carotid dissection.

Associated conditions

- ptosis
- miosis
- anhidrosis
- sympathetic pathway
- Pancoast tumor
- carotid dissection
- stroke or structural CNS lesion
- metabolic/toxic disturbance

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
third nerve palsy	Ptosis with down-and-out eye; pupil involvement suggests compressive aneurysm until proven otherwise.	Down-and-out eye, and pupil involvement require urgent aneurysm imaging.
simple anisocoria	Compare bright versus dim light.	Decide parasympathetic versus sympathetic lesion.
pharmacological miosis	Opioid/cholinergic exposure and bilateral small pupils without sympathetic pattern.	Use the ptosis endpoint to decide whether pharmacological miosis explains the clip.

Clinical examination points

- For Horner syndrome, document the visible abnormality before naming the diagnosis.
- For Horner syndrome, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Horner syndrome is usually tested through ptosis; miosis; anhidrosis. Separate it from third nerve palsy, simple anisocoria, pharmacological miosis.

Management relevance

Clinical action after recognizing Horner syndrome: confirm ptosis, look for complications, and choose treatment according to the underlying diagnosis.

Common traps

- Do not confuse with third nerve palsy: Ptosis with down-and-out eye; pupil involvement suggests compressive aneurysm until proven otherwise.
- Do not confuse with simple anisocoria: Compare bright versus dim light.
- Do not confuse with pharmacological miosis: Opioid/cholinergic exposure and bilateral small pupils without sympathetic pattern.
- Horner syndrome needs both ptosis and miosis; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Horner syndrome.
- Subject: Ophthalmology / Neurology.
- Key clue: ptosis.
- Key clue: miosis.
- Key clue: anhidrosis.
- Closest mimics: third nerve palsy, simple anisocoria, pharmacological miosis.
- Video segment: 00:00-00:40.
- One-line recall: Horner syndrome points to ptosis.

Source credit: Horner's syndrome or oculosympathetic paresis - classic triad - Dr. Rakesh Ukey MD.

Topic 74 - Fluorescein staining: corneal abrasion / ulcer

Subject: Ophthalmology

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:26

Source credit: Fluorescein staining of the cornea - Richard C. Allen MD PhD FACS



What it is

Fluorescein staining: corneal abrasion / ulcer: direct fluorescein staining video. High-yield anchors: epithelial defect; cobalt blue light; corneal abrasion; ulcer.

How to identify it in the video

direct fluorescein staining video. Watch for epithelial defect, cobalt blue light, corneal abrasion, ulcer. Video cues for Fluorescein staining: corneal abrasion / ulcer: pupil size and symmetry, direction of eye deviation or movement limitation, ptosis or lid asymmetry, nystagmus direction, light reflex or staining pattern.

Why it happens

The mechanism of Fluorescein staining: corneal abrasion / ulcer starts with epithelial defect and is expressed clinically through cobalt blue light. Related exam anchors are corneal abrasion, ulcer.

Associated conditions

- epithelial defect
- cobalt blue light
- corneal abrasion
- ulcer

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
dendritic ulcer	Branching fluorescein-staining lesion with terminal bulbs, typical of HSV keratitis.	Avoid topical steroids.
foreign body	Evert lid and inspect cornea/conjunctiva.	Remove visible object safely.
keratitis	Pain, photophobia, infiltrate, contact lens history.	Urgent slit-lamp review.

Clinical examination points

- For Fluorescein staining: corneal abrasion / ulcer, document the visible abnormality before naming the diagnosis.
- For Fluorescein staining: corneal abrasion / ulcer, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Fluorescein staining: corneal abrasion / ulcer is usually tested through epithelial defect; cobalt blue light; corneal abrasion. Separate it from dendritic ulcer, foreign body, keratitis.

Management relevance

Avoid steroids until infection is excluded; corneal ulcer or dendrite needs urgent ophthalmology care.

Common traps

- Do not confuse with dendritic ulcer: Branching fluorescein-staining lesion with terminal bulbs, typical of HSV keratitis.
- Do not confuse with foreign body: Evert lid and inspect cornea/conjunctiva.
- Do not confuse with keratitis: Pain, photophobia, infiltrate, contact lens history.

- Fluorescein staining: corneal abrasion / ulcer needs both epithelial defect and cobalt blue light; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Fluorescein staining: corneal abrasion / ulcer.
- Subject: Ophthalmology.
- Key clue: epithelial defect.
- Key clue: cobalt blue light.
- Key clue: corneal abrasion.
- Closest mimics: dendritic ulcer, foreign body, keratitis.
- Video segment: 00:00-00:26.
- One-line recall: Fluorescein staining: corneal abrasion / ulcer points to epithelial defect.

Source credit: Fluorescein staining of the cornea - Richard C. Allen MD PhD FACS.

Topic 75 - Dendritic ulcer staining

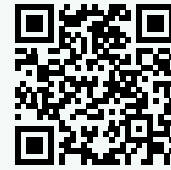
Subject: Ophthalmology

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:23

Source credit: Herpes Simplex Epithelial Keratitis - Dendritic Ulcer - Cornea Centre - Dr Mehul R Patel



What it is

Dendritic ulcer staining: fluorescein-stained dendritic ulcer. High-yield anchors: HSV keratitis; branching dendritic lesion; topical antiviral therapy.

How to identify it in the video

fluorescein-stained dendritic ulcer. Watch for HSV keratitis, branching dendritic lesion, topical antiviral therapy. Video cues for Dendritic ulcer staining: the visible endpoint of Dendritic ulcer staining, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Dendritic ulcer staining starts with HSV keratitis and is expressed clinically through branching dendritic lesion. Related exam anchors are topical antiviral therapy.

Associated conditions

- HSV keratitis
- branching dendritic lesion
- topical antiviral therapy

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
corneal abrasion	Linear or geographic epithelial defect staining with fluorescein.	No branching terminal bulbs.
fungal keratitis	Pain, photophobia, infiltrate, contact lens history.	Urgent slit-lamp review.
contact lens ulcer	Pseudomonas risk.	Stop lenses and urgent antibiotic treatment.

Clinical examination points

- For Dendritic ulcer staining, document the visible abnormality before naming the diagnosis.
- For Dendritic ulcer staining, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Dendritic ulcer staining is usually tested through HSV keratitis; branching dendritic lesion; topical antiviral therapy. Separate it from corneal abrasion, fungal keratitis, contact lens ulcer.

Management relevance

Clinical action after recognizing Dendritic ulcer staining: confirm HSV keratitis, look for complications, and choose treatment according to the underlying diagnosis.

Common traps

- Do not confuse with corneal abrasion: Linear or geographic epithelial defect staining with fluorescein.
- Do not confuse with fungal keratitis: Pain, photophobia, infiltrate, contact lens history.
- Do not confuse with contact lens ulcer: Pseudomonas risk.

- Dendritic ulcer staining needs both HSV keratitis and branching dendritic lesion; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Dendritic ulcer staining.
- Subject: Ophthalmology.
- Key clue: HSV keratitis.
- Key clue: branching dendritic lesion.
- Key clue: topical antiviral therapy.
- Closest mimics: corneal abrasion, fungal keratitis, contact lens ulcer.
- Video segment: 00:00-00:23.
- One-line recall: Dendritic ulcer staining points to HSV keratitis.

Source credit: Herpes Simplex Epithelial Keratitis - Dendritic Ulcer - Cornea Centre - Dr Mehul R Patel.

Topic 76 - Red reflex examination

Subject: Ophthalmology / Pediatrics

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:39

Source credit: Red reflex, neonatal examination before discharge, cataract, leukocoria, retinoblastoma - Dr. Tageldin Aly



What it is

Red reflex examination: red reflex demonstration. High-yield anchors: leukocoria; cataract; retinoblastoma screening.

How to identify it in the video

red reflex demonstration. Watch for leukocoria, cataract, retinoblastoma screening. Video cues for Red reflex examination: pupil size and symmetry, direction of eye deviation or movement limitation, ptosis or lid asymmetry, nystagmus direction, light reflex or staining pattern.

Why it happens

The mechanism of Red reflex examination starts with leukocoria and is expressed clinically through cataract. Related exam anchors are retinoblastoma screening.

Associated conditions

- leukocoria
- cataract
- retinoblastoma screening
- developmental age-related normal variant
- congenital or inherited disorder
- acute infection or emergency presentation

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
cataract	Dull or absent red reflex with lens opacity.	Slit-lamp confirms.
retinoblastoma	Leukocoria in a child; urgent referral is needed.	Urgent oncology/ophthalmology.
vitreous hemorrhage	Sudden floaters/vision loss with poor red reflex.	Fundus/ultrasound helps.

Clinical examination points

- For Red reflex examination, document the visible abnormality before naming the diagnosis.
- For Red reflex examination, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Red reflex examination is usually tested through leukocoria; cataract; retinoblastoma screening. Separate it from cataract, retinoblastoma, vitreous hemorrhage.

Management relevance

Clinical action after recognizing Red reflex examination: confirm leukocoria, look for complications, and choose treatment according to the underlying diagnosis.

Common traps

- Do not confuse with cataract: Dull or absent red reflex with lens opacity.
- Do not confuse with retinoblastoma: Leukocoria in a child; urgent referral is needed.

- Do not confuse with vitreous hemorrhage: Sudden floaters/vision loss with poor red reflex.
- Red reflex examination needs both leukocoria and cataract; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Red reflex examination.
- Subject: Ophthalmology / Pediatrics.
- Key clue: leukocoria.
- Key clue: cataract.
- Key clue: retinoblastoma screening.
- Closest mimics: cataract, retinoblastoma, vitreous hemorrhage.
- Video segment: 00:00-00:39.
- One-line recall: Red reflex examination points to leukocoria.

Source credit: Red reflex, neonatal examination before discharge, cataract, leukocoria, retinoblastoma - Dr. Tageldin Aly.

Topic 77 - Confrontation visual field testing

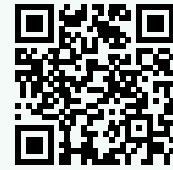
Subject: Ophthalmology / Neurology

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:27

Source credit: Ophthalmology basic exam: confrontation visual field testing - Richard C. Allen MD
PhD FACS



What it is

Confrontation visual field testing: bedside visual field demonstration. High-yield anchors: optic pathway localization; hemianopia; bitemporal hemianopia.

How to identify it in the video

bedside visual field demonstration. Watch for optic pathway localization, hemianopia, bitemporal hemianopia. Video cues for Confrontation visual field testing: pupil size and symmetry, direction of eye deviation or movement limitation, ptosis or lid asymmetry, nystagmus direction, light reflex or staining pattern.

Why it happens

The mechanism of Confrontation visual field testing starts with optic pathway localization and is expressed clinically through hemianopia. Related exam anchors are bitemporal hemianopia.

Associated conditions

- optic pathway localization
- hemianopia
- bitemporal hemianopia
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
monocular field loss	Pre-chiasmal optic nerve/retina lesion.	Test each eye separately.
homonymous hemianopia	Post-chiasmal lesion.	Same side of visual field lost in both eyes.
neglect	Patient fails to attend despite intact primary visual pathways.	Parietal lesion clue.

Clinical examination points

- Confrontation visual field testing: Check pupils, ocular alignment, extraocular movements, ptosis, diplopia direction, and visual fields.
- Confrontation visual field testing: Localize afferent and efferent pathways before naming the lesion.
- Confrontation visual field testing: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Confrontation visual field testing: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Confrontation visual field testing is usually tested through optic pathway localization; hemianopia; bitemporal hemianopia. Separate it from monocular field loss, homonymous hemianopia, neglect.

Management relevance

Use the demonstrated technique only for the correct indication; the management decision hinges on optic pathway localization, patient safety, and the endpoint shown in the video.

Common traps

- Do not confuse with monocular field loss: Pre-chiasmal optic nerve/retina lesion.
- Do not confuse with homonymous hemianopia: Post-chiasmal lesion.
- Do not confuse with neglect: Patient fails to attend despite intact primary visual pathways.
- Confrontation visual field testing needs both optic pathway localization and hemianopia; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Confrontation visual field testing.
- Subject: Ophthalmology / Neurology.
- Key clue: optic pathway localization.
- Key clue: hemianopia.
- Key clue: bitemporal hemianopia.
- Closest mimics: monocular field loss, homonymous hemianopia, neglect.
- Video segment: 00:00-00:27.
- One-line recall: Confrontation visual field testing points to optic pathway localization.

Source credit: Ophthalmology basic exam: confrontation visual field testing - Richard C. Allen MD PhD FACS.

Topic 78 - Nasal polyp on nasal endoscopy

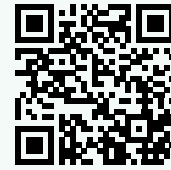
Subject: ENT

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:38

Source credit: Nasal Polyp Removal - High Impact



What it is

Nasal polyp on nasal endoscopy: short nasal endoscopy clip showing polyp. High-yield anchors: ethmoidal polyp; allergy; aspirin-exacerbated respiratory disease; cystic fibrosis.

How to identify it in the video

short nasal endoscopy clip showing polyp. Watch for ethmoidal polyp, allergy, aspirin-exacerbated respiratory disease, cystic fibrosis. Video cues for Nasal polyp on nasal endoscopy: visible mucosa or membrane, swelling, mass, perforation, or discharge, movement during phonation, swallowing, or breathing, side and symmetry.

Why it happens

The mechanism of Nasal polyp on nasal endoscopy starts with ethmoidal polyp and is expressed clinically through allergy. Related exam anchors are aspirin-exacerbated respiratory disease, cystic fibrosis.

Associated conditions

- ethmoidal polyp
- allergy
- aspirin-exacerbated respiratory disease
- cystic fibrosis

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
antrochoanal polyp	Unilateral, arises from the maxillary sinus and extends through the choana, unlike ethmoidal polyps.	Use the ethmoidal polyp endpoint to decide whether antrochoanal polyp explains the clip.
turbinate hypertrophy	Bilateral mucosal swelling that shrinks with decongestant.	Use the ethmoidal polyp endpoint to decide whether turbinate hypertrophy explains the clip.
nasal mass	Unilateral bleeding/foul discharge or firm lesion needs imaging/biopsy.	Use the ethmoidal polyp endpoint to decide whether nasal mass explains the clip.

Clinical examination points

- For Nasal polyp on nasal endoscopy, document the visible abnormality before naming the diagnosis.
- For Nasal polyp on nasal endoscopy, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Nasal polyp on nasal endoscopy is usually tested through ethmoidal polyp; allergy; aspirin-exacerbated respiratory disease. Separate it from antrochoanal polyp, turbinate hypertrophy, nasal mass.

Management relevance

Clinical action after recognizing Nasal polyp on nasal endoscopy: confirm ethmoidal polyp, look for complications, and choose treatment according to the underlying diagnosis.

Common traps

- Do not confuse with antrochoanal polyp: Usually unilateral pedunculated lesion with voice abuse or acute phonotrauma history.
- Do not confuse with turbinate hypertrophy: Bilateral mucosal swelling that shrinks with decongestant.
- Do not confuse with nasal mass: Unilateral bleeding/foul discharge or firm lesion needs imaging/biopsy.
- Nasal polyp on nasal endoscopy needs both ethmoidal polyp and allergy; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Nasal polyp on nasal endoscopy.
- Subject: ENT.
- Key clue: ethmoidal polyp.
- Key clue: allergy.
- Key clue: aspirin-exacerbated respiratory disease.
- Closest mimics: antrochoanal polyp, turbinate hypertrophy, nasal mass.
- Video segment: 00:00-00:38.
- One-line recall: Nasal polyp on nasal endoscopy points to ethmoidal polyp.

Source credit: Nasal Polyp Removal - High Impact.

Topic 79 - Deviated nasal septum examination / endoscopy

Subject: ENT

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Deviated Septum Surgery (Septoplasty) - Fauquier ENT



What it is

Deviated nasal septum examination / endoscopy: direct DNS examination or endoscopic finding. High-yield anchors: nasal obstruction; septal deviation; septoplasty.

How to identify it in the video

direct DNS examination or endoscopic finding. Watch for nasal obstruction, septal deviation, septoplasty. Video cues for Deviated nasal septum examination / endoscopy: visible mucosa or membrane, swelling, mass, perforation, or discharge, movement during phonation, swallowing, or breathing, side and symmetry.

Why it happens

The mechanism of Deviated nasal septum examination / endoscopy starts with nasal obstruction and is expressed clinically through septal deviation. Related exam anchors are septoplasty.

Associated conditions

- nasal obstruction
- septal deviation
- septoplasty

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
turbinate hypertrophy	Bilateral mucosal swelling that shrinks with decongestant.	Use the nasal obstruction endpoint to decide whether turbinate hypertrophy explains the clip.
nasal polyp	Pale, boggy, mobile grape-like masses, usually bilateral, arising from the middle meatus.	Use the nasal obstruction endpoint to decide whether nasal polyp explains the clip.
allergic rhinitis	Itching, sneezing.	Watery rhinorrhea, pale boggy turbinates.

Clinical examination points

- For Deviated nasal septum examination / endoscopy, document the visible abnormality before naming the diagnosis.
- For Deviated nasal septum examination / endoscopy, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Deviated nasal septum examination / endoscopy is usually tested through nasal obstruction; septal deviation; septoplasty. Separate it from turbinate hypertrophy, nasal polyp, allergic rhinitis.

Management relevance

Clinical action after recognizing Deviated nasal septum examination / endoscopy: confirm nasal obstruction, look for complications, and choose treatment according to the underlying diagnosis.

Common traps

- Do not confuse with turbinate hypertrophy: Bilateral mucosal swelling that shrinks with decongestant.

- Do not confuse with nasal polyp: Usually unilateral pedunculated lesion with voice abuse or acute phonotrauma history.
- Do not confuse with allergic rhinitis: Itching, sneezing.
- Deviated nasal septum examination / endoscopy needs both nasal obstruction and septal deviation; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Deviated nasal septum examination / endoscopy.
- Subject: ENT.
- Key clue: nasal obstruction.
- Key clue: septal deviation.
- Key clue: septoplasty.
- Closest mimics: turbinate hypertrophy, nasal polyp, allergic rhinitis.
- Video segment: 00:00-01:00.
- One-line recall: Deviated nasal septum examination / endoscopy points to nasal obstruction.

Source credit: Deviated Septum Surgery (Septoplasty) - Fauquier ENT.

Topic 80 - Anterior nasal packing for epistaxis

Subject: ENT / Emergency

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Anterior Nasal Packing - theprocedureclinic



What it is

Anterior nasal packing for epistaxis: short anterior nasal packing demonstration. High-yield anchors: Little's area; first aid; anterior vs posterior epistaxis.

How to identify it in the video

short anterior nasal packing demonstration. Watch for Little's area, first aid, anterior vs posterior epistaxis. Video cues for Anterior nasal packing for epistaxis: visible mucosa or membrane, swelling, mass, perforation, or discharge, movement during phonation, swallowing, or breathing, side and symmetry.

Why it happens

The mechanism of Anterior nasal packing for epistaxis starts with Little's area and is expressed clinically through first aid. Related exam anchors are anterior vs posterior epistaxis. In Anterior nasal packing for epistaxis, the outcome depends on sequence, hand position, endpoint recognition, and avoiding the unsafe step.

Associated conditions

- Little's area
- first aid
- anterior vs posterior epistaxis

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
cautery	Used for visible anterior bleeding point after vasoconstrictor/anesthetic.	Use the Little's area endpoint to decide whether cautery explains the clip.
posterior packing	Needed for posterior bleed.	Higher admission/airway monitoring need.
balloon tamponade	Pericardial effusion plus chamber collapse and obstructive-shock physiology.	Watch pressure injury and posterior bleed signs.

Clinical examination points

- Anterior nasal packing for epistaxis: Start with ABCs, oxygenation/ventilation, circulation, monitoring, and escalation triggers.

NEET PG exam angle

Anterior nasal packing for epistaxis is usually tested through Little's area; first aid; anterior vs posterior epistaxis. Separate it from cautery, posterior packing, balloon tamponade.

Management relevance

Apply first aid compression, topical vasoconstrictor, identify anterior bleed, and avoid posterior-miss delays.

Common traps

- Do not confuse with cautery: Used for visible anterior bleeding point after vasoconstrictor/anesthetic.
- Do not confuse with posterior packing: Needed for posterior bleed.

- Do not confuse with balloon tamponade: Pericardial effusion plus chamber collapse and obstructive-shock physiology.
- Safety trap in Anterior nasal packing for epistaxis: sequence, hand position, and endpoint matter as much as naming the step.

Quick revision box

- Topic: Anterior nasal packing for epistaxis.
- Subject: ENT / Emergency.
- Key clue: Little's area.
- Key clue: first aid.
- Key clue: anterior vs posterior epistaxis.
- Closest mimics: cautery, posterior packing, balloon tamponade.
- Video segment: 00:00-01:00.
- One-line recall: Anterior nasal packing for epistaxis points to Little's area.

Source credit: Anterior Nasal Packing - theprocedureclinic.

Topic 81 - Tonsillar hypertrophy / quinsy

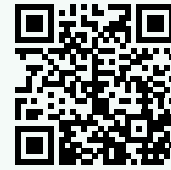
Subject: ENT

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:06

Source credit: Peritonsillar Abscess, US. JETem 2017 - JETem



What it is

Tonsillar hypertrophy / quinsy: peritonsillar abscess or tonsillar swelling video. High-yield anchors: uvula deviation; muffled voice; trismus; drainage.

How to identify it in the video

peritonsillar abscess or tonsillar swelling video. Watch for uvula deviation, muffled voice, trismus, drainage. Video cues for Tonsillar hypertrophy / quinsy: visible mucosa or membrane, swelling, mass, perforation, or discharge, movement during phonation, swallowing, or breathing, side and symmetry.

Why it happens

The mechanism of Tonsillar hypertrophy / quinsy starts with uvula deviation and is expressed clinically through muffled voice. Related exam anchors are trismus, drainage.

Associated conditions

- uvula deviation
- muffled voice
- trismus
- drainage

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
tonsillitis	Bilateral tonsillar inflammation without uvular deviation or trismus.	Use the uvula deviation endpoint to decide whether tonsillitis explains the clip.
diphtheria	Gray adherent membrane and toxic child.	Antitoxin and isolation are urgent.
retropharyngeal abscess	Neck stiffness, drooling.	Widened prevertebral space, airway risk.

Clinical examination points

- For Tonsillar hypertrophy / quinsy, document the visible abnormality before naming the diagnosis.
- For Tonsillar hypertrophy / quinsy, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Tonsillar hypertrophy / quinsy is usually tested through uvula deviation; muffled voice; trismus. Separate it from tonsillitis, diphtheria, retropharyngeal abscess.

Management relevance

Clinical action after recognizing Tonsillar hypertrophy / quinsy: confirm uvula deviation, look for complications, and choose treatment according to the underlying diagnosis.

Common traps

- Do not confuse with tonsillitis: Bilateral tonsillar inflammation without uvular deviation or trismus.
- Do not confuse with diphtheria: Gray adherent membrane and toxic child.

- Do not confuse with retropharyngeal abscess: Neck stiffness, drooling.
- Tonsillar hypertrophy / quinsy needs both uvula deviation and muffled voice; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Tonsillar hypertrophy / quinsy.
- Subject: ENT.
- Key clue: uvula deviation.
- Key clue: muffled voice.
- Key clue: trismus.
- Closest mimics: tonsillitis, diphtheria, retropharyngeal abscess.
- Video segment: 00:00-00:06.
- One-line recall: Tonsillar hypertrophy / quinsy points to uvula deviation.

Source credit: Peritonsillar Abscess, US. JETem 2017 - JETem.

Topic 82 - Ear or nasal foreign body removal

Subject: ENT / Emergency

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:34

Source credit: How to remove an ear foreign body using suction - Don't Forget The Bubbles



What it is

Ear or nasal foreign body removal: safe demonstration, preferably simulation. High-yield anchors: removal methods; button battery emergency; avoid irrigation for organic foreign body.

How to identify it in the video

safe demonstration, preferably simulation. Watch for removal methods, button battery emergency, avoid irrigation for organic foreign body. Video cues for Ear or nasal foreign body removal: visible mucosa or membrane, swelling, mass, perforation, or discharge, movement during phonation, swallowing, or breathing, side and symmetry.

Why it happens

The mechanism of Ear or nasal foreign body removal starts with removal methods and is expressed clinically through button battery emergency. Related exam anchors are avoid irrigation for organic foreign body.

Associated conditions

- removal methods
- button battery emergency
- avoid irrigation for organic foreign body

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
wax	Cerumen blocks canal view.	Remove wax before diagnosing middle-ear disease.
otitis externa	Canal tenderness and edema; tragal/pinna movement is painful.	Use the removal methods endpoint to decide whether otitis externa explains the clip.
live insect	Immobilize with oil/lidocaine before removal.	Avoid pushing deeper.

Clinical examination points

- Ear or nasal foreign body removal: Inspect pinna/canal, test tragal tenderness, visualize tympanic membrane landmarks, mobility, perforation, and discharge.
- Ear or nasal foreign body removal: Start with ABCs, oxygenation/ventilation, circulation, monitoring, and escalation triggers.

NEET PG exam angle

Ear or nasal foreign body removal is usually tested through removal methods; button battery emergency; avoid irrigation for organic foreign body. Separate it from wax, otitis externa, live insect.

Management relevance

For Ear or nasal foreign body removal, stabilize airway, breathing, circulation, and hemodynamics before definitive treatment for removal methods.

Common traps

- Do not confuse with wax: Cerumen blocks canal view.

- Do not confuse with otitis externa: Canal tenderness and edema; tragal/pinna movement is painful.
- Do not confuse with live insect: Immobilize with oil/lidocaine before removal.
- Ear or nasal foreign body removal needs both removal methods and button battery emergency; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Ear or nasal foreign body removal.
- Subject: ENT / Emergency.
- Key clue: removal methods.
- Key clue: button battery emergency.
- Key clue: avoid irrigation for organic foreign body.
- Closest mimics: wax, otitis externa, live insect.
- Video segment: 00:00-00:34.
- One-line recall: Ear or nasal foreign body removal points to removal methods.

Source credit: How to remove an ear foreign body using suction - Don't Forget The Bubbles.

Topic 83 - Straight leg raise test

Subject: Orthopedics / Neurology

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:20

Source credit: Straight Leg Raise Neural Tension Test for the Sciatic Nerve - www.sportsinjuryclinic.net



What it is

Straight leg raise test: SLR test demonstration. High-yield anchors: sciatica; L4-L5/L5-S1 disc prolapse; nerve root irritation.

How to identify it in the video

SLR test demonstration. Watch for sciatica, L4-L5/L5-S1 disc prolapse, nerve root irritation. Video cues for Straight leg raise test: the visible endpoint of Straight leg raise test, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Straight leg raise test starts with sciatica and is expressed clinically through L4-L5/L5-S1 disc prolapse. Related exam anchors are nerve root irritation.

Associated conditions

- sciatica
- L4-L5/L5-S1 disc prolapse
- nerve root irritation
- stroke or structural CNS lesion
- metabolic/toxic disturbance
- drug-induced syndrome when relevant
- traumatic injury
- sports injury

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
hamstring tightness	Pain/tightness without meningeal irritation.	Straight-leg discomfort may be symmetric.
hip pathology	Groin pain and painful hip range of motion separate from radicular SLR.	Use the sciatica endpoint to decide whether hip pathology explains the clip.
femoral stretch test	Anterior thigh pain from L2-L4 root irritation.	Use the sciatica endpoint to decide whether femoral stretch test explains the clip.

Clinical examination points

- Straight leg raise test: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Straight leg raise test: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Straight leg raise test is usually tested through sciatica; L4-L5/L5-S1 disc prolapse; nerve root irritation. Separate it from hamstring tightness, hip pathology, femoral stretch test.

Management relevance

Use the demonstrated technique only for the correct indication; the management decision hinges on sciatica, patient safety, and the endpoint shown in the video.

Common traps

- Do not confuse with hamstring tightness: Pain/tightness without meningeal irritation.
- Do not confuse with hip pathology: Groin pain and painful hip range of motion separate from radicular SLR.
- Do not confuse with femoral stretch test: Anterior thigh pain from L2-L4 root irritation.
- Straight leg raise test needs both sciatica and L4-L5/L5-S1 disc prolapse; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Straight leg raise test.
- Subject: Orthopedics / Neurology.
- Key clue: sciatica.
- Key clue: L4-L5/L5-S1 disc prolapse.
- Key clue: nerve root irritation.
- Closest mimics: hamstring tightness, hip pathology, femoral stretch test.
- Video segment: 00:00-00:20.
- One-line recall: Straight leg raise test points to sciatica.

Source credit: Straight Leg Raise Neural Tension Test for the Sciatic Nerve - www.sportsinjuryclinic.net.

Topic 84 - FABER / Patrick test

Subject: Orthopedics

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Rheumatology BACK exam: FABER test (Patrick's Test) - AlbertaRheumatology



What it is

FABER / Patrick test: FABER test demonstration. High-yield anchors: hip pathology; sacroiliac disease; ankylosing spondylitis.

How to identify it in the video

FABER test demonstration. Watch for hip pathology, sacroiliac disease, ankylosing spondylitis. Video cues for FABER / Patrick test: the visible endpoint of FABER / Patrick test, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of FABER / Patrick test starts with hip pathology and is expressed clinically through sacroiliac disease. Related exam anchors are ankylosing spondylitis.

Associated conditions

- hip pathology
- sacroiliac disease
- ankylosing spondylitis
- traumatic injury
- sports injury
- chronic degenerative disease

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
FADIR test	Flexion-adduction-internal rotation reproduces impingement/labral pain.	Use the hip pathology endpoint to decide whether FADIR test explains the clip.
straight leg raise	Reproduces radicular pain below knee at 30-70 degrees.	Use the hip pathology endpoint to decide whether straight leg raise explains the clip.
Thomas test	Opposite thigh lifts when hip flexion contracture is present.	Use the hip pathology endpoint to decide whether Thomas test explains the clip.

Clinical examination points

- FABER / Patrick test: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- FABER / Patrick test: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

FABER / Patrick test is usually tested through hip pathology; sacroiliac disease; ankylosing spondylitis. Separate it from FADIR test, straight leg raise, Thomas test.

Management relevance

Use the demonstrated technique only for the correct indication; the management decision hinges on hip pathology, patient safety, and the endpoint shown in the video.

Common traps

- Do not confuse with FADIR test: Flexion-adduction-internal rotation reproduces impingement/labral pain.
- Do not confuse with straight leg raise: Reproduces radicular pain below knee at 30-70 degrees.
- Do not confuse with Thomas test: Opposite thigh lifts when hip flexion contracture is present.
- FABER / Patrick test needs both hip pathology and sacroiliac disease; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: FABER / Patrick test.
- Subject: Orthopedics.
- Key clue: hip pathology.
- Key clue: sacroiliac disease.
- Key clue: ankylosing spondylitis.
- Closest mimics: FADIR test, straight leg raise, Thomas test.
- Video segment: 00:00-01:00.
- One-line recall: FABER / Patrick test points to hip pathology.

Source credit: Rheumatology BACK exam: FABER test (Patrick's Test) - AlbertaRheumatology.

Topic 85 - Schober test

Subject: Orthopedics / Rheumatology

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Modified Schober's Test - MSK Medicine



What it is

Schober test: Schober or modified Schober test demonstration. High-yield anchors: ankylosing spondylitis; reduced lumbar flexion.

How to identify it in the video

Schober or modified Schober test demonstration. Watch for ankylosing spondylitis, reduced lumbar flexion. Video cues for Schober test: the visible endpoint of Schober test, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Schober test starts with ankylosing spondylitis and is expressed clinically through reduced lumbar flexion.

Associated conditions

- ankylosing spondylitis
- reduced lumbar flexion
- traumatic injury
- sports injury
- chronic degenerative disease

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
mechanical back pain	No inflammatory stiffness.	Improves with rest and lacks sacroiliitis signs.
lumbar disc prolapse	Radicular pain, neurologic deficit.	Positive SLR, MRI if severe/persistent.

Clinical examination points

- Schober test: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Schober test: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Schober test is usually tested through ankylosing spondylitis; reduced lumbar flexion. Separate it from mechanical back pain, lumbar disc prolapse.

Management relevance

Use the demonstrated technique only for the correct indication; the management decision hinges on ankylosing spondylitis, patient safety, and the endpoint shown in the video.

Common traps

- Do not confuse with mechanical back pain: No inflammatory stiffness.
- Do not confuse with lumbar disc prolapse: Radicular pain, neurologic deficit.

- Schober test needs both ankylosing spondylitis and reduced lumbar flexion; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Schober test.
- Subject: Orthopedics / Rheumatology.
- Key clue: ankylosing spondylitis.
- Key clue: reduced lumbar flexion.
- Closest mimics: mechanical back pain, lumbar disc prolapse.
- Video segment: 00:00-01:00.
- One-line recall: Schober test points to ankylosing spondylitis.

Source credit: Modified Schober's Test - MSK Medicine.

Topic 86 - Ankle anterior drawer test / ATFL injury

Subject: Orthopedics

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:31

Source credit: Anterior Drawer - Ankle (ATFL) - MSK Medicine



What it is

Ankle anterior drawer test / ATFL injury: ankle drawer test demonstration. High-yield anchors: lateral ankle sprain; anterior talofibular ligament.

How to identify it in the video

ankle drawer test demonstration. Watch for lateral ankle sprain, anterior talofibular ligament. Video cues for Ankle anterior drawer test / ATFL injury: the visible endpoint of Ankle anterior drawer test / ATFL injury, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Ankle anterior drawer test / ATFL injury starts with lateral ankle sprain and is expressed clinically through anterior talofibular ligament.

Associated conditions

- lateral ankle sprain
- anterior talofibular ligament
- traumatic injury
- sports injury
- chronic degenerative disease

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
syndesmotic injury	High ankle pain.	Squeeze/external rotation test, widened mortise.
deltoid ligament injury	Stress the specific ligament and assess endpoint.	Swelling, and instability direction.
Achilles rupture	Palpable gap and absent plantarflexion on Thompson test.	Use the lateral ankle sprain endpoint to decide whether Achilles rupture explains the clip.

Clinical examination points

- Ankle anterior drawer test / ATFL injury: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Ankle anterior drawer test / ATFL injury: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Ankle anterior drawer test / ATFL injury is usually tested through lateral ankle sprain; anterior talofibular ligament. Separate it from syndesmotic injury, deltoid ligament injury, Achilles rupture.

Management relevance

Use the demonstrated technique only for the correct indication; the management decision hinges on lateral ankle sprain, patient safety, and the endpoint shown in the video.

Common traps

- Do not confuse with syndesmotic injury: High ankle pain.
- Do not confuse with deltoid ligament injury: Stress the specific ligament and assess endpoint.
- Do not confuse with Achilles rupture: Palpable gap and absent plantarflexion on Thompson test.
- Ankle anterior drawer test / ATFL injury needs both lateral ankle sprain and anterior talofibular ligament; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Ankle anterior drawer test / ATFL injury.
- Subject: Orthopedics.
- Key clue: lateral ankle sprain.
- Key clue: anterior talofibular ligament.
- Closest mimics: syndesmotic injury, deltoid ligament injury, Achilles rupture.
- Video segment: 00:00-00:31.
- One-line recall: Ankle anterior drawer test / ATFL injury points to lateral ankle sprain.

Source credit: Anterior Drawer - Ankle (ATFL) - MSK Medicine.

Topic 87 - Thompson test for Achilles tendon rupture

Subject: Orthopedics

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:21

Source credit: Thompson Test | Achilles Tendon Rupture - TheraXPro



What it is

Thompson test for Achilles tendon rupture: calf squeeze test demonstration. High-yield anchors: Achilles rupture; absent plantar flexion.

How to identify it in the video

calf squeeze test demonstration. Watch for Achilles rupture, absent plantar flexion. Video cues for Thompson test for Achilles tendon rupture: the visible endpoint of Thompson test for Achilles tendon rupture, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Thompson test for Achilles tendon rupture starts with Achilles rupture and is expressed clinically through absent plantar flexion.

Associated conditions

- Achilles rupture
- absent plantar flexion
- traumatic injury
- sports injury
- chronic degenerative disease

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
gastrocnemius tear	Calf pain with preserved Thompson plantarflexion.	Use the Achilles rupture endpoint to decide whether gastrocnemius tear explains the clip.
ankle sprain	Localized ligament tenderness.	Ottawa rules guide X-ray.
peripheral nerve palsy	Map motor/sensory deficit to named nerve and confirm with NCS/EMG.	Use the Achilles rupture endpoint to decide whether peripheral nerve palsy explains the clip.

Clinical examination points

- Thompson test for Achilles tendon rupture: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Thompson test for Achilles tendon rupture: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Thompson test for Achilles tendon rupture is usually tested through Achilles rupture; absent plantar flexion. Separate it from gastrocnemius tear, ankle sprain, peripheral nerve palsy.

Management relevance

Use the demonstrated technique only for the correct indication; the management decision hinges on Achilles rupture, patient safety, and the endpoint shown in the video.

Common traps

- Do not confuse with gastrocnemius tear: Calf pain with preserved Thompson plantarflexion.
- Do not confuse with ankle sprain: Localized ligament tenderness.
- Do not confuse with peripheral nerve palsy: Map motor/sensory deficit to named nerve and confirm with NCS/EMG.
- Thompson test for Achilles tendon rupture needs both Achilles rupture and absent plantar flexion; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Thompson test for Achilles tendon rupture.
- Subject: Orthopedics.
- Key clue: Achilles rupture.
- Key clue: absent plantar flexion.
- Closest mimics: gastrocnemius tear, ankle sprain, peripheral nerve palsy.
- Video segment: 00:00-00:21.
- One-line recall: Thompson test for Achilles tendon rupture points to Achilles rupture.

Source credit: Thompson Test | Achilles Tendon Rupture - TheraXPro.

Topic 88 - Thyroid swelling moving with deglutition

Subject: Surgery

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:13

Source credit: Thyroid Swelling - movement with deglutition demonstration - THE WHITE ARMY



What it is

Thyroid swelling moving with deglutition: neck swelling moving with swallowing. High-yield anchors: thyroid swelling; pretracheal fascia; deglutition.

How to identify it in the video

neck swelling moving with swallowing. Watch for thyroid swelling, pretracheal fascia, deglutition. Video cues for Thyroid swelling moving with deglutition: the visible endpoint of Thyroid swelling moving with deglutition, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Thyroid swelling moving with deglutition starts with thyroid swelling and is expressed clinically through pretracheal fascia. Related exam anchors are deglutition.

Associated conditions

- thyroid swelling
- pretracheal fascia
- deglutition

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
lymph node	Separate from midline tract/thyroid.	Assess mobility, tenderness, and drainage area.
thyroglossal cyst	Moves with tongue protrusion.	Ultrasound then Sistrunk operation.
dermoid cyst	Midline subcutaneous lesion that usually does not move with tongue protrusion.	Use the thyroid swelling endpoint to decide whether dermoid cyst explains the clip.

Clinical examination points

- For Thyroid swelling moving with deglutition, document the visible abnormality before naming the diagnosis.
- For Thyroid swelling moving with deglutition, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Thyroid swelling moving with deglutition is usually tested through thyroid swelling; pretracheal fascia; deglutition. Separate it from lymph node, thyroglossal cyst, dermoid cyst.

Management relevance

Clinical action after recognizing Thyroid swelling moving with deglutition: confirm thyroid swelling, look for complications, and choose treatment according to the underlying diagnosis.

Common traps

- Do not confuse with lymph node: Separate from midline tract/thyroid.
- Do not confuse with thyroglossal cyst: Moves with tongue protrusion.

- Do not confuse with dermoid cyst: Midline subcutaneous lesion that usually does not move with tongue protrusion.
- Thyroid swelling moving with deglutition needs both thyroid swelling and pretracheal fascia; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Thyroid swelling moving with deglutition.
- Subject: Surgery.
- Key clue: thyroid swelling.
- Key clue: pretracheal fascia.
- Key clue: deglutition.
- Closest mimics: lymph node, thyroglossal cyst, dermoid cyst.
- Video segment: 00:00-00:13.
- One-line recall: Thyroid swelling moving with deglutition points to thyroid swelling.

Source credit: Thyroid Swelling - movement with deglutition demonstration - THE WHITE ARMY.

Topic 89 - Thyroglossal cyst moving with tongue protrusion

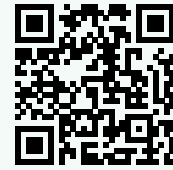
Subject: Surgery / ENT

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:20

Source credit: Thyroglossal Duct Cyst Seen Elevating on Protrusion of Tongue - Rhonique Morgan



What it is

Thyroglossal cyst moving with tongue protrusion: midline neck swelling moving with tongue protrusion. High-yield anchors: thyroglossal duct cyst; hyoid bone relation; Sistrunk operation.

How to identify it in the video

midline neck swelling moving with tongue protrusion. Watch for thyroglossal duct cyst, hyoid bone relation, Sistrunk operation. Video cues for Thyroglossal cyst moving with tongue protrusion: the visible endpoint of Thyroglossal cyst moving with tongue protrusion, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Thyroglossal cyst moving with tongue protrusion starts with thyroglossal duct cyst and is expressed clinically through hyoid bone relation. Related exam anchors are Sistrunk operation.

Associated conditions

- thyroglossal duct cyst
- hyoid bone relation
- Sistrunk operation

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
thyroid swelling	Moves with swallowing but not tongue protrusion.	Thyroid function/ultrasound.
dermoid cyst	Midline subcutaneous lesion that usually does not move with tongue protrusion.	Use the thyroglossal duct cyst endpoint to decide whether dermoid cyst explains the clip.
lymph node	Separate from midline tract/thyroid.	Assess mobility, tenderness, and drainage area.

Clinical examination points

- For Thyroglossal cyst moving with tongue protrusion, document the visible abnormality before naming the diagnosis.
- For Thyroglossal cyst moving with tongue protrusion, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Thyroglossal cyst moving with tongue protrusion is usually tested through thyroglossal duct cyst; hyoid bone relation; Sistrunk operation. Separate it from thyroid swelling, dermoid cyst, lymph node.

Management relevance

Clinical action after recognizing Thyroglossal cyst moving with tongue protrusion: confirm thyroglossal duct cyst, look for complications, and choose treatment according to the underlying diagnosis.

Common traps

- Do not confuse with thyroid swelling: Moves with swallowing but not tongue protrusion.
- Do not confuse with dermoid cyst: Midline subcutaneous lesion that usually does not move with tongue protrusion.

- Do not confuse with lymph node: Separate from midline tract/thyroid.
- Thyroglossal cyst moving with tongue protrusion needs both thyroglossal duct cyst and hyoid bone relation; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Thyroglossal cyst moving with tongue protrusion.
- Subject: Surgery / ENT.
- Key clue: thyroglossal duct cyst.
- Key clue: hyoid bone relation.
- Key clue: Sistrunk operation.
- Closest mimics: thyroid swelling, dermoid cyst, lymph node.
- Video segment: 00:00-00:20.
- One-line recall: Thyroglossal cyst moving with tongue protrusion points to thyroglossal duct cyst.

Source credit: Thyroglossal Duct Cyst Seen Elevating on Protrusion of Tongue - Rhonique Morgan.

Topic 90 - Hernia cough impulse

Subject: Surgery

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:20

Source credit: Visible cough impulse in a patient of ventral hernia - Dr. Md.Ibrahim Khalil Shahin



What it is

Hernia cough impulse: expansile cough impulse demonstration. High-yield anchors: reducible hernia; expansile cough impulse; direct vs indirect hernia.

How to identify it in the video

expansile cough impulse demonstration. Watch for reducible hernia, expansile cough impulse, direct vs indirect hernia. Video cues for Hernia cough impulse: the visible endpoint of Hernia cough impulse, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Hernia cough impulse starts with reducible hernia and is expressed clinically through expansile cough impulse. Related exam anchors are direct vs indirect hernia.

Associated conditions

- reducible hernia
- expansile cough impulse
- direct vs indirect hernia

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
hydrocele	Transilluminates and cannot get above swelling.	Cough impulse absent.
lymph node	Separate from midline tract/thyroid.	Assess mobility, tenderness, and drainage area.
saphena varix	Bluish groin swelling with cough impulse.	Disappears lying down, venous hum possible.
femoral hernia	Below and lateral to pubic tubercle.	Higher strangulation risk.

Clinical examination points

- For Hernia cough impulse, document the visible abnormality before naming the diagnosis.
- For Hernia cough impulse, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Hernia cough impulse is usually tested through reducible hernia; expansile cough impulse; direct vs indirect hernia. Separate it from hydrocele, lymph node, saphena varix.

Management relevance

Clinical action after recognizing Hernia cough impulse: confirm reducible hernia, look for complications, and choose treatment according to the underlying diagnosis.

Common traps

- Do not confuse with hydrocele: Transilluminates and cannot get above swelling.
- Do not confuse with lymph node: Separate from midline tract/thyroid.

- Do not confuse with saphena varix: Bluish groin swelling with cough impulse.
- Hernia cough impulse needs both reducible hernia and expansile cough impulse; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Hernia cough impulse.
- Subject: Surgery.
- Key clue: reducible hernia.
- Key clue: expansile cough impulse.
- Key clue: direct vs indirect hernia.
- Closest mimics: hydrocele, lymph node, saphena varix, femoral hernia.
- Video segment: 00:00-00:20.
- One-line recall: Hernia cough impulse points to reducible hernia.

Source credit: Visible cough impulse in a patient of ventral hernia - Dr. Md.Ibrahim Khalil Shahin.

Topic 91 - Varicose vein Trendelenburg test

Subject: Surgery

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Trendelenburg's Test - OSCE Guide Clip - Geeky Medics



What it is

Varicose vein Trendelenburg test: Trendelenburg test for varicose veins. High-yield anchors: saphenofemoral incompetence; perforator incompetence.

How to identify it in the video

Trendelenburg test for varicose veins. Watch for saphenofemoral incompetence, perforator incompetence. Video cues for Varicose vein Trendelenburg test: the visible endpoint of Varicose vein Trendelenburg test, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Varicose vein Trendelenburg test starts with saphenofemoral incompetence and is expressed clinically through perforator incompetence.

Associated conditions

- saphenofemoral incompetence
- perforator incompetence

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
Buerger test	Elevation pallor and dependent rubor assess arterial disease, not venous reflux.	Use the saphenofemoral incompetence endpoint to decide whether Buerger test explains the clip.
Perthes test	Assesses deep venous patency before varicose vein surgery.	Use the saphenofemoral incompetence endpoint to decide whether Perthes test explains the clip.
tourniquet tests	Tourniquet level localizes saphenofemoral or perforator incompetence.	Use the saphenofemoral incompetence endpoint to decide whether tourniquet tests explains the clip.

Clinical examination points

- Varicose vein Trendelenburg test: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Varicose vein Trendelenburg test: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Varicose vein Trendelenburg test is usually tested through saphenofemoral incompetence; perforator incompetence. Separate it from Buerger test, Perthes test, tourniquet tests.

Management relevance

Use the demonstrated technique only for the correct indication; the management decision hinges on saphenofemoral incompetence, patient safety, and the endpoint shown in the video.

Common traps

- Do not confuse with Buerger test: Elevation pallor and dependent rubor assess arterial disease, not venous reflux.
- Do not confuse with Perthes test: Assesses deep venous patency before varicose vein surgery.
- Do not confuse with tourniquet tests: Tourniquet level localizes saphenofemoral or perforator incompetence.
- Varicose vein Trendelenburg test needs both saphenofemoral incompetence and perforator incompetence; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Varicose vein Trendelenburg test.
- Subject: Surgery.
- Key clue: saphenofemoral incompetence.
- Key clue: perforator incompetence.
- Closest mimics: Buerger test, Perthes test, tourniquet tests.
- Video segment: 00:00-01:00.
- One-line recall: Varicose vein Trendelenburg test points to saphenofemoral incompetence.

Source credit: Trendelenburg's Test - OSCE Guide Clip - Geeky Medics.

Topic 92 - Buerger test

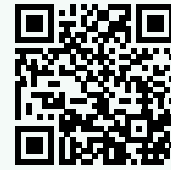
Subject: Surgery / Vascular

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: How to perform Buerger's Test - Trust me-ED



What it is

Buerger test: peripheral arterial disease bedside test. High-yield anchors: elevation pallor; dependent rubor; Buerger angle.

How to identify it in the video

peripheral arterial disease bedside test. Watch for elevation pallor, dependent rubor, Buerger angle. Video cues for Buerger test: the visible endpoint of Buerger test, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Buerger test starts with elevation pallor and is expressed clinically through dependent rubor. Related exam anchors are Buerger angle.

Associated conditions

- elevation pallor
- dependent rubor
- Buerger angle

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
venous insufficiency	Edema, pigmentation, ulcers near medial malleolus.	Duplex venous scan.
cellulitis	Hot tender erythematous skin with systemic inflammation.	Use the elevation pallor endpoint to decide whether cellulitis explains the clip.
neuropathic foot	Loss of protective sensation.	Ulcers at pressure points, pulses may be present.

Clinical examination points

- Buerger test: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Buerger test: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Buerger test is usually tested through elevation pallor; dependent rubor; Buerger angle. Separate it from venous insufficiency, cellulitis, neuropathic foot.

Management relevance

Use the demonstrated technique only for the correct indication; the management decision hinges on elevation pallor, patient safety, and the endpoint shown in the video.

Common traps

- Do not confuse with venous insufficiency: Edema, pigmentation, ulcers near medial malleolus.
- Do not confuse with cellulitis: Hot tender erythematous skin with systemic inflammation.
- Do not confuse with neuropathic foot: Loss of protective sensation.

- Buerger test needs both elevation pallor and dependent rubor; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Buerger test.
- Subject: Surgery / Vascular.
- Key clue: elevation pallor.
- Key clue: dependent rubor.
- Key clue: Buerger angle.
- Closest mimics: venous insufficiency, cellulitis, neuropathic foot.
- Video segment: 00:00-01:00.
- One-line recall: Buerger test points to elevation pallor.

Source credit: How to perform Buerger's Test - Trust me-ED.

Topic 93 - Simple interrupted suturing

Subject: Surgery

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Simple interrupted suture - OSCE Guide - Geeky Medics



What it is

Simple interrupted suturing: short simple interrupted suture demonstration. High-yield anchors: wound closure; needle angle; knot placement; spacing.

How to identify it in the video

short simple interrupted suture demonstration. Watch for wound closure, needle angle, knot placement, spacing. Video cues for Simple interrupted suturing: patient or mannequin position, hand and instrument placement, sequence of steps, endpoint and safety checks.

Why it happens

The mechanism of Simple interrupted suturing starts with wound closure and is expressed clinically through needle angle. Related exam anchors are knot placement, spacing. In Simple interrupted suturing, the outcome depends on sequence, hand position, endpoint recognition, and avoiding the unsafe step.

Associated conditions

- wound closure
- needle angle
- knot placement
- spacing

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
mattress suture	Everting tension-relieving stitch.	Different bite pattern from simple interrupted.
subcuticular suture	Intradermal cosmetic closure, no external interrupted knots.	Use the wound closure endpoint to decide whether subcuticular suture explains the clip.
continuous suture	One running strand.	Faster but failure can affect whole closure.

Clinical examination points

- For Simple interrupted suturing, document the visible abnormality before naming the diagnosis.
- For Simple interrupted suturing, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Simple interrupted suturing is usually tested through wound closure; needle angle; knot placement. Separate it from mattress suture, subcuticular suture, continuous suture.

Management relevance

Irrigate, anesthetize, approximate without strangulation, and provide tetanus/antibiotics when indicated.

Common traps

- Do not confuse with mattress suture: Everting tension-relieving stitch.

- Do not confuse with subcuticular suture: Intradermal cosmetic closure, no external interrupted knots.
- Do not confuse with continuous suture: One running strand.
- Safety trap in Simple interrupted suturing: sequence, hand position, and endpoint matter as much as naming the step.

Quick revision box

- Topic: Simple interrupted suturing.
- Subject: Surgery.
- Key clue: wound closure.
- Key clue: needle angle.
- Key clue: knot placement.
- Closest mimics: mattress suture, subcuticular suture, continuous suture.
- Video segment: 00:00-01:00.
- One-line recall: Simple interrupted suturing points to wound closure.

Source credit: Simple interrupted suture - OSCE Guide - Geeky Medics.

Topic 94 - Surgical hand scrub / gowning / gloving

Subject: Surgery

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Gowning and gloving - AMBOSS tutorial - AMBOSS: Medical Knowledge Distilled



What it is

Surgical hand scrub / gowning / gloving: short sterile gowning and gloving demonstration. High-yield anchors: surgical asepsis; sterile field; closed gloving.

How to identify it in the video

short sterile gowning and gloving demonstration. Watch for surgical asepsis, sterile field, closed gloving. Video cues for Surgical hand scrub / gowning / gloving: patient or mannequin position, hand and instrument placement, sequence of steps, endpoint and safety checks.

Why it happens

The mechanism of Surgical hand scrub / gowning / gloving starts with surgical asepsis and is expressed clinically through sterile field. Related exam anchors are closed gloving. In Surgical hand scrub / gowning / gloving, the outcome depends on sequence, hand position, endpoint recognition, and avoiding the unsafe step.

Associated conditions

- surgical asepsis
- sterile field
- closed gloving

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
sterile-field contamination	Any hand drop below waist, non-sterile contact, or wet strike-through contaminates the field.	Restart or reglove.
open gloving	Used when the operator is not yet gowned.	Fingers touch only the folded glove cuff.
alcohol hand rub	Hand rub is acceptable only when hands are visibly clean.	Scrub technique is needed for visible soil or protocol-specific first scrub.

Clinical examination points

- For Surgical hand scrub / gowning / gloving, document the visible abnormality before naming the diagnosis.
- For Surgical hand scrub / gowning / gloving, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

Surgical hand scrub / gowning / gloving is usually tested through surgical asepsis; sterile field; closed gloving. Separate it from sterile-field contamination, open gloving, alcohol hand rub.

Management relevance

Use the demonstrated technique only for the correct indication; the management decision hinges on surgical asepsis, patient safety, and the endpoint shown in the video.

Common traps

- Do not confuse with sterile-field contamination: Any hand drop below waist, non-sterile contact, or wet strike-through contaminates the field.

- Do not confuse with open gloving: Used when the operator is not yet gowned.
- Do not confuse with alcohol hand rub: Hand rub is acceptable only when hands are visibly clean.
- Safety trap in Surgical hand scrub / gowning / gloving: sequence, hand position, and endpoint matter as much as naming the step.

Quick revision box

- Topic: Surgical hand scrub / gowning / gloving.
- Subject: Surgery.
- Key clue: surgical asepsis.
- Key clue: sterile field.
- Key clue: closed gloving.
- Closest mimics: sterile-field contamination, open gloving, alcohol hand rub.
- Video segment: 00:00-01:00.
- One-line recall: Surgical hand scrub / gowning / gloving points to surgical asepsis.

Source credit: Gowning and gloving - AMBOSS tutorial - AMBOSS: Medical Knowledge Distilled.

Topic 95 - Leopold maneuvers

Subject: OBGY

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Leopold's Maneuvers - Fundamentals of Fetal Health Surveillance - UBC CPD



What it is

Leopold maneuvers: abdominal palpation in pregnancy demonstration. High-yield anchors: fetal lie; presentation; position; engagement.

How to identify it in the video

abdominal palpation in pregnancy demonstration. Watch for fetal lie, presentation, position, engagement. Video cues for Leopold maneuvers: the visible endpoint of Leopold maneuvers, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Leopold maneuvers starts with fetal lie and is expressed clinically through presentation. Related exam anchors are position, engagement.

Associated conditions

- fetal lie
- presentation
- position
- engagement
- pregnancy-related emergency
- procedure-related complication
- maternal-fetal safety issue

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
lie vs presentation vs position vs attitude	Lie is long-axis relation, presentation is presenting part.	Position is denominator direction, attitude is flexion.

Clinical examination points

- Leopold maneuvers: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- Leopold maneuvers: Know the named positive endpoint and the structure being stressed.

NEET PG exam angle

Leopold maneuvers is usually tested through fetal lie; presentation; position. Separate it from lie vs presentation vs position vs attitude.

Management relevance

Use the demonstrated technique only for the correct indication; the management decision hinges on fetal lie, patient safety, and the endpoint shown in the video.

Common traps

- Do not confuse with lie vs presentation vs position vs attitude: Lie is long-axis relation, presentation is presenting part.

- Leopold maneuvers needs both fetal lie and presentation; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Leopold maneuvers.
- Subject: OBGY.
- Key clue: fetal lie.
- Key clue: presentation.
- Key clue: position.
- Closest mimics: lie vs presentation vs position vs attitude.
- Video segment: 00:00-01:00.
- One-line recall: Leopold maneuvers points to fetal lie.

Source credit: Leopold's Maneuvers - Fundamentals of Fetal Health Surveillance - UBC CPD.

Topic 96 - McRoberts maneuver for shoulder dystocia

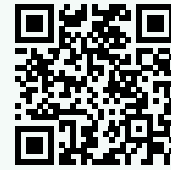
Subject: OBGY / Emergency

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: SimMom ADM Shoulder Dystocia - Laerdal Medical



What it is

McRoberts maneuver for shoulder dystocia: simulation video of McRoberts maneuver. High-yield anchors: shoulder dystocia; first-line maneuver; suprapubic pressure.

How to identify it in the video

simulation video of McRoberts maneuver. Watch for shoulder dystocia, first-line maneuver, suprapubic pressure. Video cues for McRoberts maneuver for shoulder dystocia: the visible endpoint of McRoberts maneuver for shoulder dystocia, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of McRoberts maneuver for shoulder dystocia starts with shoulder dystocia and is expressed clinically through first-line maneuver. Related exam anchors are suprapubic pressure.

Associated conditions

- shoulder dystocia
- first-line maneuver
- suprapubic pressure
- pregnancy-related emergency
- procedure-related complication
- maternal-fetal safety issue

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
fundal pressure	Contraindicated in shoulder dystocia because it worsens impaction.	Worsens impaction and increases uterine rupture risk.
Woods screw	Rotational maneuver after first-line McRoberts/suprapubic pressure if needed.	Use the shoulder dystocia endpoint to decide whether Woods screw explains the clip.
Rubin maneuver	Adducts fetal shoulder by pressure on posterior aspect.	Rotational rescue step.

Clinical examination points

- McRoberts maneuver for shoulder dystocia: Position the patient correctly, stabilize the proximal segment, apply the maneuver gently, and compare both sides.
- McRoberts maneuver for shoulder dystocia: Know the named positive endpoint and the structure being stressed.
- McRoberts maneuver for shoulder dystocia: Start with ABCs, oxygenation/ventilation, circulation, monitoring, and escalation triggers.

NEET PG exam angle

McRoberts maneuver for shoulder dystocia is usually tested through shoulder dystocia; first-line maneuver; suprapubic pressure. Separate it from fundal pressure, Woods screw, Rubin maneuver.

Management relevance

For McRoberts maneuver for shoulder dystocia, stabilize airway, breathing, circulation, and hemodynamics before definitive treatment for shoulder dystocia.

Common traps

- Do not confuse with fundal pressure: Contraindicated in shoulder dystocia because it worsens impaction.
- Do not confuse with Woods screw: Rotational maneuver after first-line McRoberts/suprapubic pressure if needed.
- Do not confuse with Rubin maneuver: Adducts fetal shoulder by pressure on posterior aspect.
- McRoberts maneuver for shoulder dystocia needs both shoulder dystocia and first-line maneuver; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: McRoberts maneuver for shoulder dystocia.
- Subject: OBGY / Emergency.
- Key clue: shoulder dystocia.
- Key clue: first-line maneuver.
- Key clue: suprapubic pressure.
- Closest mimics: fundal pressure, Woods screw, Rubin maneuver.
- Video segment: 00:00-01:00.
- One-line recall: McRoberts maneuver for shoulder dystocia points to shoulder dystocia.

Source credit: SimMom ADM Shoulder Dystocia - Laerdal Medical.

Topic 97 - Bimanual uterine compression / uterine massage for PPH

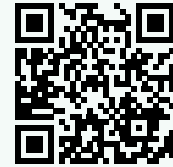
Subject: OBGY / Emergency

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: BEMONC Bimanual Uterine Compression demo - Jennifer Araneta



What it is

Bimanual uterine compression / uterine massage for PPH: simulation/mannequin demonstration. High-yield anchors: atonic PPH; uterine massage; bimanual compression; oxytocin.

How to identify it in the video

simulation/mannequin demonstration. Watch for atonic PPH, uterine massage, bimanual compression, oxytocin. Video cues for Bimanual uterine compression / uterine massage for PPH: the visible endpoint of Bimanual uterine compression / uterine massage for PPH, symmetry, rhythm and suppressibility, associated bedside sign.

Why it happens

The mechanism of Bimanual uterine compression / uterine massage for PPH starts with atonic PPH and is expressed clinically through uterine massage. Related exam anchors are bimanual compression, oxytocin.

Associated conditions

- atonic PPH
- uterine massage
- bimanual compression
- oxytocin
- pregnancy-related emergency
- procedure-related complication
- maternal-fetal safety issue

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
traumatic PPH	Firm uterus with bleeding suggests laceration.	Rupture, or hematoma.
retained placenta	Placenta incomplete or not delivered.	Examine placenta and consider manual removal.
uterine inversion	Shock, severe pain, absent fundus, mass at introitus.	Replace uterus urgently.

Clinical examination points

- Bimanual uterine compression / uterine massage for PPH: Start with ABCs, oxygenation/ventilation, circulation, monitoring, and escalation triggers.

NEET PG exam angle

Bimanual uterine compression / uterine massage for PPH is usually tested through atonic PPH; uterine massage; bimanual compression. Separate it from traumatic PPH, retained placenta, uterine inversion.

Management relevance

Atonic PPH needs uterine massage, oxytocin, bimanual compression, escalation, and resuscitation.

Common traps

- Do not confuse with traumatic PPH: Firm uterus with bleeding suggests laceration.
- Do not confuse with retained placenta: Placenta incomplete or not delivered.
- Do not confuse with uterine inversion: Shock, severe pain, absent fundus, mass at introitus.
- Bimanual uterine compression / uterine massage for PPH needs both atonic PPH and uterine massage; a clip showing only one anchor is a weaker fit.

Quick revision box

- Topic: Bimanual uterine compression / uterine massage for PPH.
- Subject: OBGY / Emergency.
- Key clue: atonic PPH.
- Key clue: uterine massage.
- Key clue: bimanual compression.
- Closest mimics: traumatic PPH, retained placenta, uterine inversion.
- Video segment: 00:00-01:00.
- One-line recall: Bimanual uterine compression / uterine massage for PPH points to atonic PPH.

Source credit: BEMONC Bimanual Uterine Compression demo - Jennifer Araneta.

Topic 98 - Pap smear collection technique

Subject: OBGY

Video

[Click here to watch video](#)

Suggested segment: 00:00-00:54

Source credit: Pap Smear Demonstration on Model Figure - Dr Natalia Novikova



What it is

Pap smear collection technique: simulation or model-based Pap smear demonstration. High-yield anchors: transformation zone; Ayre spatula; cytobrush; cervical cancer screening.

How to identify it in the video

simulation or model-based Pap smear demonstration. Watch for transformation zone, Ayre spatula, cytobrush, cervical cancer screening. Video cues for Pap smear collection technique: patient or mannequin position, hand and instrument placement, sequence of steps, endpoint and safety checks.

Why it happens

The mechanism of Pap smear collection technique starts with transformation zone and is expressed clinically through Ayre spatula. Related exam anchors are cytobrush, cervical cancer screening.

Associated conditions

- transformation zone
- Ayre spatula
- cytobrush
- cervical cancer screening
- pregnancy-related emergency
- procedure-related complication
- maternal-fetal safety issue

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
VIA	Acetic acid visual inspection.	Not cytology sampling from transformation zone.
HPV testing	Molecular high-risk HPV test.	Sample handling differs from Pap cytology.
colposcopy-guided biopsy	Directed tissue biopsy after abnormal screen, not screening collection itself.	Use the transformation zone endpoint to decide whether colposcopy-guided biopsy explains the clip.

Clinical examination points

- Pap smear collection technique: Inspect pinna/canal, test tragal tenderness, visualize tympanic membrane landmarks, mobility, perforation, and discharge.

NEET PG exam angle

Pap smear collection technique is usually tested through transformation zone; Ayre spatula; cytobrush. Separate it from VIA, HPV testing, colposcopy-guided biopsy.

Management relevance

Sample the transformation zone and follow age-appropriate cervical-screening algorithms.

Common traps

- Do not confuse with VIA: Acetic acid visual inspection.
- Do not confuse with HPV testing: Molecular high-risk HPV test.
- Do not confuse with colposcopy-guided biopsy: Directed tissue biopsy after abnormal screen, not screening collection itself.
- Safety trap in Pap smear collection technique: sequence, hand position, and endpoint matter as much as naming the step.

Quick revision box

- Topic: Pap smear collection technique.
- Subject: OBGY.
- Key clue: transformation zone.
- Key clue: Ayre spatula.
- Key clue: cytobrush.
- Closest mimics: VIA, HPV testing, colposcopy-guided biopsy.
- Video segment: 00:00-00:54.
- One-line recall: Pap smear collection technique points to transformation zone.

Source credit: Pap Smear Demonstration on Model Figure - Dr Natalia Novikova.

Topic 99 - IUCD insertion technique

Subject: OBGY

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Insertion of an IUCD - OBG by Dr Raina



What it is

IUCD insertion technique: simulation/model-based Copper-T insertion demonstration. High-yield anchors: timing; contraindications; complications; perforation risk.

How to identify it in the video

simulation/model-based Copper-T insertion demonstration. Watch for timing, contraindications, complications, perforation risk. Video cues for IUCD insertion technique: patient or mannequin position, hand and instrument placement, sequence of steps, endpoint and safety checks.

Why it happens

The mechanism of IUCD insertion technique starts with timing and is expressed clinically through contraindications. Related exam anchors are complications, perforation risk. In IUCD insertion technique, the outcome depends on sequence, hand position, endpoint recognition, and avoiding the unsafe step.

Associated conditions

- timing
- contraindications
- complications
- perforation risk
- pregnancy-related emergency
- procedure-related complication
- maternal-fetal safety issue

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
emergency contraception	Post-coital contraception timing.	Copper IUCD can be used within guideline window.
postpartum IUCD	Insertion timing is immediately postplacental or delayed.	Expulsion counseling matters.
interval IUCD	Inserted when not pregnant outside postpartum/postabortal setting.	Use the timing endpoint to decide whether interval IUCD explains the clip.

Clinical examination points

- For IUCD insertion technique, document the visible abnormality before naming the diagnosis.
- For IUCD insertion technique, correlate history, focused examination, and the most specific confirmatory test.

NEET PG exam angle

IUCD insertion technique is usually tested through timing; contraindications; complications. Separate it from emergency contraception, postpartum IUCD, interval IUCD.

Management relevance

Exclude pregnancy and infection risk; counsel about perforation, expulsion, bleeding, and strings.

Common traps

- Do not confuse with emergency contraception: Post-coital contraception timing.
- Do not confuse with postpartum IUCD: Insertion timing is immediately postplacental or delayed.
- Do not confuse with interval IUCD: Inserted when not pregnant outside postpartum/postabortal setting.
- Safety trap in IUCD insertion technique: sequence, hand position, and endpoint matter as much as naming the step.

Quick revision box

- Topic: IUCD insertion technique.
- Subject: OBGY.
- Key clue: timing.
- Key clue: contraindications.
- Key clue: complications.
- Closest mimics: emergency contraception, postpartum IUCD, interval IUCD.
- Video segment: 00:00-01:00.
- One-line recall: IUCD insertion technique points to timing.

Source credit: Insertion of an IUCD - OBG by Dr Raina.

Topic 100 - Neonatal resuscitation initial steps

Subject: Pediatrics / Emergency

Video

[Click here to watch video](#)

Suggested segment: 00:00-01:00

Source credit: Cord Management, Initial Steps, & Ventilation, Neonatal Resuscitation Education Video 1, 2023 - TexSun NeoEd



What it is

Neonatal resuscitation initial steps: mannequin/simulation video of initial NRP steps. High-yield anchors: warm; dry; stimulate; airway positioning; assess breathing and heart rate; bag-mask ventilation.

How to identify it in the video

mannequin/simulation video of initial NRP steps. Watch for warm, dry, stimulate, airway positioning. Video cues for Neonatal resuscitation initial steps: patient or mannequin position, hand and instrument placement, sequence of steps, endpoint and safety checks.

Why it happens

The mechanism of Neonatal resuscitation initial steps starts with warm and is expressed clinically through dry. Related exam anchors are stimulate, airway positioning, assess breathing and heart rate, bag-mask ventilation. In Neonatal resuscitation initial steps, the outcome depends on sequence, hand position, endpoint recognition, and avoiding the unsafe step.

Associated conditions

- warm
- stimulate
- airway positioning
- assess breathing
- heart rate
- bag-mask ventilation
- developmental age-related normal variant
- congenital or inherited disorder

Differential diagnosis / how to distinguish

Differential diagnosis	How to distinguish	Specific exam clue / test
when to start chest compressions	In NRP, start compressions if HR remains below 60/min after effective PPV.	Use the warm endpoint to decide whether when to start chest compressions explains the clip.
oxygen use	Use gestation-appropriate oxygen and pulse oximetry targets.	Avoid routine 100 percent oxygen.
delayed cord clamping	Delay only if newborn is vigorous and resuscitation is not immediately required.	Use the warm endpoint to decide whether delayed cord clamping explains the clip.

Clinical examination points

- Neonatal resuscitation initial steps: Start with ABCs, oxygenation/ventilation, circulation, monitoring, and escalation triggers.

NEET PG exam angle

Neonatal resuscitation initial steps is usually tested through warm; dry; stimulate. Separate it from when to start chest compressions, oxygen use, delayed cord clamping.

Management relevance

Warm, dry, stimulate, position airway, assess breathing and heart rate, then provide PPV if needed.

Common traps

- Do not confuse with when to start chest compressions: In NRP, start compressions if HR remains below 60/min after effective PPV.
- Do not confuse with oxygen use: Use gestation-appropriate oxygen and pulse oximetry targets.
- Do not confuse with delayed cord clamping: Delay only if newborn is vigorous and resuscitation is not immediately required.
- Safety trap in Neonatal resuscitation initial steps: sequence, hand position, and endpoint matter as much as naming the step.

Quick revision box

- Topic: Neonatal resuscitation initial steps.
- Subject: Pediatrics / Emergency.
- Key clue: warm.
- Key clue: dry.
- Key clue: stimulate.
- Closest mimics: when to start chest compressions, oxygen use, delayed cord clamping.
- Video segment: 00:00-01:00.
- One-line recall: Neonatal resuscitation initial steps points to warm.

Source credit: Cord Management, Initial Steps, & Ventilation, Neonatal Resuscitation Education Video 1, 2023 - TexSun NeoEd.

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